The rules governing medicinal products in the European Union VOLUME 10 –

Guidance document applying to clinical trials

CLINICAL TRIALS REGULATION (EU) NO 536/2014 QUESTIONS & ANSWERS

VERSION 7.4

Amended and endorsed through written procedure

by the Clinical Trials Coordination and Advisory Group

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Important notice: The views expressed in this questions and answers document are not legally binding. Ultimately, only the European Court of Justice can give an authoritative interpretation of Community law. This document aims at informing on the technical aspects of Commission Clinical Trials Regulation (EU) No 536/2014 with a view to facilitating its implementation.

This document sets out frequently-asked 'questions and answers' regarding the implementation of the rules on clinical trials. Updates to this questions and answers document used to be presented and discussed within the "Expert group on clinical trials". The group ceased to exist in April 2024.

In order to assure the coherent implementation of the Clinical Trial Regulation, the Commission consults the Clinical Trials Coordination and Advisory Group (CTAG) as its members are the National Contact Points defined in the CTR, for necessary amendments and endorsement.

The aim of this document is to provide general guidance on the implementation of the CTR, and should be read in combination with:

- Guidance for the Transition of clinical trials from the Clinical Trials Directive to the Clinical Trials Regulation: https://health.ec.europa.eu/document/download/10c83e6b-2587-420d-9204- d49c2f75f476_en?filename=transition_ct_dir-reg_guidance_en.pdf

- The CTIS Sponsor Handbook , which is the primary training resource for CTIS, and is available here: https://www.ema.europa.eu/en/documents/other/clinical-trial-information-system-ctis-sponsor-handbook_en.pdf
- More specific documents published on Eudralex 10: https://ec.europa.eu/health/documents/eudralex/vol-10_en#fragment1

  Chapter 7 on "Safety Reporting" was drafted by the Clinical Trials Facilitation and Coordination Group of the Heads of Medicines Agency (CTFG) and was endorsed by the Expert Group on Clinical Trials of the European Commission.

  Q&A 2.8 "How to use conditions" was endorsed also by CTFG.

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THE SCOPE OF THE EU CLINICAL TRIALS REGULATION

1.1 Question: What are the new characteristics of the Clinical Trials Regulation (EU) No 536/2014 as compared to the Clinical Trials Directive 2001/20/EC?

1. **Answer:** The new Clinical Trials legislation has taken the legal form of a Regulation[1]and will replace national law. This will ensure that the rules for assessing clinical trial applications and for conducting clinical trials are identical throughout the EU. This is vital to ensure that Member States, in authorising and supervising the conduct of a clinical trial, base themselves on the same rules.

2. The Clinical Trials Regulation aims to create an environment that is favourable for conducting clinical trials, with the highest standards of patient safety, for all EU Member States. It will not only harmonize decisions, but also foster work sharing and collaboration between Member States.

3. The main characteristics of the new Regulation are:

    - A streamlined application procedure via a single entry point - an EU portal and database, for all clinical trials conducted in EEA. Registration via the portal will be a prerequisite for the assessment of any application;
    - A single set of documents to be prepared and submitted for the application defined in Annex I of the Regulation;
    - A single authorisation procedure for all clinical trials, allowing a faster and thorough assessment of an application by all Member States concerned, and ensuring one single assessment outcome and authorisation per Member State;
    - A harmonised procedure for the assessment of applications for clinical trials, which is divided in two parts. Part I is jointly assessed by all Member States concerned. Part II is assessed by

[1] OJ L 158, 27.05.2014 https://eur-lex.europa.eu/legal-content/EN/ALL/?uri=CELEX:32014R0536

each Member State concerned separately;

- Strictly defined deadlines for the assessment of clinical trial application;
- The involvement of the ethics committees in the assessment procedure in accordance with the national law of the Member state concerned but within the overall timelines defined by the Regulation;
- Simplified reporting procedures which will spare sponsors from submitting broadly identical information separately to various bodies and different Member States;
- Clinical trials conducted outside the EU, but referred to in a clinical trial application within the EU, will have to comply with regulatory requirements that are at least equivalent to those applicable in the EU;
- Strengthened transparency for clinical trials data;
- A coordination and advisory committee that will serve as a forum for exchanging best practices between Member States;
- Union controls in Member states and third countries to ensure that clinical trials rules are being properly supervised and enforced.

1.2 Question: Till when is the Clinical Trial Directive 2001/20/EC applicable?

4. **Answer:** Directive 2001/20/EC will be repealed on the day of entry into application of the Clinical Trials Regulation (EU) No 536/2014. It will however still apply three years from that day to:

- Clinical trials applications submitted before the entry into application of Regulation (EU) No 536/2014 and
- Clinical trials applications submitted within one year after the entry into application of Regulation (EU) No 536/2014, if the sponsor opts for the old system.

1.3. Question: What is a “clinical trial”?

5. **Answer:** Article 2(2) (1 and 2) of the Clinical Trials Regulation provides a definition of a "clinical study" as well as a “clinical trial”:

    a. A ‘Clinical study’ means any investigation in relation to humans intended: (a) to discover or verify the clinical, pharmacological or other pharmacodynamic effects of one or more medicinal products; (b) to identify any adverse reactions to one or more medicinal products; or (c) to study the absorption, distribution, metabolism and excretion of one or more medicinal products; with the objective of ascertaining the safety and/or efficacy of those medicinal products;
    b. "Clinical trial’ means a clinical study which fulfils any of the following conditions:

        (a) the assignment of the subject to a particular therapeutic strategy is decided in advance and does not fall within normal clinical practice of the Member State concerned; (b) the decision to prescribe the investigational medicinal products is taken together with the decision to include the subject in the clinical study; or (c) diagnostic or monitoring procedures in addition to normal clinical practice are applied to the subjects.

    c. The decision tree in Annex I can be used to identify whether a trial is a clinical trial in the sense of Regulation (EU) No 536/2014.

1.4. Question: what document/data shall be submitted with an application?

6. **Answer**: The CTR provides an exhaustive list of the documentary and information requirements, for both the ethics committee and competent authority in the concerned Member State, for a clinical trial application, substantial modification and/or subsequent addition of a Member State. The submission needs to be in compliance with the CTR, in particular but not limited to annex I and II, adapted as necessary in line with national legislation (for example with regards to damage compensation, financial agreements, proof of payment). The dossier should be complete and clear in order to facilitate the coordinated assessment within the CTR deadlines.

7. A set of templates was developed to streamline the implementation of the Part II requirements, and published on the volume 10 website. Specific

Member States Part II requirements can be accessed through the weblinks provided in annex III of this document, which also provides e-mail addresses for enquiries about Part I and Part II national requirements. Please note that the national competent authorities may not reply to enquiries for which a reply is already available either in the Clinical Trials Regulation (EU) 536/2014 or in the Questions and Answers document available on Eudralex volume 10 or in national Question and Answer documents. The European Commission is not responsible for the quality and completeness of the information reported in the Annex III nor for the functioning of the websites. For questions and remarks on the links and on the information reported in the websites listed below, please contact the national contact point(s).

8. Documents and/or information that are not required as per the CTR, including signatures on various part I and II documents (e.g. investigator CV, Declaration of Interest, cover letter), are not part of the clinical trial application, Request for Information, or subsequent steps can therefore not be requested by Member States. The site suitability statement mentioned in Annex I, section N., point 67. shall be submitted according to the system of the Member State concerned, meaning that signature requirements for this document are subject to national law. Importantly, electronic submission of the CTA to CTIS by the sponsor is regarded as equivalent to signing the document in accordance to Annex I.3. CTR is a regulation, which is directly applicable and ensures complete harmonisation of the sector, national laws should be set out to support its full implementation.

1.5. How to proceed in case of discrepancies between the CTR and ICH Good clinical practice guidance?

9. **Answer:** In the EU, all submissions concerning a clinical trial, including information on the clinical trial sites, are done by or on behalf of the sponsor (and not by the investigator) via CTIS or EudraVigilance.

10. The single decision, per Member State, on each clinical trial application and subsequent changes to that application at Member State level represents the outcome of scientific and ethical review, involving an Ethics Committee, in compliance with the CTR and in accordance with the national law of the MSC.

11. As recognized also in ICH, sponsors and investigators need to comply with the applicable regulatory requirements. The CTR aims to ensure maximum possible harmonisation across the EU/EEA (recital 5) and, takes precedence over

conflicting rules in guidelines, albeit ICH or other guidelines. Documents or data that are not foreseen by the Regulation (e.g. the progress report as defined in the ICH E.6 guidance) shall not be requested or submitted based on recommendations in different guidelines. This approach reflects also recital 24, stating that the content of the application dossier should be harmonised to simplify the application process for clinical trials.

12. When Member States are allowed margin for national practices it is explicitly provided in the CTR itself, see for example Article 34 (military, prison), Article 74 (legal representative/contact person) or Article 86 (fees, cost recovery).

1.6. Question: What is a “low-intervention clinical trial”?

13. Answer: A “low intervention clinical trial” is defined in Article 2 (2)(3) of the Clinical Trials Regulation as a clinical trial which fulfils all of the following conditions:

(a) the investigational medicinal products, excluding placebos, are authorised;

(b) according to the protocol of the clinical trial, (i) the investigational medicinal products are used in accordance with the terms of the marketing authorisation; or (ii) the use of the investigational medicinal products is evidence-based and supported by published scientific evidence on the safety and efficacy of those investigational medicinal products in any of the Member States concerned; and

(c) the additional diagnostic or monitoring procedures do not pose more than minimal additional risk or burden to the safety of the subjects compared to normal clinical practice in any Member State concerned;

14. The decision tree in Annex I can be used to identify whether a trial is a low-intervention clinical trial in the sense of the Clinical Trials Regulation.

1.7. Question: What can be considered as a “non-interventional study”?

15. Answer: According to Article 1 of the Clinical Trials Regulation, non-interventional studies are excluded from the scope of this Regulation.

16. A “non-interventional study” is defined in Article 2(2)(4) of the Clinical Trials Regulation as "a clinical study other than a clinical trial".

17. Thus, a study is non-interventional if it does not fulfil any of the following conditions which define a Clinical Trial (according to Article 2 (2)(2) of the Clinical Trials Regulation:

a) the assignment of the subject to a particular therapeutic strategy is decided in advance and does not fall within normal clinical practice of the Member State concerned;

b) the decision to prescribe the investigational medicinal products is taken together with the decision to include the subject in the clinical study; or

c) diagnostic or monitoring procedures in addition to normal clinical practice are applied to the subjects.

18. The decision tree in Annex I can be used to identify whether a trial is a non-intervention clinical trial in the sense of Regulation (EU) No 536/2014.

19. The purpose for excluding these trials from the scope of the Regulation (EU) No 536/2014 is that these trials are typically considered to have the lowest risk. Moreover, this restriction shall ensure that medical activities which are normal clinical practice (see also Q&A 1.18) and as such, part of the general medical surveillance of a patient, are excluded from the scope of the Regulation (EU) No 536/2014.

1.8. Question: Is the definition of 'medicinal product' relevant for the scope of the Clinical Trials Regulation?

20. Answer: Yes.

21. When assessing whether a study is a clinical trial as defined in Regulation (EU) No 536/2014, the first question is always whether the object of the study is a medicinal product (see also the algorithm in Annex I).

22. 'Medicinal product' is defined in Article 1(2) of Directive 2001/83/EC. Article 1(2) of the Medicinal Products Directive defines “medicinal product” as follows: “(a) Any substance or combination of substances presented as having properties for treating or preventing disease in human beings; or (b) Any substance or combination of substances which may be used in or administered to human beings either with a view to restoring, correcting or modifying physiological functions by exerting a pharmacological, immunological or metabolic action, or to making a medical diagnosis.”

23. A substance is thus a medicinal product either by virtue of its "presentation" or its "function". A substance constitutes a medicinal product if it falls within either of these two categories.

24. To establish the 'borderline' between a medicinal product and other products, the established criteria, as further explained in detailed Commission guidance apply. Such Commission guidance exists in particular for the borderline:

a. Medicinal product – cosmetic product[2]

b. Medicinal product – medical device[3]

c. Medicinal product - food supplement[4]

25. With regard to a medicinal product by "virtue of function", in some cases it may not be 100% certain whether the product which is object of the study exerts a pharmacological, immunological or metabolic action. The term "medicinal product", as read in the context of the Clinical Trials Regulation should also encompass the products where the pharmacological, immunological, or metabolic action is still uncertain and being explored.

26. This includes also medicinal products which are specifically addressed in the EU law on pharmaceuticals, such as advanced therapy medicinal products[5] or medicinal products derived from human blood or human plasma as defined in Article 1(10) of Directive 2001/83/EC. It is important to keep in mind that specific guidance[6] exists on the classification of a medicinal product as an advanced therapy medicinal product for marketing authorization applications.

27. The Regulation also applies to interventional clinical trials with medicinal products for the paediatric population and interventional clinical trials with medicinal products manufactured or reconstituted in a (hospital)

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[2]Available here: https://ec.europa.eu/growth/sectors/cosmetics/products/borderline-products_en

[3] Available here : https://ec.europa.eu/growth/sectors/medical-devices/guidance_en

[4] DIRECTIVE 2002/46/EC published on 10 June 2002 at http://eur-lex.europa.eu/legal- content/EN/TXT/PDF/?uri=CELEX:32002L0046&from=EN

[5] As defined in Article 2(1)(a) of Regulation (EC) No 1394/2007 of the European Parliament and of the Council of 13 November 2007 on advanced therapy medicinal products and amending Directive 2001/83/EC and Regulation (EC) No 726/2004 (OJ L 324, 10.12.2007, p. 121) (hereinafter Regulation (EC) No 1394/2007).

[6] Available here : https://www.ema.europa.eu/en/documents/scientific-guideline/reflection-paper-classification- advanced-therapy-medicinal-products_en-0.pdf

pharmacy and intended to be supplied directly to the clinical trials subjects.

28. To draw the 'borderline' between these sectoral legislations (e.g. medicinal products/food, medicinal products/cosmetic products, medicinal products/medical devices), the established criteria as set out in the case law of the European Court of Justice apply and reference is made to the relevant guidelines[7].

29. The classification of a substance as a medicinal product is the sole responsibility of the member states. Sponsors should seek advice at the level of the member states concerned if the status of a research product is unclear.

1.9. Question: What is not considered as "normal clinical practice"?

30. For the classification as a clinical trial vs. a non-interventional study the assignment to one of the following therapeutic strategies is NOT considered „normal clinical practice" as defined by Article 2 (6) of Regulation (EU) 536/2014:

- Administration of a medicinal product without a marketing authorisation in the EEA[8]
- Administration of a medicinal product in healthy volunteers or in patients without clinical indication or medical need.
- Other unproven interventions as defined in Article 37 of the Declaration of Helsinki.
- Blinding or randomisation of treatment allocation.
- Additional or more frequent/increased diagnostic or monitoring procedures or sampling performed solely for the purposes of the clinical study.
- Any procedures not considered clinical practice for the individual patient within the framework of the National Healthcare System of the Member State concerned with the clinical study.

31. With regard to off-label use of medicinal products with a marketing

[7] cf., for example, http://ec.europa.eu/growth/sectors/cosmetics/products/borderline-products/index_en.htm

[8] The systematic investigation of medicinal products where no marketing authorization is foreseen, e.g. magisterial formulations, is restricted to clinical trials.

authorisation in the EEA it is within the competence of each Member State to determine if established off-label use in principle is considered within their normal clinical practice and can be investigated in a non-interventional study or not.

32. Sponsors are recommended at the planning stage of such a clinical study/clinical trial to seek advice from all Member States where the study/trial is intended to take place. A clinical trial application should then be submitted to all Member States where the conduct of a non-interventional study is not possible.

1.10. Question: A study might involve the administration of a medicinal product, while the object of the investigation is not the administered medicinal product, but exclusively the physiology of the body. Are these studies 'clinical trials' as defined in Regulation (EU) No 536/2014?

33. Answer: No.

34. There may be studies, which have the only objective to investigate the physiology of the body. In these investigations the medicinal product is used as *a tool* with the aim to provoke a *well characterized physiological response* in humans. These studies should not address the diagnostic, prophylactic or therapeutic potential of the medicinal product nor its pharmacokinetic or pharmacodynamic profile. For medicinal products that do not have marketing authorisation, the *desired pharmacological response should be corroborated by published scientific evidence in humans on safety and efficacy supporting the chosen dose level and route of administration*. Examples are a study of the physiology of the retina where a pupil dilator may be used in order to enable the study of the physiology of the retina. Another example is the use of a vasodilator to study how the endothelial function is affected by disease (or other factors not including medicinal products), the use of diagnostic agents to study the effect of disease (or other factors not including medicinal products) or the use of a challenge agent to study the effect of disease (or other factors not including medicinal products). This issue is also relevant for radiopharmaceuticals used as diagnostic agents (see Q1.8).

35. These studies are not 'clinical trials' as defined in article 2(2)(2) of Regulation (EU) No 536/2014. Consequently, the medicinal product

administered is not an investigational medicinal product as defined in article 2 (2)(5) of Regulation (EU) No 536/2014.

36. These studies are not regulated at EU-level. It is up to Member States to decide whether and how they to regulate these studies. For medicinal products that do not have a marketing authorisation, the desired pharmacological response should be corroborated by published scientific evidence on safety and efficacy in humans, supporting the chosen dose level and route of administration.

37. However, care has to be taken as to whether the object of an investigation is being 'switched', in the course of a study, from the physiology of the body to the pharmacological effect triggered by the medicinal product. In this case, a study may 'turn into' a clinical trial which falls within the scope of Regulation (EU) No 536/2014, provided it is not non- interventional (defined in article 2 (2)(4) of Regulation (EU) No 536/2014).

1.11. Question: How does the issue set out in Question 1.6 apply to PET studies?

38. **Answer**: A radiopharmaceutical used as diagnostic agent in a positron emission tomography (PET) study is a medicinal product.

39. If the object of the study is the diagnostic potential of the diagnostic agent, the study is a clinical trial and the diagnostic agent is the investigational medicinal product (IMP).

40. Studies may have as object a medicinal product 'A' (radiopharmaceutical or other) while, in addition, a diagnostic agent 'B' is used to study the effect of the medicinal product 'A'. In this case, the study is a clinical trial. In this study, the medicinal product 'A' is an investigational medicinal product as defined in article 2 (2)(5) of Regulation (EU) No 536/2014. However, the medicinal product 'B' is not an investigational medicinal product as defined in article 2 (2)(5) of the Clinical Trials Regulation.

41. If the object of the study is only a physiological characteristic where the PET is merely used to study that characteristic, i.e. there is no medicinal product being the object of the study, the study is not a clinical trial. These studies are not regulated at EU-level. It is up to Member States to decide whether and how they to regulate these studies.

1.12. Question: A study might involve a medical device – what does this mean in terms of EU regulation of clinical trials?

42. Answer: In terms of EU-regulation for clinical trials, a medical device can play a role in different contexts:

43. a) The object of the study is one integral product which is a 'combination' of a medical device and a medicinal product[9]: In these cases, firstly the regulatory status of this product (either medicinal product or medical device) needs to be determined in accordance with the definitions in the applicable legislation[10].

In deciding whether the product falls under the definition 'medicinal product' or 'medical device', particular account shall be taken of the principal mode of action. Further information is set out in Commission guidance[11].

If this assessment reveals that the product which is the object of the study is a medicinal product, the regulatory framework of the Clinical Trials Regulation applies. If this assessment reveals, however, that the product which is the object of the study is a medical device, the Clinical Trials Regulation does not apply. For example, in the case of a prefilled syringe, this product would usually be a medicinal product (with an integral 'delivery product')[12]. An interventional study would be a clinical trial and thus fall within the regulatory framework of the Clinical Trials Regulation.

b) The object of the study is a medicinal product - however, during the clinical trial medical devices are used (this is frequently the case in practice; sometimes the medical devices are supplied by the sponsor) without these being the object of a study: In these cases, the Clinical Trials Regulation applies. The medical devices not being object of the study have to comply with the EU-rules for the placing on the market and putting into service of medical devices.

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[9] This includes also 'combined advanced therapy medicinal products' as defined in Article 2(1)(d) of the Regulation (EC) 1394/2007 of the European Parliament and of the Council of 13 November 2007 on advanced therapy medicinal products.

[10] Regarding medical devices Directive 93/42/EEC which will be repealed by Regulation (EU) 2017/745, regarding in vitro diagnostic medical devices Directive 98/79/EC, which will be repealed by Regulation (EU) 2017/746 and regarding active implantable medical devices Directive 90/385/EEC which will be repealed by Regulation (EU) 2017/745. For further info see https://ec.europa.eu/growth/sectors/medical-devices/regulatory-framework_en#new_regulations).

[11] https://ec.europa.eu/growth/sectors/medical-devices/regulatory-framework_en#current_legislation

[12] See point B.2.1 of MEDDEV 2.1/3 rev 3 available here: https://ec.europa.eu/growth/sectors/medical-devices/guidance_en

The object of the study is two separate products: one is a medicinal product and one is a medical device. These two separate products may be administered/used on subjects in the same group ('arm'), or in different 'arms' (for example, a study might compare a warming medical device applied on the skin with a warming medicinal product applied topically). In these cases the Clinical Trials Regulation applies to the aspect of the study having the medicinal product as the object of the study. Regarding the medical device being the object of the study, the Clinical Trials Regulation does not apply, but the EU-rules applicable to medical devices would apply.

1.13. Question: Is a study addressing the time of surgery a clinical trial, if patients receive otherwise standard treatment with medicines?

44. **Answer**: This is a case by case decision and it depends on whether the object of the study is one of those listed in article 2 (2)(1) of the Clinical Trials Regulation and whether it fulfils the conditions in article 2 (2)(2) of the Regulation. If this is not the case, the study is not a clinical trial. The sponsor has the responsibility to provide clear information on the object of the study.

1.14. Question: Does the Clinical Trials Regulation apply to clinical trials with IMPs which fall under the 'hospital exemption' for advanced therapy medicinal products?

45. **Answer**: Yes. The 'hospital exemption' for advanced therapy medicinal products, which is contained in article 3(7) of the Directive 2001/83/EC is irrelevant for the scope of the Clinical Trials Regulation. Regulation (EU) No 536/2014 applies to any clinical trial with advanced therapy investigational medicinal products (see definition in article 2(2)(7) of the Regulation).

1.15. Question: Is an authorised medicinal product used as comparator in a clinical trial considered to be an investigational medicinal product?

46. Answer: Yes. According to article 2 (2)(5) of the Clinical Trials Regulation, an investigational medicinal product (IMP) is "*a medicinal product which is being tested or used as a reference, including as a placebo, in a clinical trial*".

47. Comparators are medicinal products used as a reference in a clinical trial vis-à-vis the substance being tested.

48. The purpose for the inclusion of comparators into the definition of IMP is that they play a fully equivalent, symmetric role as counterparts to the "tested products", and this from the inception of the protocol to the interpretation of the study results. The comparator is an IMP and the conditions (circuit, storage, traceability, return, destruction and accountability methods) under which the comparator is used are to be strictly the same as those of the "tested product", taking into account whether the IMP is an authorised IMP and whether the clinical trial is a low-intervention trial.

1.16. Question: What are the regulatory requirements for IMPs?

49. Answer: Regarding IMPs there are a number of regulatory requirements. Note, however, that the regulatory framework is adapted to situations where the IMP is used in the authorised form and for the authorised indication. This holds in particular for:

a. the information requirements for request for authorisation to be submitted to the national competent authority of the Member State concerned; and

b. the requirements for the labelling of IMP a set out in articles 66-69 of Regulation (EU) No 536/2014. (See also question 2.6).

1.17. Question: What is considered to be an auxiliary product?

50. Answer: Investigational medicinal products shall be distinguished from

auxiliary medicinal products. Auxiliary medicinal products are used in the context of a clinical trial as described in the protocol[13] for background treatments, as challenging agents, rescue medication or to assess the end-points. (See also section 8 of this Q&A on *"Authorisation of manufacturing and importation of IMPs"* and the recommendations of the expert group on clinical trials on *"Auxiliary medicinal products in clinical trials*", version March 2024[14]).

51. The documentation requirements set out in sections F and G of Annex I of the Clinical Trials Regulation also apply to auxiliary medicinal products. However, where the auxiliary medicinal product is authorised in the Member State concerned and not modified, no additional information is required. See chapter 3.2 and 3.3 of the recommendation paper.

52. In principle, only authorised medicinal products should be used as auxiliary medicinal products in clinical trials (article 59 of the Clinical Trials Regulation). However, in certain circumstances unauthorised auxiliary medicines may be used. This has to be justified in the protocol.

53. The acceptable reasons for admitting non-authorised auxiliary medicinal products would be related to the availability of authorised auxiliary medicinal products (e.g. no authorised medicinal products exist in the EU, or the amounts available are not sufficient to satisfy the need of the clinical trial). The lower price of non-authorised auxiliary medicinal product shall not be considered as a legitimate justification[15].

1.18. Question: Can a study be considered as clinical trial within the scope of Regulation (EU) No 536/2014 if it starts after administration/exposure of the investigational medicinal product has finished?

54. Answer: Yes. The start of a clinical trial is defined in Article 2(25) of Regulation (EU) No 536/2014 (see also Q&A 10.1). Normally, it is the first act of recruitment of a potential subject, unless otherwise defined in the Protocol. It cannot be excluded, however, that a protocol will set the start of clinical study after the exposure to the investigational medicinal product has finished (eg. clinical study that starts after the administration of an

[13] Article 2(2)(8) of Regulation (EU) No 536/2014

[14] https://health.ec.europa.eu/document/download/1bf35e45-134c-4f2e-9c68-0e8537eef867_en?filename=mp_axmp_rec_en.pdf

[15] Recital 53 of Regulation (EU) No 536/2014

ATMP to investigate long term efficacy and safety; follow-up for late onset side-effects of oncological treatments; or a clinical study comparing response in patient populations on different prior treatment regimes).

55. If the study fulfils the criteria of a clinical trial, and is not a non-interventional study, Regulation (EU) No 536/2014 applies. When assessing whether the study shall be considered as a clinical trial or not, a reference should be made to the algorithm in Annex I.

56. In these cases, since the administration of the medicinal product is finished by the time the trial starts, certain rules relating to the IMP (e.g. on labelling) would not be applicable.

57. In these trials and in particular, when the medicinal product had not been administered in the context of a clinical trial and therefore in accordance with good clinical practice, additional design considerations ensuring data robustness is especially important.

58. In studies when IMP exposure has started before authorization and trial start, the protocol needs to describe particularities for the sponsor in terms of recording study start.

1.19. Question: Which principles of Good Laboratory Practice (GLP) need to be taken into account for the authorisation of clinical trials?

59. **Answer:** In accordance with article 25 (3) of the Clinical Trials Regulation, non-clinical information submitted in an application dossier shall be based on data derived from studies complying with Union law on the principles of good laboratory practice (GLP) as laid out in Directive 2004/10/EC on non-clinical safety studies, as applicable at the time of performance of those studies.

60. Therefore non-clinical safety studies must be conducted in a test facility that is part of the national GLP monitoring programme of an European Union (EU) Member State, Organisation for Economic Co-operation and Development (OECD) Member Country or fully adherent to the Mutual Acceptance of Data (MAD), and found in compliance with the principles of GLP.

61. A recommendation paper on principles of GLP for clinical trial applications under the EU clinical trials regulation was issued by the Clinical Trials Coordination Group in 2024 and is available here (Heads of Medicines

Agencies: Clinical Trials Coordination Group (hma.eu). It shares the EU position on the OECD GLP compliance of pivotal non-clinical data submitted to support a clinical trial application and provide transparency on regulatory acceptability for sponsors and test facilities, and other interested parties. This document is fully compliant with Regulation (EU) No 536/2014 of the European Parliament and of the Council of 16 April 2014 on clinical trials on medicinal products for human use, and provides more in-depth clarification and guidance.

62. Sponsors who need to provide the GLP table of pivotal non-clinical studies in a clinical trial application (CTA) must ensure that the information submitted is accurate and up to date. Should the GLP status of a test facility change after the submission of a CTA or authorization of a clinical trial, sponsors are expected to take appropriate action in accordance with the Clinical Trials Regulation (EU) No 536/2014 (Recital 48) Article 53 and notify the relevant receiving authorities via CTIS for all relevant trials. Sponsors must assess any impact on the validity of their data and update their dossier if needed. It is recommended that sponsors establish and maintain a robust communication system and ensure that facilities promptly notify them of any changes in GLP status.

1.20. Question: Which principles of Good Laboratory Practice (GLP) need to be taken into account in relation to Advanced Therapy Medicinal Products (ATMPs)?

63. Answer: It is generally expected that non-clinical safety studies are carried out in conformity with the principles of good laboratory practice (GLP). However, it is recognised that, due to the specific characteristics of ATMPs, it would not always be possible to conduct these studies in conformity with GLP. Exploratory pre-clinical studies, where safety information is obtained alongside with other information (e.g. in dose finding studies), are also not expected to be conducted under GLP.

64. If a pivotal non-clinical safety study has not been conducted in conformity with the GLP principles, a proper justification should be submitted. This justification should also address the potential impact of the non-compliance on the reliability of the safety data.

65. When pivotal non-clinical safety studies are not conducted in compliance with GLP, detailed documentation of study conduct and archiving of data should be ensured. Additionally, the conduct of the study should be in accordance with a prospectively designed study protocol. A summary of deviations from the protocol and their potential impact on the outcome of the study should be included in the relevant study report. The sponsor of

the non-clinical study should consider appointing a person responsible for the oversight of the conduct of the study and the study reports.

66. Applicants who submit pivotal safety studies that are non-GLP compliant in the context of an application for a clinical trial or a marketing authorisation may be asked to submit additional data to justify the reliability of the studies or to permit a site visit to verify the conditions under which the study has been conducted.

1.21. Question: What are the languages requirements for documents that constitute part I of the application dossier?

67. Answer: The language of the application dossier or parts thereof shall be determined by the Member States. The CTR asks the Member States to consider using a commonly understood language in the medical field for documentation that does not go to the subject.

68. Member States have indicated in annex II which documents from the part I (i.e. CTR annex I, sections B to J) can be accepted in English, and what documents are (obligatory) to be submitted in other languages as well. It should be noted that translated documents adhere to the same publication rules as the original document.

1.22. Question: What are the legal warranties for the validity of decisions by tacit approval?

69. Answer: The CTR introduces a set of requirements to the Member States regarding the assessments and decisions on initial clinical trial applications and applications for substantial modifications and for subsequent addition of MSCs (Art 14. 11). It obliges Member States to define the scope of the review by the ethics committee in the part I and II of the assessment report in national law, and to ensure that the timeline and procedures for the ethical review are compatible with the rules in the CTR (Art 4). There is a specific obligation for Member States concerned to submit a part II assessment report to the sponsor within defined timelines.

70. The process for decision on a clinical trial in a given Member State is described in articles 8, 14 and in the relevant articles in Chapter III for substantial modifications. The CTR requires an explicit conclusion on the

(harmonised) part I (article 8(6)). A decision can only be taken when the Reporting Member State has submitted the assessment report part I with a positive conclusion (or positive conclusion with conditions) as referred to in Art 8.2. However, an explicit confirmation of the part I conclusion by a Member Stated concerned (that is not the reporting MS) or a part II conclusion is not a requirement for a Member State decision (article 8(6)) to be positive.

71. A tacit approval of the application is a legally binding decision at Member State level.

72. Importantly, the CTR does not put any obligation on the sponsor to verify that an ethical review or an assessment of the part II documentation has taken place and this is implicitly assumed (in light of article 4). This means that the sponsor can start the clinical trial when its application is authorised by tacit approval in any Member State Concerned.

1.23. Appeal and implementation of change of decision due to an appeal

73. Answer: Changing of a decision from refusal to authorisation in an appeal procedure under the CTR (Art 8.4.) is expected to be exceptional and limited to cases when a MSC refuses an application with a positive part I conclusion by the RMS. Justified grounds for this in addition to the disagreement ground as of Art 8..2. a) to c) are when part II aspects are not complied with or where the ethics committees have issued a negative opinion in accordance with national law. Member States shall provide a procedure for such cases. As for documentary upload to submit an appeal, Member States might have a divergent approach, some will base the appeal exclusively on the documentation in support of the original decision. i.e. on the decision made and thus submission of new documentation would be out of scope. Some other MS would revert the decision on the basis of new documentation. In this case, they can authorise the trial and then apply a corrective measure requesting the sponsor to modify the trial via a substantial modification. In the CM functionality there is the possibility for discussion among MSs and therefore the MS that apply the CM may discuss this with others before applying the CM to "submit SM". With the SM the sponsor can modify the trial docs as applicable. As an additional possibility for documentary upload is by the MS in the case of a positive appeal through a placeholder in CTIS.

74. In accordance with Art 8.5. when part I conclusion is that the trial is not acceptable, that conclusion is the conclusion of all MSCs. In this case, since no appeal is possible, the sponsors might want to resubmit the application once the reasons for the negative conclusion are addressed.

1.24. Question: How are patient facing documents expected to be submitted?

75. Answer: Patient facing documents are documents, other than recruitment material or subject information sheets, presented to clinical trial subjects during the conduct of the clinical trial. These can be questionnaires, patient diary, patient card or patient reported outcomes (PRO/ePRO). Below, the text explains the difference between recruitment material and patient facing documents.

76. Annex I of the CTR describes the content of the clinical trial application part I and part II. Section K60 of annex I refers exclusively to recruitment material (copies of the advertising material, including any printed materials, and audio or visual recordings), and section L61 refers exclusively to all information given to the subjects together with the informed consent form (or, where applicable, to their legally designated representatives) before their decision to participate or abstain from participation in the clinical trial. Recruitment material or subject information sheets are to be submitted in part II, and no other documentation shall be submitted under these sections. If along the clinical trial duration specific concerns arise requiring informing the clinical trial subjects or requiring a re- consent these materials should also be provided within a substantial modification as part II documents.

77. Patient facing documents that are related to primary and secondary endpoints of the clinical trial shall be provided together with the protocol in part I of the clinical trial application, in line with Annex I of the CTR (section D14 and D17l). Patient facing documents that are linked to other endpoints of the clinical trial shall not be submitted.

78. In case a clinical trial will only be conducted in one MS (mononational), patient facing document should only be submitted in Part I in the MS language (or English if acceptable according to Annex II of this document).

79. In case a clinical trial will be conducted in more than one Member State (multinational), patient facing documents should be submitted in English in Part I.

80. Any translation of the patient facing documents will need to be submitted in Part II in line with Annex II of this document (i.e., for several Member States patient facing documents should be provided to the CT subjects in a language understandable for the CT subjects). Sponsors are responsible for ensuring the quality of the texts to be provided to the subjects.

81. There is currently no legal basis in the CTR to request the submission of all patient facing documents in the part II documentation package and/or to require their translation.

APPLICATIONS LIMITED TO PART I (ARTICLE 11 OF REGULATION (EU) NO 536/2014), ADDITIONAL MEMBER STATE (ARTICLE 14 OF REGULATION (EU) NO 536/2014) AND OTHER MEASURES RELATED TO THE APPLICATION PROCEDURE

2.1. Question: Is it possible for a sponsor to submit a whole application (Part I and II) to some Member States concerned (on the basis of article 5) at the same time as an application limited to Part I only (on the basis of article 11) to other Member States concerned?

82. Answer: Yes. Such a mixed application is permitted.

83. It implies that the Member States in which the sponsor submitted the whole application (Part I and Part II) would assess the whole dossier on the basis of articles 5, 6 and 7 of the Regulation (aspects covered by Part I and II), and after the positive decisions by these Member States concerned (MSC) are issued a clinical trial can start in those MSC.

84. The other MSC covered by an application limited to Part I only assess the aspects covered by Part I on the basis of article 5 and 6, together with the MSC who received the full application. Please refer to Q&A 3.6, **NOTA BENE** for further information.

85. The conclusion on Part I with regard to the latter Member States is valid for 2 years and the sponsor can during this period submit the additional part II to the respective MSC (refer to Q2.2 for further details). Only when MSC have issued the positive decision on the full application (Part I and Part II) the sponsor can start the trials in these MSC. If within 2 years the sponsor does not submit Part II in these Member States, the aspects covered by Part I of the clinical trial application shall be deemed to have lapsed with respect to these Member States.

2.2. Question: In cases of applications limited to Part I (article 11) how should a sponsor proceed to submit an application for Part II?

86. Answer: Following the notification of the conclusion on Part I, but only during the subsequent 2 years, a sponsor may submit an application for aspects covered by Part II of the assessment report, declaring that he is not aware of any new substantial scientific information that would change the validation of any item submitted in the application on aspects covered by Part I which were already assessed by the Member States concerned (MSC). The list of the documentation and information required is set out in CTR Annex I and shall be limited to sections K to R of this Annex.

87. However, if at this stage the sponsor becomes aware of the need for a substantial modification of Part I, different scenarios are possible. Please refer to Q&A 3.6 , **NOTA BENE** for further information.

2.3. Question: When is it possible for a sponsor to submit an application for the subsequent addition of a Member State (article 14 of the Clinical Trials Regulation)?

88. Answer: An application for the extension of a clinical trial to another Member State can only be submitted:

(a) after the decision of all MSC which received an initial whole (art 5) or both part I and II in the case of staggered (art 11) application is notified or made by tacit approval under Art 8.6. and at least one of them authorised the trial. This means that in multi-country trials, the last Member State (for staggered applications this is the last MS that received a part II) notifying its decision (or authorised the trial by tacit approval) determines when a subsequent addition of a Member State can be submitted (the "slowest" MS drives the process).

(b) if there is no ongoing assessment of a part I and part I/II SM in any of the MSC meaning that all MSCs issued a decision on a previous SM application or authorised it through tacit approval (the "slowest" MS drives the process).

89. An application for the extension of a clinical trial to another Member State can be submitted if there is an ongoing assessment of a part II SM in any of the other MSC.

2.4. Question: After the receipt of a decision, does the sponsor have the option to appeal against this decision?

90. **Answer**: The Clinical Trials Regulation states that Member States shall provide an appeal procedure in respect of a refusal of both initial applications, addition of a Member State and applications for a substantial modification related to articles 8, 14, 19, 20 and 23. If the MSC disagrees with the positive conclusion by the RMS, the criteria for refusal (and the possibility for an appeal) are described in Art 8.4, 14.10, 19.2, 20.7 and 23.4. The respective national laws apply. When the conclusion of the RMS as regards part I is that the trial is not acceptable, that conclusion can not be appealed and shall be the conclusion of all MSC.

2.5. Question: Where an application for a clinical trial is submitted in more than one Member State, does a sponsor have to await positive decisions from all Member States concerned, before commencing the trial in any of the Member States concerned?

91. **Answer**: No.

92. The sponsor/investigator can commence a clinical trial in the Member State concerned if a positive decision on both Part I and II of the assessment report has been issued by the Member State concerned.

2.6. Question: Chapter X and Annex VI of the Clinical Trials Regulation refer to the content of the labelling of the investigational medicinal product (IMP). Does this mean a mock-up needs to be submitted?

93. Answer: No.

94. Only the text that is labelled on the IMP, as per Chapter X and Annex VI of the Clinical Trials Regulation, should be included in the application dossier. The label text submitted should be in a tabular format following structure given in Annex VI. If any information is omitted from the label because it is made available by other means, for example by use of a centralised information system, reference should be made to the exact section in the protocol where the information can be located or where justification for omission is provided.

2.7. Question: How will a request for information (RFI) during the initial assessment of a clinical trial application, the assessment of an application for substantial modification and/or the assessment of application for subsequent addition of a Member State concerned be managed?

95. **Answer**: Regulation 536/2014 foresees strict timelines for the assessment of initial clinical trial applications as well as for the assessment of applications for substantial modifications and the subsequent addition of a Member State concerned. Sponsors shall submit the requested additional information within the period set by the Member State which shall not exceed 12 days from the receipt of the request of the reporting MS (part I, Art 6.8, Art. 14.6 and Art 18.6) or MS concerned (part II, Art7.3, Art. 14.7 and Art 20.6).

96. Where the sponsor does not provide the additional information within the period set, the application shall be deemed to have lapsed. Depending on the content of the application (Part I and/or Part II), the request for additional information shall be submitted by the Reporting Member State

for part I of the application and by the concerned Member State for part II of the applications.

97. In order to make a timely response by the sponsor feasible and to avoid unnecessary rejections of trial applications, the Reporting Member State (or MSC in case of part II) will formulate requests for information with clear and concise instructions to the sponsor on how to address the considerations stemming from the assessment. In general, it is expected that due to time limitations, only one request for information will be feasible during the assessment period. Therefore, the RFI should focus only on critical issues that need to be addressed by the sponsor as to allow authorization or authorization with conditions and to avoid rejection of the application. In case of an authorization with conditions, it is expected that the conditions in the decision are linked to matters that were raised during the RFI phase. Recommendations to the sponsor by the MSCs can be included with the conclusion of the assessment.

98. As a response to a RFI, the sponsor shall submit a document that includes the responses to all questions. In addition, in those instances, when the response necessitates changes to the clinical trial documentation (e.g. protocol, iMPD, IB), an updated version of the relevant documents including track changes, as well as a clean version of the same documents are expected to be submitted at the same time.

99. Therefore, in order to shorten the assessment and approval timelines and to avoid unnecessary rejections due to time-constraints, the submission of complete and high- quality applications is of particular importance.

2.8. Question: What should be understood by conditions?

100. Answer: Regulation 536/2014 allows that the decision on an initial clinical trial application (Art 8.1), or a substantial amendment (Art. 19.1, 20.5, 23.1) or an addition of a member state concerned (Art 14.3) could be authorised, authorized subject to conditions or be rejected.

101. An authorisation of a clinical trial subject to conditions is restricted to conditions which by their nature cannot be fulfilled at the time of that authorisation.

102. The start of a clinical trial[1] is only possible when the application has been assessed and found to have a positive benefit-risk balance at the time of the authorisation. If not, the application should be rejected. Exceptionally, the sponsor must first fulfil a condition within a defined deadline described in the condition text, e.g, in an approved substantial

[1] CTR article 2.2(25): " 'Start of a clinical trial' means the first act of recruitment of a potential subject for a specific clinical trial, unless defined differently in the protocol".

modification application, which could mean that the start of the trial or the inclusion of the first subject is delayed until the condition is met.

103. Conditions should be clear and related to an issue already identified in the request for information (RFI) submitted during the assessment. Exceptionally, sponsors may agree in the initial application cover letter to a condition without a prior consideration (See CTCG cover letter template for initial application on HMA website under 'Key Documents List ') Usually a single round of RFI is expected with a short time for providing an answer. All critical issues raised in the RFI are expected to be solved in the answer to it, including submission of the corresponding updated documents (e.g. protocol, Investigator`s Brochure or IMPD), when the answer imply changes for them (reference to Q&A on RFI). Therefore, CT applications for authorisation should be complete from the initial submission in order to maximize the chance for approval.

104. When all Member States concerned are in agreement, conditions can be used:

(a) To request additional data not available at the time of the authorisation, e.g. data needed for later trial parts, but not preventing the start of the trial.

(b) To indicate aspects that the sponsor need to fulfil after the authorisation, e.g. submission of minutes of the safety data monitoring board meetings.

105. Conditions are always included in the respective conclusion section of the EU Portal/database (CTIS) by the reporting MS (part I) or MS concerned (part II), as well as in the assessment report. If the trial is authorised with condition(s) then they are always recorded in the decision of the MSC.

106. Data and/or document upload in CTIS by the sponsor to fulfil a condition is not a substantial modification per se. Therefore it can be done either (1) directly, (through the process of a non-SM relevant for the supervision of a trial) or (2) as (part of) a SM application. This allows sponsors to submit the requested data/documents as soon as possible or when it is requested by the regulatory bodies.

107. It is important to note, however, that submitted data/document provided by the sponsor to fulfil a condition can trigger a request for a substantial modification as part of a corrective measure (CM) from any of the MSC. Alternatively, in those cases, when the condition requests that certain information and/or documents are uploaded as a substantial modification, the procedure for the submission of SMs needs to be followed.

2.9. Question: Will the assessment report on part I and II be made public at the time of decision?

108. **Answer**: The Revised CTIS transparency rules (https://www.ema.europa.eu/en/documents/other/revised-ctis-transparency- rules_en.pdf) do not foresee the publication of the assessment report on part I and II, as detailed in their Annex. Conclusions and decision outcomes of an application, however, together with the corresponding dates, are made publicly available at the time of decision for every trial.

109. A specific document was developed to give more insight in the application of the disclosure rules (see Annex I of Guidance document on how to approach the protection of personal data and commercially confidential information while using the Clinical Trials Information System (CTIS)[2].

2.10. Question: How will missing or incomplete documents in an application for the subsequent addition of a Member State (article 14) be addressed ?

110. **Answer:** The Clinical Trials regulation (art. 14(3)) foresees a period of 52 days from the date of submission to the notification of the decision for the subsequent addition of a Member State. There is no validation period foreseen in the Regulation.

111. In case, when documents are missing or incorrect (e.g. because they contain nonsensical information or information in a wrong language making the review impossible), the "Request for additional information" process will be used to request the sponsor to submit the necessary documents and information. This implies that the RMS (in case of missing translations of part I documents (art. 14(6), in line with article 26 of the CTR) or the Member State to be added (for missing part II documents (art 14(8)) asks the sponsor to reply within a very short period of time to be set by the Member State.

112. In these cases, the 52 days can still only be prolonged with maximum 31 days as foreseen in art. 14 (6) and (8).

[2] See Guidance https://accelerating-clinical-trials.europa.eu/system/files/2023-07/guidance-document-how-approach- protection-personal-data-commercially-confidential-information-while_.pdf and Annex I Annex I to the Guidance document on protection of PD and CCI

2.11. Question: Can the decision on part I of a clinical trial application be changed at the moment of the addition of a Member State Concerned (article 14) ?

113. **Answer**: No.

114. The Clinical Trial Regulation is clear in its instruction to avoid re-assessment of the application by all the Member States concerned which were involved in the initial authorisation of the clinical trial at the moment of an article 14 application. Additionally, article 14 does not foresee a mechanism to revise the conclusion on Part I of the assessment report.

115. Nevertheless, art. 14 (5) foresees that the additional Member State concerned (AMS) communicate considerations on the application to the reporting Member State (RMS) and the other Member State Concerned (MSC). A mechanism to request additional information to the sponsor is foreseen, as well as a coordinated review by all MSC and a consolidation by the RMS. At the end, the RMS shall take due account of the considerations and records how the considerations are dealt with.

116. In exceptional cases, the RMS and MSC could therefore decide on additional actions leading to changes of the Part I as a results of those considerations, either through the decision of the AMS or through corrective measures.

2.12. Question: Can a subsequent addition of a Member State Concerned (art. 14) be submitted if another addition of a Member State Concerned (art. 14) is ongoing ?

117. **Answer:** Yes. Parallel Additional Member State Concerned application submissions are possible.

2.13. Question: Can a staggered part II initial application be submitted to a MSC if a subsequent addition of a Member State Concerned (art. 14) is ongoing ?

118. **Answer:** Yes. A staggered part II initial application under Art 11 can be submitted to a MSC when there is an ongoing assessment for the addition of a new MSC under Art 14, if the trial has been authorized in at least one of the MSC, which received the full application.

2.14. Question: How will missing or incomplete documents in the part II application that follows a previously submitted part I application (article 11 – partial submission) be addressed ?

119. **Answer**: The CTR foresees that an application can be limited to Part I of the assessment report. In this case:

a) The application for Part I will follow the process as laid down in art. 5, 6 and 7

b) The subsequent application for Part II will be assessed in accordance with art. 7 and notification of decision will happen in line with art. 8

120. For the subsequent submission of part II, there is no specific validation step described, nor is there a reference to art. 5. When documents are missing or of low quality (e.g. because they contain nonsensical information making any assessment impossible), this should therefore be solved through the Request for Information mechanism described in art. 7 (3). The Member State Concerned will ask the sponsor for the missing documents, within a very short period of time to be set by the Member State.

121. The total timeline can only be prolonged with maximum 31 days as foreseen in art.7(3).

2.15. In case the sponsor of a clinical trial is not the product owner (PO) of the IMP and should not have access to the quality IMPD (IMPD-Q) or associated considerations/RFI in order to protect commercially confidential information, what options do exist for the PO and the sponsor?

NB: As current version of CTIS does not include the functionality for the submission of a document part of the IMPD by a third party other than the sponsor, this Q&A aims to provide guidance on a proposed solution to handle this scenario. In light of the experience gathered with this process, updates will be regularly shared on the EMA website and with sponsors and interested parties.

122. In case of a new clinical trial application in CTIS: if the PO is also the sponsor of another clinical trial with the same IMP ongoing under the Regulation, a cross-reference to the other clinical trial should be made via the function "Associated clinical trials". The cross-reference should be described in the cover letter, and an explanatory document should be uploaded in the IMPD-Q section. If the reference trial is not ongoing under the Regulation, this trial should first be transitioned in accordance with the Commission guidance for transition of clinical trials from CTD to CTR before it can be used for cross-reference.

When the planned trial is not performed in a similar population, or when e.g. the dose or route of administration is different compared with the ongoing trial with the same IMP, the parallel submission described under point 125 can be used for considerations to reach the PO directly.

123. The transition of a trial ("Trial A") with cross-reference to another trial ongoing under the Directive ("Trial B") is acceptable. In this case "Trial B" should be transitioned before the first substantial modification of "Trial A". The reference via the function "Associated trial" should then be made with the first substantial modification of "Trial A".

124. In case the product owner is not a sponsor of a clinical trial in the EEA, two applications in CTIS can be submitted in parallel.

Full cooperation between PO and trial sponsor is required for this approach. Sufficient information regarding the drug substance/product and IMP information should be shared with the sponsor by the PO as a basis for the sponsor's risk assessment and responsibility for the clinical trial. Any changes in the IMPD-Q only

that could impact the safety and/or quality of the IMP should also be shared between the PO and trial sponsor. Contractual agreements should be in place to define bilateral responsibilities and sharing of information.

1. The PO can submit the IMPD-Q to CTIS via an initial application for Part I only ("IMPD-Q- only application"). The "IMPD-Q-only application" must be submitted at the same time as the initial application of the trial for which the IMP is intended ("sponsor trial"). It is recommended that both submissions are not more than 24 hours apart.

The same MSCs should be selected for both applications, and the same RMS should be proposed. RMS and MSCs will then follow the validation and assessment workflow of the "IMPD-Q-only application" according to CTIS timelines. Should there be validation or assessment considerations on quality documentation, an RFI will be issued in the "IMPD-Q-only application" to the PO.

In case of validation considerations in only one of the two applications, a validation consideration will also be raised in the other application to harmonise the timelines of both procedures. It is recommended to address all validation considerations at the same time.

2. The "IMPD-Q-only application" shall only include the following:
    - an explanatory cover letter including reference information for the planned "sponsor trial" and a statement by the product owner to acknowledge the legal, procedural and technical rules of the Regulation, CTIS and this Q&A and a commitment to fully cooperate with the sponsor to fulfil their legal requirements.
    - the IMPD-Q and any supporting documents in accordance with Annex I, section F and G of the Regulation
    - placeholder documents for all mandatory documents part I in CTIS that are not applicable.

In CTIS the following information should be completed:

1) trial title → "IMPD-Q-only application"
2) primary objective → "IMPD-Q-only application"
3) therapeutic area
4) clinical trial phase
5) age range
6) substance (via EV MPD)
7) pharmaceutical form
8) mode of administration
9) maximum dose
10) maximum treatment duration

Information in 3 – 10 should correspond to the planned “sponsor trial”.

3. The PO should complete all necessary roles (sponsor, contact point etc.) with information on their own organisation and personnel. However, the PO does not become a sponsor under the Regulation. The responsibilities are limited to those for the IMPD and the correspondence with the MSCs, as defined in the contractual agreement mentioned above. All other mandatory fields should be completed with “IMPD-Q-only” for text fields or “0” for numeric fields.

4. In the “sponsor trial” reference should be made to the “IMPD-Q-only application” in the cover letter, stating the EU-CT number of the “IMPD-Q-only application”. Instead of the IMPD-Q, a Letter of access document should be uploaded referring to the “IMPD-Q-only application”. Content of the labelling of the IMP in accordance with Annex I, section J of the Regulation should (also) be uploaded in the “sponsor trial”.

5. **Important! After conclusion of Part I assessment the “IMPD-Q-only application” will not be subject to approval and cannot be used for further trials.**
   The “sponsor trial” cannot be approved if the assessment of the “IMPD-Q-only application” results in the Part I Conclusion that the IMPD-Q is not acceptable.
   This also applies if an authorised IMPD-Q in an ongoing trial is not considered adequate for the „sponsor trial“, e.g. when this includes a paediatric population not included earlier or uses a higher dose or different route of administration.

6. The conclusion of the “IMPD-Q-only application” needs to be acceptable or acceptable with conditions for the “sponsor trial” to be approvable. Conditions raised for the “IMPD-Q-only application” need also to be raised for the “sponsor trial“.
7. If a Member State concerned disagrees with the conclusion of the RMS regarding Part I of the assessment report of the “IMPD-Q-only application“, the same disagreement should be issued for the “sponsor trial“.
8. In case the IMPD-Q is to be used for another application of the same sponsor trial (subsequent addition of a member state, substantial modification or a resubmission) then the procedure described above needs to be repeated.

For a substantial modification of the IMPD-Q, the product owner should withdraw the initial IMPD-Q only application and resubmit the substantially modified documentation to all MSC. This allows the existing information in the IMPD-Q-only application to be re-used and to track the

application via the resubmission number (e.g.-00, -01, -02…). This number can then be referenced in the corresponding application in the “sponsor trial”. Note that with each substantial modification application or changes performed as a result of an RFI, the sponsor should highlight the specific changes compared to the earlier submitted version, i.e. in a track changes document and/or in a table listing changes introduced in each version.

A substantial modification of the IMPD-Q documentation requires the parallel submission of an application for substantial modification in the “sponsor trial”. The sponsor should provide a summary description of IMPD changes, while the detailed summary of changes should be part of the IMPD-Q-only application. An updated list from version to version is considered helpful.
In the same way the addition of a subsequent member state to the “sponsor trial” also requires a withdrawal and a resubmission of the “IMPD- Q-only” application to RMS and the additional MS only, in parallel with the AM application in the “sponsor trial”.

9. The “IMPD-Q-only” application is principally envisioned to link a full IMPD for one or more IMPs with a clinical trial in which these IMPs are to be used. The scenario that a substance owner (SO) submits the IMPD-Q for the drug substance (DS) part as “IMPD-Q-only” and the drug product (DP) part is submitted in the sponsor trial is only possible if the applicable product legislation allows this (e.g. where a drug substance master file is allowed). A split application is not allowed for biological medicinal products or ATMPs.
   Full cooperation between PO and trial sponsor is required for this approach. Contractual agreements should be in place to define bilateral responsibilities and sharing of information (see also point 125).

10. When a clinical trial is transitioned from the Directive to the Regulation which includes a reference to an IMPD not available in CTIS (and where the option described under Point 124 is not feasible), then an “IMPD-Q-only” submission should be done together with the first substantial modification of Part I after transition.

SUBSTANTIAL MODIFICATIONS

3.1. How is "substantial modification" defined?

125. See Article 2(2)(13) of CTR.

Modifications to a trial are regarded as ‘substantial’ when they are likely to have a significant impact on:

- the safety or rights of the subjects **and/or**
- the reliability and robustness of the data generated in the clinical trial.

It is, in principle, the responsibility of the sponsor to assess whether a modification is to be regarded as ‘substantial’. This assessment is to be made case-by-case based on the definition as per CTR. In case of doubt, sponsors are encouraged to contact the national contact point.

The sponsor should assess also whether a substantial modification (or the combination of a number of substantial modifications) leads to changes in the clinical trial to an extent that it has to be considered as a completely new clinical trial, which would require an application for a new trial authorisation. For example, unplanned introduction of a new IMP, a change of the main objective, primary end point of the clinical trial in all phases or an unplanned and unjustified addition of a trial arm or placebo group are considered as resulting in a new clinical trial and would therefore require a new trial authorization.

For a non-exhaustive list of examples of substantial and non-substantial modifications please see Annex IV.

It remains the duty and responsibility of the sponsor to correctly decide whether a modification is substantial or non-substantial. Even if a non-substantial modification is submitted together with a substantial modification, only the substantial aspects are assessed. Therefore, the non-substantial modification cannot under any circumstances be considered to have been assessed and approved.

3.2. How are the different changes to ongoing clinical trials classified in the CTR?

126. In compliance with the CTR, a change to a trial data-field or document in CTIS is either:

- a substantial modification (Art 2(2)(13))
- a non-substantial modification relevant to the supervision of the trial (Art 81(9))

- a non-substantial modification (changes outside the scope of substantial modifications and changes irrelevant to the supervision of the trial).

A substantial modification of trial data or document (incl. protocol, IB or IMPD) follows the process as outlined in Chapter III of the CTR.

There is no legal basis in the CTR to submit changes other than through an SM or the Art 81(9) route. Therefore, there is no functionality developed in CTIS to support changes to trial data/documents other than via an SM or as an Art 81(9) non-substantial modification route with notification.

In clinical trials with adaptive design (e.g. complex clinical trials), changes described and specified in the latest authorised protocol can be implemented. This is not the case if the assessing Member State requires an authorisation for these changes via a SM application.

When the route to fulfil a condition is not defined by the relevant MS at the time of setting the condition, it is up to the sponsor to decide on the appropriate route (SM or Art 81(9)) for document or data submission to fulfil a condition.

The CTIS will not be able to differentiate between the different types of content changes in a given document. A good example is the IB: a new version of this document can be uploaded as an SM (e.g. with changes impacting benefit/risk in the trial) or as an art.81(9) (e.g. annual update with no significant changes on subjects safety and/or benefit/risk in the trial). It is up to the sponsor to define the correct path, depending on the nature of the changes. This guidance will facilitate that task. Incorrect use of the non-substantial modification functionality by the sponsor could lead to corrective measures by MSC(s).

The application for a substantial modification shall describe the type of change with a justification (e.g. inclusion of a new exclusion criteria in the protocol due to a potential liver toxicity described in section x of the IB). See Annex II of the CTR and the template for Substantial Modification under 'Key Documents List' at this link Heads of Medicines Agencies: Clinical Trials Coordination Group (CTCG)

3.3. What can be the result of the assessment of an SM application?

127. An application for a substantial modification can contain multiple changes concerning Part I, Part II or both and will result in a single decision for that application in each MSC (Clinical Trials Regulation Art. 19.1). According to the decision, the substantial modification can be: authorised, authorised subject to conditions or refused.

128. MSC (Part II) and RMS (Part I) can recommend the removal of certain changes or elements from the application during the RFI phase (see Q&A 2.7) of the

assessment process in order to support authorisation of the submitted application. When the sponsor follows these recommendations, the cover letter could be updated to reflect these modifications to the original application (Annex II.B.3). Cover letter should be updated as part of the RFI response, but not updated under the Form section.

129. It is possible to authorise an SM with conditions clarifying all required changes. Conditions need to be linked to matters that have been raised during RFIs and listed in the conclusion section of the assessment report (Art 6.3) and in the decision of the MSC (Art 8.3)

130. Conditions are set to identify aspects that cannot be fulfilled at the time of authorisation (Art 19.1, Q&A 2.8.) Setting a condition is only possible if the overall risk/benefit balance of the trial remains positive (please see CTAG meeting minutes of November 2025 available at this link Register of Commission expert groups and other similar entities).

3.4. What are the sponsor's responsibilities regarding changes to a clinical trial, which are not substantial modifications (SM), but are relevant for the supervision of the trial (Art. 81(9))?

131. Information on any changes to a clinical trial, which are not SMs but are, nevertheless, relevant for the supervision of the clinical trials by the Member States concerned, shall be permanently updated in the EU database by the sponsor, in line with article 81(9) of Regulation (EC) No 536/2014. For a non-exhaustive list of non-substantial modifications please consult Annex IV of this document.

132. Art 81(9) allows to update certain specified information in the CTIS without the need for an SM application, when this information is necessary for oversight but does not have a substantial impact on the safety and/or rights of subjects and/or data reliability and/or robustness. In CTIS, the non-substantial modification (NSM) application is strictly intended for non-substantial modifications under article 81(9), and such applications should not contain other non-substantial modifications or any substantial modifications.

133. Art 81(9) changes can be submitted only if the change does not trigger additional changes that would require an SM application.

134. The combination of different Art 81(9) changes can cumulate into a change that needs to be submitted as an SM.

135. Specific examples for Art 81.9 changes (e.g. update of sponsor's or CRO contact details) are described in Annex IV of this guidance.

Importantly, this route can be used to update information to fulfil a condition, depending on the instructions of the RMS (Part I conditions) or the MSC (Part II conditions).

3.5. What are the sponsor's responsibilities regarding changes to a clinical trial, which are non-substantial modifications (NSM)?

136. A non-substantial modification (NSM) is a modification that would not have substantial impact on the safety and rights of the subjects and/or the reliability and robustness of the data, and the information is not necessary for oversight over the clinical trial.

137. Correction of typos and other administrative changes with no impact on the content and meaning of the information are always expected to be updated as NSM.

138. NSM should not be notified as such. NSMs need to be listed and identified as NSMs in the cover letter of the next SM application that contains substantial changes to the same Part of the dossier. NSMs are not expected to be described in detail in the subsequent SM cover letter.

These changes should be added to the application dossier at the time submitting the next SM application, but only if the scope of the non-substantial changes matches with the scope of the substantial changes, meaning:

- Part I non-substantial changes should only be included in an application that contains substantial changes to Part I;
- Part II non-substantial changes should only be included in an application that contains substantial changes to Part II;
- Part I & II non-substantial changes should only be included in an application that contains substantial changes to Part I and Part II.

139. In case the SM application is rejected and the documents with NSM are reverted, the NSMs should be resubmitted with the next SM application. In the meantime, NSMs will have to be recorded in the Trial Master File and made available on request for inspection purposes as appropriate.

Importantly, sponsors are encouraged **not** to submit non-substantial changes during the RFI phase of any ongoing assessment (initial, substantial modification, addition of a new Member State concerned), unless they are required as part of the RFI response.

140. For standalone NSM applications (article 81.9) even though these appear 'authorised' in CTIS they do not undergo assessment. Therefore, the non-substantial modification cannot under any circumstances be considered to have been assessed and approved.

3.6. When can a sponsor submit a substantial modification application concerning Part I only?

141. Article 2(13) defines 'substantial modification', and Article 16 clarifies that sponsors shall submit an SM application dossier to the Member States concerned through CTIS in order to obtain an authorisation.

To ensure the stability of Part I, the SM application with Part I in scope should be submitted to all Member States concerned after the RMS issued the Part I assessment report and its conclusion (See CTCG Guidance for Sponsors on HMA website under 'Kye Documents List'). There cannot be other SM applications ongoing with Part I in scope.

Importantly, if there is an SM with Part II in scope that affects Part I (e.g., informed consent), sponsors should submit SM Part I and II.

142. A Part I SM cannot be submitted if there is an ongoing assessment in a different Member State concerned for an additional Member State concerned (Article 14), since this procedure also involves changes in the Part I dossier (translations).

143. The SM can be implemented only after the SM application has been authorised in that MSC.

144. Sponsors are encouraged to submit high quality full applications for a fast and smooth assessment of the application dossiers.

NOTA BENE: For a procedure allowing sponsors to submit a substantial modification application concerning Part I in Member States where only Part I was submitted, please refer to the CTAG meeting minutes of November 2025 available at this link Register of Commission expert groups and other similar entities. See also the *CTCG Guidance for Sponsors on Article 11 workaround* under 'Key Documents List' at this link Heads of Medicines Agencies: Clinical Trials Coordination Group (CTCG)

3.7. When can a sponsor submit a substantial modification application concerning Part II only?

145. An SM application with Part II in scope can be submitted in a Member State concerned if all the following apply:

- this MSC has fully authorised the trial (regardless of whether it was through a full (art 5), staggered (art 11) or additional MS (art 14) application)
- there is no other ongoing SM assessment (Part I, I+II or II) in this MSC.

146. Within the same Member State concerned, sponsors can submit a single SM application with Part II in scope to amend different documents/information.

147. To ensure the stability of Part II, the SM application with Part II in scope shall be submitted to the Member States concerned if there are no other SM applications ongoing in the same Member States concerned with Part II in scope.

148. Across different Member States concerned, sponsors can submit different SM applications with Part II in scope on the same document/information. Assessments on the same or different document/information can run in parallel.

149. A Part II only SM can be submitted if there is an ongoing assessment in a different Member State concerned for an additional Member State concerned (Article 14).

150. The same rules apply for non-SM changes relevant to the supervision of the trial (Art 81(9)).

3.8. When can a sponsor submit a substantial modification application concerning Part I and II?

151. An SM application with Part I and II (to all or some MSCs) in scope can be submitted to MSCs if all the following apply:

a) the decision of at least one MSC receiving the initial trial application is authorised (or made by tacit approval under Art 8.6).

b) there is no ongoing application for an additional MSC (Article 14). In case of Art 14, a Part I or Part I+II SM application cannot be submitted until there is a decision notified on the Art 14 application (as it might have Part I implications).

c) there is no other ongoing SM assessment (Part I, I+II or II) in any of the MSCs; Also note that any Part I and Part II NSMs for supervision (Art 81.9) must be updated in CTIS before a Part I or Part I+II SM application can be submitted.

NOTA BENE: For an SM application with Part I and Part II in scope to an MSC initially intended to receive Part I only, please see CTAG meeting minutes of November 2025 at this LINK and the CTCG Guidance for Sponsors on the HMA website under 'Key Documents List'.

3.9. How should a sponsor proceed in case a substantial modification is required while the assessment of another application for the same clinical trial is ongoing (under evaluation)?

152. In case the sponsor realises that an SM application may be needed while an assessment on the same Part in scope is still on-going, the following options are available, depending on the urgency of that need:

a) wait for the on-going assessment to end before submitting the SM
b) OR withdraw the on-going application and introduce the additional SM

153. For changes limited to Part II of the application, parallel submission of Part II SM to other MSC are allowed.

154. If safety measures are required while any assessment is still ongoing, the sponsor should take the appropriate measure and notify the MSC. An SM can then be submitted once the ongoing SM is finalised.

3.10 How should a sponsor proceed when a substantial modification is related to a document common to various clinical trials of the same sponsor and same IMP?

155. In cases of substantial modifications (SM) related to the investigational medicinal product dossier (IMPD) (Quality, safety or efficacy), to the investigator's brochure (IB), reference safety information or any other common document used in multiple clinical trials it is recommended to submit the same substantial modification to multiple trials when these trials use the same documents. In these cases, maintaining the harmonisation of the non-trial specific sections(s) of the IMPD across trials would be advantageous. A robust procedure to support this would have a positive impact on the capacity of sponsors and regulatory bodies to maintain product level documents and information on a portfolio of trials up-to-date in the most efficient manner, and improve overall consistency of product information in EU and at global level (in case of multi- country trials with third countries).

156. Parallel submission of the same SM to enable changes to these documents across trials of the same sponsor and the same IMP is accepted and encouraged (Annex II(A)(1)). In this case, CTIS functionalities are developed to allow the sponsor to submit one single SM application covering multiple trials, provided that all the SM changes introduced are applicable for all the trials where the SM has been submitted. (e.g. identical changes for all the data and documents included in the SM application). Different language variations of the same change to the same document are acceptable.

157. The sponsor will be able to submit the multi-trial SM only for those trials that have already been authorized (or authorized with conditions)(see rules above) , and do not have outstanding parallel assessment or pending notification of a decision in CTIS.

158. In accordance with Article 25 and Annex II of the CTR, the cover letter (submission of several language versions with identical content is acceptable) in the application dossier for the SM shall contain a list of all clinical trials to which

the application for SM relates, with the EU trial numbers and respective SM code numbers (to be attributed by the sponsor) of each of those clinical trials.

159. The assessment of the submitted multi-trial SM application will be performed and recorded in the EU database independently for each trial by the relevant Member States Concerned and reporting Member States. This means that it might be possible for the sponsor to receive several identical RFIs for several trials. Each trial will show its own record in CTIS for validation conclusion, assessment part I and part II conclusions, as applicable, and decision of the SM.

160. Additionally the sponsor may submit in an initial application the same IMPD and IB (or other relevant documents) that was previously submitted in an application for an on-going trial or for an application that is being/has been evaluated (e.g. an on-going/completed assessment of an initial application, a SM or an additional member state application). In such an event, it is recommended that reference to these applications is made in the cover letter and the EU trial number of reference should be recorded as structured data in the initial application.

161. When submitting an initial CTA, reference can only be made to documents already submitted with another clinical trial within the CTR framework in CTIS to all concerned Member States. It is possible to reference to a prior submitted transition clinical trial if the documents referred to have been uploaded in full.

162. There are specific considerations in case of SM to the Investigational Medicinal Product Dossier (IMPD). The IMPD shall give information on the quality of any investigational medicinal product, the manufacture and control of the investigational medicinal product, and data from non-clinical studies and from its clinical use.

The content of the IMPD is described in Annex I of the CTR, and contains:

- Quality data
- Non-clinical pharmacology and toxicology data
- Data from previous clinical trials and human experience Overall risk and benefit assessment

163. Most of that information is product/substance –specific, with some variance in its extent and details based on the study phase and design (dosages used, blinding, comparator and placebo strategies varying per trial). Part of that information may also be given in the Investigators Brochure (IB).

164. The section with overall risk and benefit assessment is trial-specific. It is possible to cross-refer to the relevant sections in the protocol in the overall risk and benefit assessment section of the IMPD.

165. In addition, whenever it is possible, it is encouraged to cross-refer to the IB for the reference safety information and the summaries of pre-clinical and clinical parts of the IMPD in accordance with Annex I (G)(51).

166. Changes to the IMPD, including changes to the quality section, with an impact on subjects safety, benefit/risk to the trial or on data robustness shall be submitted as an SM and assessed according to Chapter III of the CTR.

167. There are two possibilities available to submit SM to the IMPD in multiple trials:

- Option 1: Multi-trial substantial modifications
- Option 2: In cases of trials using the same IMP (active substance, content/concentration, formulation, route) it is accepted to submit the same substantial modification to multiple trials when these trials use the same IMP and IMPD in accordance to above detailed process.

168. Option 2: Reference to a related trials mother trial

If an IMPD has been approved in an MSC for any CT and has not been modified, it is accepted that the IMPD document itself is not submitted for each and every trial with the same IMP in that MSC. The CTR allows this for the different sections of the IMPD and the CTIS enables users to provide a reference for common IMPD-Q or IMPD-efficacy and IMPD-safety.

Instead of submitting a complete IMPD in the following related trials, a reference to the first trial containing the approved IMPD could be acceptable under certain conditions. Most importantly, every MSC in every daughter trial has to be a MSC also in the reference ("mother") trial as well. This condition ensures that each MSC in each trial sharing the same IMPD, has the possibility to assess and issue a decision of substantial modifications to the shared IMPD. If this condition is not met, the addition of the reference to the mother trial will be rejected by an MSC for the daughter trial.

Importantly, the addition of a MSC to a daughter trial will not be possible when the additional MSC is not an MSC in the reference trial.

169. Setting up and maintaining a reference from a daughter trial to a mother trial is a manual operation – CTIS does not foresee automatic checks on the conditions.

A reference requires information at two levels in the application dossier of the daughter trial:

- A link to the "mother" trial needs to be established in the section "Associated clinical trials". In the case of a different sponsor, a delegation letter needs to be introduced.
- In the IMPD section (quality and/or safety and efficacy), a justification for no IMPD upload needs to be filled in (being a reference to the approved IMPD in another trial).

170. The referencing is a unilateral and non-permanent process in CTIS – there are no automated checks foreseen at the level of the mother trial. There are no requirements at the level of the "mother" trial

171. The RMS and the MSC need to verify the correctness of the referencing during the validation based on the information provided with the cover letter (preferably in the form of a clear 'association matrix') and check whether the conditions for the referencing (i.e. the MSC is also a MSC in the mother trial) are met. In the case of multinational trials, all MSC need to be MSC in the mother trial as well.

172. Having a single IMPD shared in a portfolio of trials with the same sponsor and same IMPD also means that if a change through an SM is approved to the IMPD in the reference ("mother") trial, the updated IMPD is valid also in all daughter

trials referring to the original one. Important to note is that it is not required to submit a SM application to the daughter trial(s) as long as the conditions for referencing remain met.

173. The same principle applies for updates to information in CTIS, which are not substantial modifications but are relevant for the supervision of the clinical trial and introduced through the art. 81.9 route. It is important to note that in this scenario, only the mother trial needs to have no ongoing activities, where in the multi-trial SM scenario, all trials need to be "open" for the submission of the SM application.

174. Additional conditions would be that the sponsor submits a list with the cover letter of the SM application an association matrix, where all trials using the IMPD in the reference ("mother") trial are listed and identified as daughter trials to this 'mother' trial. In case there are several IMPs in the mother trial with each its own IMPD, it needs to be specified which daughter trial is referring to IMP. Vice versa, when a sponsor associates a trial as a daughter trial to one or in case of several IMPs to several mother trials, a clear and comprehensive 'association matrix' needs to be submitted with the cover letter each time.

175. The association and the nature of association between the different trials need to be clear at all time. If the sponsor does not comply/misuse these rules, MSC can trigger corrective actions for requiring the submission of a separate per CT SM with the IMPD submitted to the daughter trials and the removal of the reference.

176. Once a CT is ongoing, defining a new mother CT in order to cross-refer to its last authorised version of the IMPD would require a SM in the daughter CT.

<u>End of the reference trial</u>

When the end of the reference trial is foreseen, sponsors of the daughter trials may choose to continue using a shared IMPD for several trials. In these cases, the IMPD shall be migrated from the mother trial to a select daughter trial or to a new clinical trial. In this case, the IMPD shall be submitted to the new mother trial via an SM or in the case of a new trial with the initial application, and once this is approved, the reference to the IMPD in the daughter trials shall be updated via an SM to the IMPD section in CTIS to contain correct information about the new mother trial.

This could be done before or, preferably, after the end of the original mother trial as long as the IMPD remains unchanged. On the other hand, changes to the IMPD via SM will require that the document is approved in a mother trial, which is ongoing. This means that by the time of the first SM to the IMPD, all daughter trials shall have the correct reference recorded in their IMPD section.

177. In order to ensure continuity, good communication between sponsors is essential when the daughter trials and the mother trial is conducted by different sponsors.

3.11. Is the addition of an additional Member State considered a substantial modification?

178. No. The subsequent addition of another Member State concerned to extend an authorised clinical trial requires the submission of an application dossier in accordance with Article 14. An application dossier in this regard may be submitted only after the notification date of the initial authorisation decision (see also Q2.3) and not when there is an ongoing SM Part I and II or Part I only.

3.12. Is the deletion of a Member State considered a substantial modification?

179. No. The deletion of a Member State concerned is not considered a SM.

180. Various scenarios are possible to deal with such cases:

Scenario 1: The sponsor decides to withdraw an application for a clinical trial in an MSC.

This may happen at any time until the decision is made, providing reasons. However, in cases of withdrawal of an application before the reporting date, the withdrawal will apply to the entire application in all Member States concerned. After the reporting date, but before the decision is taken by a particular Member State concerned, the sponsor has the option to withdraw the application in one, several or all Member States concerned.

Scenario 2: The sponsor decides to withdraw an application in case of mixed applications. Scenario 1 above applies also in this case. If an MSC received only Part I, the application could be withdrawn at any point after the reporting date even if the clinical trial is already authorised in one or more of the other Member States concerned that received a full application.

Scenario 3: The sponsor decides to terminate early an ongoing clinical trial in one of the Member States concerned (i.e. after authorisation or authorisation subject to condition(s) in that Member State concerned). The sponsor should notify the Member State concerned of the early termination.

181. In case of early termination due to reasons of the subjects' safety (Article 38(1)) the notification shall be made without undue delay but not later than 15 days

from the date of the early termination including the reasons for such actions and specify follow- up measures. Early termination in such cases in principle would apply to all Member States concerned.

182. In case of early termination for reasons not affecting the benefit-risk balance, the CTR does not set up a timeline for such notification but requires that the sponsor informs each Member State concerned of the reasons for such action and, where appropriate, on the follow up measures for the subjects (Article 37(7)). In this latter case, sponsors are recommended to inform all Member States concerned about the early termination without undue delay.

183. In all scenarios described above, while the clinical trial is ongoing in other Member States concerned, scientifically, the sponsor should assess the potential impact on the overall recruitment/sample size of the clinical trial and submit a substantial modification to the other Member States concerned if necessary (e.g. to add more sites in MSC).

3.13. Is a change of the Principal Investigator considered a substantial modification?

184. Yes. See article 15 of the CTR.

185. The change of the Principal Investigator in a clinical trial site may only be implemented if it has been approved by the MSC.

186. If an SM of the Principal Investigator cannot be submitted to CTIS for any reason, it is recommended to wait for this on-going assessment to end before submitting this SM part II in the applicable MSC, provided that the approved Principal Investigator retains the responsibility for the conduct of the clinical trial at the applicable clinical trial site and has delegated significant trial-related duties to appropriately qualified persons of the study team.

187. The SM Principal Investigator should be submitted without undue delay after the previous SM Part II has been concluded.

188. In exceptional cases, due to unforeseen urgent circumstances (e.g. Principal Investigator has become seriously ill, incapacitated or has died), the Principal Investigator can be replaced without awaiting prior authorization. The sponsor must ensure that significant trial-related duties can be performed by appropriately qualified persons of the study team.

189. The sponsor should submit the SM Part II without undue delay and clearly indicate in the cover letter that this is an urgent SM together with appropriate justification.

190. In case the change of the Principal Investigator results in a breach that could significantly impact the safety of a trial subject or the reliability and robustness of the data generated in the clinical trial, this breach shall be notified to the MSC

as a ‘serious breach’ without undue delay but not later than seven days after the sponsor becomes aware of that breach (Article 52).

3.14. Can a substantial modification of aspects covered by Parts I and II of the assessment report be partially authorised (e.g. only the Part II)?

191. No. The decision on an SM application with Part I and II in scope applies to both Parts.

192. This implies that when a Member State Concerned refuses to authorise such an SM either because it disagrees with the conclusion of the Reporting Member State (Part I), or finds that the aspects covered by Part II of the assessment report are not complied with or has an ethics committee issue a negative opinion and therefore, this leads to a refusal of the whole application (Part I and Part II).

193. In the specific case where a sponsor would not respond in a timely manner to a Request for Information on Part II aspects, the lapsing of the application causes the whole SM application (I & II) to lapse for that Member State. This lapsing does not prevent the authorization of the part I SM in the other MSC.

3.15. Can there be different decision relating to a Part I SM in different Member States concerned?

194. Yes. See in particular Articles 18 and 19 of the CTR.

195. The CTR foresees an assessment of a substantial modification of an aspect covered by Part I.

196. In case of multinational trials, all Member States jointly review the application. The RMS will assess the SM and will submit a conclusion at the end of this assessment.

197. Nevertheless, each Member State Concerned takes an individual decision and can disagree with a positive conclusion by the RMS only on the grounds as described in Article 8(2) of the CTR. Such divergencies relating to Part I can also arise when Member States are added after the initial application was approved (Article 14), since these new Member States did not participate in the initial Part I review. This might lead to the situation that for a given clinical trial, several versions of the Part I documents may exist. The CTIS reflects these versions and contains both an overview of the document versions authorised at trial level and at Member State level.

198. In case of disagreement from one or several Member States to a positive conclusion of a part I substantial modification, the Sponsor can submit

subsequent part I substantial modifications. The basis for these SM will be the authorised versions of the part I document.

199. Sponsors are encouraged to carefully review the considerations and justification of the MSC that disagreed on the previous part I SM in order to have one common version of the part I dossier across the MSCs.

200. As a general rule, sponsors are encouraged to include in the initial application all EU/EEA Member States where the trial is planned to be conducted to promote alignment of the Part I dossier.

3.16. How should the change of the source country of an IMP or AxMP be implemented?

201. There are several different scenarios depending on the different sourcing strategies (locally by investigator site or centrally by the sponsor), the authorization status of the medicinal product or the submission of the Investigational Medicinal Product (IMP) /Auxiliary medicinal product (AxMP) in the application (e.g. by brand name or by substance code, ATC category). These are described in detail in Annex V.

WITHDRAWALS

4.1 Question: In which circumstances can a sponsor withdraw an application for a clinical trial?

202. **Answer:** The sponsor has the option to withdraw an application for a clinical trial at any time until the decision is made.

203. However, in cases of withdrawal of an application before the reporting date (article 6(6) of the Clinical Trials Regulation), the withdrawal will apply to the entire application in all Member States concerned.

204. After the reporting date, but before the decision is taken by a particular Member State concerned, the sponsor has the option to withdraw the application in one, two or all Member States concerned

205. In cases when the procedure of article 11 is applied and Part II is submitted later to one or more Member States concerned (within the 2-year period), the application for Part II can be withdrawn from one or more Member Sates concerned. The sponsor can also withdraw the entire application (also the previously submitted Part I) if he so chooses, until the decision is made.

206. Once the decision regarding an application is taken, a sponsor no longer has the possibility to withdraw this application. If a CT does not start and the sponsor decides not to carry out the clinical trial in a Member State concerned, the application will expire after 2 years from the notification date of the authorisation. Otherwise, once the CT starts, it may be a case of early termination if it does not proceed. (Please refer to chapter 10 for more information).

4.2. Question: Can an application be re-submitted?

207. **Answer:** Yes. A re-submission entails that the application was withdrawn in all MSCs, refused in all MSCs, or the application lapsed in **all** MSCs after the authorisation.

208. If the application would have been withdrawn or lapsed in one or several MSC, but not in all, re-submission is not possible . The MSC(s) that were withdrawn can be added through an additional

MSC application in line with article 14 of the CTR.

209. In the specific case of a refusal at the level of a given MSC, the sponsor can later decide to include the MSC again (e.g. when the aspects leading to the refusal have been addressed via part I SM) via the additional MSC application.

4.3. Question: In which circumstances can a sponsor withdraw an application for a substantial modification of a clinical trial?

210. Answer: Withdrawal of an application for a substantial modification of the clinical trial is possible:

a) In the case of a substantial modification of Part 1 or Part I and Part II, the withdrawal applies to all Member States concerned and can take place at any point during the assessment until the decision is issued.

b) In case of a substantial modification of Part II only, an application can be withdrawn from one or more Member States concerned, at any point during the assessment until the decision is issued.

211. These possibilities for withdrawal allow the sponsor to withdraw an application in cases such as an urgent safety measure or if other substantial modifications are required. Therefore a sponsor may choose not to wait for the end of the assessment of an ongoing application for a substantial modification and withdraw the application to submit a new one, with the updated substantial modification.

SPONSOR/LEGAL REPRESENTATIVE; INVESTIGATOR

5.1. Question: How is “sponsor” defined?

212. Answer: “Sponsor” is defined in article 2(2)(14) of The Clinical Trials Regulation as “an individual, company, institution or organization which takes responsibility for the initiation, management and for setting up the financing of a clinical trial.”

213. Thus, the sponsor can be an individual, a company, an institution or an organisation. Article 71 states that a trial may have one or more sponsors. A loose, informal networks of researchers and research institutions may jointly conduct a clinical trial as co-sponsors.

214. Article 71 also clarifies that sponsor and investigator may be the same person. The sponsor does not need to be located in an EU Member State. (See also Q5.6)

5.2. Question: How responsibilities are shared in case of co- sponsorship?

215. Answer: In case a clinical trial has more than one sponsor, all co-sponsors shall in principle have the responsibilities of the sponsor (article 72 of Regulation (EU) No 536/2014). This implies that all of them are jointly responsible (e.g. also for the safety issues) and a Member State concerned may expect the execution of a sponsor's obligations from any of the co-sponsors.

216. However, the co-sponsors shall jointly determine, in a written contract which sponsor will be responsible for the following tasks:

a) compliance with a sponsor's obligations in the authorisation procedure (including any substantial modification and the procedure for the addition of a Member State concerned);
b) a contact point for receiving questions from subjects, investigators or any Member State concerned regarding the clinical trial and for replying to them;

c) implementing corrective measures imposed by any of the Member states concerned.

217. Each task mentioned above can be attributed to one single sponsor. Co- sponsors cannot have a joint responsibility for any of the tasks mentioned above. This means that the responsibility for compliance with each of the above tasks will lie with one single sponsor and cannot be shared by several sponsors. This does not preclude however, that if desired, the sponsor can delegate certain tasks to third parties (see also Q&A 5.4).

218. The co-sponsors may split up all remaining responsibilities by contractual agreement. If they do not do this, the principle of joint responsibility applies.

219. However, in each trial, the sponsor bearing the overall responsibility to ensure compliance with the obligations in the authorization procedure remains responsible to fulfil this role and therefore this sponsor needs to have full access to the documentation.

220. It is assumed that co-sponsors have agreed through a contractual agreement on the exchange of information necessary to allow the responsible sponsor to take informed decision for compliance on behalf of all sponsors during the authorization procedure.

5.3. Question: Is the person financing a clinical trial always considered as "sponsor" in the sense of article 2(2)(14) of Regulation (EU) No 536/2014?

221. **Answer:** A sponsor is defined in article 2(2)(14) of the Clinical Trials Regulation as "an individual, company, institution or organization which takes responsibility of the initiation, for the management and for setting up the financing of a clinical trial".

222. Every clinical trial has to have a sponsor.

223. In light of the definition, the sponsor is the person who presents himself as the person taking the responsibility for the clinical trial. The sponsor would as well be responsible for setting up financial arrangements allowing the conduct of clinical trial (this does not however mean necessarily by funding it him/herself). The person funding a clinical trial *may* however be the sponsor.

5.4. Question: Can the sponsor delegate tasks/functions?

224. **Answer:** The sponsor may delegate his trial-related tasks/functions to an individual, company, institution or organization. ([21]) The Clinical Trials Regulation does not restrict the scope of such delegation and explicitly states that the delegation may concern even *all* sponsor tasks.

225. In cases where there are tasks/functions delegated the sponsor remains ultimately responsible for ensuring that the conduct of the trials and the final data generated by those trials comply with the requirements of Regulation (EU) 536/201 as well as with those of Directive 2001/83/EC in the case of a marketing authorisation application. This applies in particular to ensuring the safety of the subjects and the reliability and robustness of the data generated in the clinical trial.

226. Any trial-related tasks/functions that are delegated to a third party should be specified in a written contract between the sponsor and the third party and when relevant made clear to the investigator (eg. responsibilities regarding safety reporting).

5.5. Question: Does Regulation (EU) No 536/2014 establish that the sponsor, investigator, any person to whom sponsor has delegated task or his legal representative according to article 74 are liable under civil and criminal law?

227. Answer: No.

228. The Clinical Trials Regulation, in referring to the "responsibility for the initiation, management and for setting up the financing of a clinical trial" (article 2(2)(14) of Clinical Trials Regulation refers to the responsibility for compliance with the Regulation.

229. Responsibility in terms of civil law (i.e. liability, for example compensation for damages occurred to a patient), or criminal law (i.e. punishment, for example criminal sanction of a bodily injury caused by negligence), is not governed by the Clinical Trials Regulation, cf. article 75. In this respect, the applicable laws of the Member States apply (see article 95 of the Regulation). Neglecting

the duties or responsibilities laid out in this regulation and causing damages or bodily injury to a person can and would result in a corresponding civil and/or criminal liability according to the legal system of the respective Member State.

230. This also holds for cases where the sponsor has a legal representative in an EU Member State or EEA State. While the existence of a legal representative within the EU/EEA might be supportive to ensure effective sanctioning under national civil or criminal law, the rules for civil and criminal liability remain governed by the national laws of the Member States.

5.6. Question: Can a sponsor established in a third country open a subsidiary or branch in a Member State in order to comply with the requirement of Regulation (EU) No 536/2014 that the sponsor or a legal representative of the sponsor must be established in the EU?

331. **Answer**: Yes.

332. Article 74 of the Clinical Trials Regulation requires that the sponsor or, in principle, a legal representative of the sponsor is established in the EU.

333. This does not exclude the possibility that this establishment is a branch or subsidiary of a legal person having its principal seat outside the EU. This establishment could be the sponsor or act as legal representative of the sponsor established outside the EU.

5.7. Question: What are the requirements for the legal representative of a non EEA-sponsor in view of article 74 of Regulation (EU) No 536/2014?

334. **Answer:** If the sponsor is not established in the EU a legal

representative of the sponsor has to be established in the EU.[1]

335. Only one legal representative can act on behalf of one sponsor in one clinical trial.

336. If the sponsor is the same for several different trials, it is acceptable (but not obligatory) to have one central legal representative in EU for all non-EU sponsored trials, as long as the responsibilities provided for by the regulation can be effectively performed.

337. It is also acceptable to use an established company as a legal representative.

338. The applicant for the application to the Member State (competent authority and the Ethics Committee) might be different from the legal representative.

339. According to article 74(1) of the Clinical Trials Regulation the legal representative shall ensure compliance with the sponsor's obligations pursuant to the Regulation. This implies that the legal representative has the same responsibilities and liabilities as the sponsor and should act on behalf of the sponsor based on a contractual agreement. It also implies that the Member States may address the legal representative with any request related to the conduct of a clinical trial.

340. In order to enable the legal representative to ensure compliance with the sponsor's obligations under the Clinical Trials Regulation it is recommended that the contract obliges the sponsor to provide the legal representative with all necessary information and the legal representative to immediately notify the sponsor in case (s)he becomes aware of any incompliance with the Regulation.

341. Member States may choose not to require the establishment of a legal representative, provided that they ensure that the sponsor establishes at least a contact person on their territory in respect of that clinical trial.

[1] Article 74(1) of Regulation (EU) No 536/2014.

5.8. Question: What should be included in the protocol synopsis described in Annex I, D.24 ?

342. **Answer**: Sponsors should include the information below in the protocol synopsis (maximum two pages) to be submitted with the clinical trial application according to Annex I D24. National language requirements for the preparation of the protocol synopsis are in Annex II. Sponsors should consider to make the synopsis understandable to a layperson.

343. The protocol synopsis can be part of the protocol or a separate document (e.g. when it is submitted in different language versions), in the latter case, it should always be submitted to CTIS together with the protocol.

344. Content of the protocol synopsis:

a) EU trial number and full trial title

b) **Rationale**: Specify background and hypothesis of the trial.

c) **Objective**: Specify the main and secondary objectives of the trial.

d) **Main trial endpoints**: Describe the main trial endpoints and when they are assessed, e.g. the main trial endpoint is the percent change in the number of events from baseline to a specified time or the total number of adverse reactions at a particular time after baseline.

e) **Secondary trial endpoints**: Describe the secondary trial endpoints, and when they are assessed e.g. number of adverse events until 30 days post end of treatment.

f) **Trial design**: Describe the design and the expected duration of the trial for the individual subjects, e.g. double-blind placebo controlled clinical trial where subjects are participating for X weeks.

g) **Trial population**: Describe the trial population, indicating the main inclusion criteria including age and disease/healthy volunteer and the main exclusion criteria to protect the subject,
e.g. patients with moderate asthma 18-55 years with normal kidney and liver function and without gastrointestinal ulcer or risk factors for a cardiac arrhythmia; healthy volunteers 18-60 years not exposed to X-Ray examinations during the last 12 months.

h) **Interventions**: Describe interventions and treatment duration, also including background treatment if any, e.g. one group receives a 10 mg tablet of product X twice daily for Z weeks while also receiving product Y as background

treatment and the other group receives a placebo tablet twice daily as well as product Y.

Also describe trial-related diagnostic and monitoring procedures used.

i) **Ethical considerations relating to the clinical trial including the expected benefit to the individual subject or group of patients represented by the trial subjects as well as the nature and extent of burden and risks.**

A benefit-risk analysis should be done for the trial-specific treatments and interventions, clearly explaining if the trial involves an expected individual benefit (e.g. as required in emergency situations) or a group benefit. When a trial is placebo-controlled, a brief justification should be given. If a non-therapeutic trial is carried out in vulnerable groups, e.g. in minors, incapacitated persons, pregnant or breastfeeding women, their inclusion has to be justified and it should be explained why the risks and burden are considered minimal and why the trial can only be performed in this particular patient group.

The trial-specific risks and burdens for subjects and caregivers (if applicable) related to diagnostic, therapeutic and monitoring procedures should be justified, e.g. the amount and number of blood samples, the number of site visits, physical examinations or other tests, as well as physical and physiological discomfort associated with trial participation.

SUBMISSION OF RESULTS OF CLINICAL TRIALS

6.1. Question: what do sponsors have to submit when it comes to summary of results?

245. Answer: To be compliant with the Clinical Trials Regulation, sponsors have to submit two documents: a summary of results and a summary written in a manner that is understandable to laypersons. These documents need to be submitted to CTIS within 1 year from the end of the clinical trial irrespective of the outcome of the clinical trial. For paediatric trials, the summary of results needs to be submitted within six months from the end of the end of the clinical trial (Commission Communication 2009/C28/01: C_2009028EN.01000101.xml (europa.eu)). The summary of results shall be submitted at the latest within twelve months after the trial has ended on two occasions: (1) the paediatric clinical trial does not fall within the scope of Article 46(1) of the Paediatric Regulation (EU no 1901/2006), that is, where it is not a study sponsored by a marketing-authorisation holder involving the use in the paediatric population of an already authorised medicinal product — and where it is for scientific reasons not possible to meet the six-month deadline, the summaries may be submitted at the latest within twelve months from the end of the clinical trial.

As per Article 37 (4), “where, for scientific reasons detailed in the protocol, it is not possible to submit a summary of the results within one year, the summary of results shall be submitted as soon as it is available. In this case, the protocol shall specify when the results are going to be submitted, together with a justification.”

246. The content of the summary of results is laid out in Annex IV, while the content of the summary of results for laypersons is laid out in Annex V. The summary of results should include at least results of the primary and secondary endpoints.

The layperson summary should report at least the overall results, the primary endpoints, and patient-relevant secondary endpoints (See also the recommendations of the expert group on clinical trials on "*Summaries of Clinical Trial Results for Laypersons*" February 2018[1]

[1] https://health.ec.europa.eu/document/download/0559976c-ce71-4894-bbc2-cca922acb5ad_en?filename=2017_01_26_summaries_of_ct_results_for_laypersons.pdf

247. When the trial is prematurely ended/early terminated due to lack of subjects or lack of data to analyse, the sponsor shall notify each Member State concerned through the EU portal of the reasons for such action and, when appropriate, follow-up measures for the subjects. Sponsors have to consider whether the trial has included EU/EEA subjects as follows:

a. If a trial was conducted globally but early terminated in the EU/EEA with no EU/EEA subjects ever included, the sponsor should anyhow upload a PDF in CTIS in both spaces for *summary of results* and *layperson summary* with this statement "the trial has been conducted without subjects from EU/EEA, and the statistical analysis does not include data generated from the participation of EU/EEA trial subjects". This is for transparency and clarity towards the public. The act of uploading such documents with this statement is also to ensure compliance with the provisions for early terminated trials. The same document can be uploaded in both spaces.

b. Regardless of the situation in other jurisdictions, the summary of results and the layperson summary are expected to be submitted whenever a trial that included EU/EEA subjects is early terminated, even when a sound statistical analysis cannot be performed because of the limited quantity and/or quality of data.

c. Where trial subjects were enrolled in only some of the MSCs that authorised the trial, this should be clearly indicated in both the summary of results and the summary of results for laypersons.

248. If instead a trial was authorised, it never included EU/EEA subjects, and then it expired after 2 years, neither a summary of results nor a summary of results for laypersons is expected to be submitted in CTIS. This applies irrespective of the trial's status in other jurisdictions.

249. If a trial was authorised, it enrolled subjects in the EU/EEA but was subsequently halted or suspended and ultimately expired, the sponsor should submit both a summary of results and a summary of results for laypersons. Both summaries should explain why the trial was halted or suspended and why it was not resumed. Where the available data are insufficient to allow a sound statistical analysis, primary endpoint data points should be omitted from both summaries, which should instead clearly state that a sound statistical analysis could not be performed because of insufficient data

In these cases, the layperson summary and the summary of results should exclude primary endpoint data points and include a statement indicating that sound statistical analysis of the information due to insufficient data was not possible.

250. In addition, and according to the abovementioned CT EG guidance document, where a clinical trial had to close early, the information included in the summary of results and in the layperson summary should explain the reason for this, for example, evidence of lack of efficacy, safety events, poor recruitment etc. This is expected to be done in sections 3.2 ("When was this study done?") and as a critical change to the study under 3.3. ("What was the main objective of this study?").

Scenarios

1. Authorised trial→ never started in EU/EEA nor globally → Expired → no results

2. Authorised trial→ never started in EU/EEA but yes globally → Expired → no results

3. Authorised trial→ no subjects included in EU/EEA → sponsor wants to close the trial because of lack of recruitment → no end of trial but submission of notification of early termination → declaration stating that the content of the document is not as defined in Annex IV and V and the absence of EU data.

4. Authorised trial→ started in EU/EEA involving subjects → notification of early termination not linked to safety reasons → submission of summary of results and layperson summary + reasons of early termination + follow up measures.

5. Authorised trial→ started in EU/EEA involving subjects→ notification of early termination linked to safety reason no later than 15 days → submission of summary of results and layperson summary + reasons of early termination + follow up measures.

6. If the trial is not authorised in EU/EEA, neither summary of results nor layperson summary have to be submitted in CTIS.

NB: the CTR states that "In the case of early termination of the clinical trial, **the date** of the early termination shall be deemed to be **the date** of the end of the clinical trial. (emphasis added)" but it does not mean that early termination is equivalent to end of trial. Indeed, the CTR reports two different definitions. In case of lack of recruitment, sponsors should notify the early termination of a trial and explain the reasons. Sponsors should not provide the summary of results nor the layperson summary in the MSC where the

trial early terminated with no subjects but a document explaining that statistical analysis cannot be performed. Instead, summary of results and layperson summaries should be provided when the CT ends in the other MSC.

6.2. Question: What is a clinical trial sub-study?

251. Answer: A sub-study is a discrete separate study, which is part of a clinical trial and should be described in the application form and in the protocol. Examples include pharmacokinetic or pharmacogenetic sub-studies.

252. Participation of clinical trial subjects in a sub-study either involves the entire trial population or a specified subgroup of subjects receiving the investigational medicinal products (IMPs) as specified in the protocol. Sub-studies should not include a trial population that is different from that of the main trial. For a sub-study an additional informed consent is required. It should be clear to subjects participating in a clinical trial if the decision to take part in a sub-study is optional and separate from that of the main trial. An optional sub-study should be mentioned in the main informed consent form (ICF) and a more detailed ICF for the sub-study should be provided and signed.

6.3. Question: Is the summary of results of a sub-study of a clinical trial to be reported to the EU portal?

253. Answer: Sub-studies are part of the protocol and investigate a specific question in the clinical trial. Therefore, results of a sub-study are expected to be available at the same time as results of the rest of the clinical trial. Therefore, a summary of results of a clinical trial including sub-studies is due within 1 year after end of the clinical trial. The plan for analysis of sub-study results should be provided within the global plan of analysis of the results of the clinical trial.

254. When additions of sub-studies occur at different time points along the clinical trial duration, the estimated dates when results for each sub-study will be available should be provided.

255. If the analysis of the results of the sub-study is going to be delayed, the sponsor has to provide a justification for it, and indicate the date

when the summary of those results will be submitted. However, publication of the results of a sub- study should not cause any delay in the publication of the summary of the available results of the main parts of the clinical trial.

6.4. Which are the CTIS publication rules?

256. Answer: Increased transparency under the CTR supports public scrutiny, improves research efficiency and provides public with the necessary information to identify ongoing trials for their participation. Article 81(4) of the Regulation states that trial-related documents in CTIS shall be publicly accessible unless justifiable exemptions (e.g. personal or proprietary data protection). Importantly, according to Article 17(1)(a) of Regulation (EU) 2022/123, the trial protocol of trials with the potential to address public health emergencies needs to be published after the decision on the application; according to Article 17(1)(b) the summary of the results shall be submitted within a timeline set by the Agency that is shorter than the timeline laid down in Article 37 of Regulation (EU) No 536/2014.

257. The publication rules of CTIS are based on the Revised CTIS transparency rules thoroughly described in the Guidance document on how to approach the protection of personal data and commercially confidential information (CCI) while using CTIS and its Annex I (see Guidance document on how to approach the protection of personal data and commercially confidential information while using CTIS: https://accelerating-clinical-trials.europa.eu/document/download/824905dd-3033-41e6-a871-67b20c4f4c94_en?filename=annex-i-guidance-document-how-approach- protection-personal-data-commercially-confidential_.pdf and Annex I https://accelerating-clinical-trials.europa.eu/document/download/824905dd-3033- 41e6-a871-67b20c4f4c94_en?filename=annex-i-guidance-document-how-approach-protection-personal-data-commercially-confidential_.pdf) . The revised CTIS transparency rules foresee the disclosure of structured data and key documents of public interest as per timelines based on the trial category, trial phase and population age, in order to protect CCI. In addition, to enable protection of personal data and/or CCI, CTIS offers the users the possibility to upload redacted document versions 'for publication' and unredacted document versions 'not for publication' of those key documents of interest that are publicly disclosed as per revised CTIS transparency rules.

258. To capitalise on increased transparency under the CTR, sponsors are encouraged to submit trial documents 'for publication' with a minimum amount of redactions limited to personal data and commercially sensitive information from the submitted documents to allow their publication after the decision on the application. In addition, non- redacted versions should be submitted with all information relevant for the assessment. It needs to be emphasized that the redacted documents have to remain meaningful to the public, including potential trial subjects and health care professionals.

259. The approach of using redaction would enable the earliest publication of trial documents and at the same time keep sponsors confidence in using EU for clinical trials, especially for early development where sensitivities are highest. Extensive deferrals could significantly reduce the utility of clinical data in CTIS.

6.5. What are considered intermediate data analysis in light of article 37.8 of the CTR?

260. Where the clinical trial provides for an intermediate data analysis before the end of the clinical trial, a summary of these results should be submitted to CTIS within one year of the intermediate data analysis date with the exception of:

a) In cases where the blind of the sponsor and/or investigators should be maintained like intermediate data analysis from Independent Data Monitoring Committees like a DSMB.
b) In cases where the protocol provides clear criteria on the decision how to continue the clinical trial (e.g. on dosing regime in an early phase trial).
c) In cases as described in the protocol that there are justified reasons for which it is not possible to submit a summary of the intermediate results within one year.

261. The summary of the intermediate data results should be in line with annex IV of the Clinical Trials Regulation and is restricted to the endpoints of the intermediate analysis as defined in the clinical trial protocol. The summary might be expanded if justified, e.g. dose selection in integrated protocols based on safety and/or pharmacological data or closing of intervention arm at the completion of subprotocol (e.g. a treatment arm, domain with a specific patient population) in a single complex clinical trial application in CTIS (see also Q&A on complex clinical trials) .

262. In case, the results of the intermediate data analyses lead to a prematurely end of the clinical trial, an unexpected event or an urgent safety measure, these should be notified to the MS concerned without undue delay and not later than the maximum timelines as set in the respective articles in the CTR. The summary of the results of the intermediate results should be submitted to CTIS as soon as possible and not be delayed until one year after the intermediate data analysis date.

263. Based on the revised transparency rules, summary of intermediate data analysis will not be made public. These results should therefore also be taken up in the summary of the final results of the clinical trial which will be published.

264. The RMS can always request to receive intermediate data analysis results to ensure the rights, safety, dignity and well-being of subjects and/or reliability and robustness of data, even if this is not foreseen in the protocol.

6.6. Submission of Interim Results and Summary of Results in CTIS

265. Interim results: 'interim results' and 'intermediate data analysis' are synonymous and refer to the requirements of CTR's Article 37.8.

266. Interim results are expected if specified in the clinical trial protocol or if requested by the NCAs.

267. The timeframe for posting intermediate results is always within 12 months of the interim analysis date, with no distinction between adult and paediatric trials.

268. For end-of-trial results, the requirements differ. Please refer to the CTR Q&A 6.1.

269. Interim results are uploaded in CTIS in the same placeholder as results.

270. Sponsors need to simply select 'Intermediate Results' from the dropdown menu.

271. Interim results are never made public in CTIS according to CTIS revised transparency rules. Please refer to the CTIS revised transparency rules available in the following link: Revised CTIS

transparency rules

272. If a study has been transitioned to CTIS, interim results must be posted in CTIS. For any interim analyses conducted after the transition, results must be posted in CTIS as per regulatory requirements. However, if the interim results were already posted in EudraCT before the transition, they can remain there and do not need to be re-uploaded to CTIS.

273. Summary of Results: The Summary of results is expected to be submitted in English and no translations are required**.**

274. The language requirements for the Lay person summary of results is explained in the Good Lay Summary Practice Guidance and in the Summaries of Clinical Trial Results for Laypersons both available in Eudralex vol 10 in the following links: https://health.ec.europa.eu/document/download/0559976c-ce71-4894-bbc2-cca922acb5ad_en?filename=2017_01_26_summaries_of_ct_results_for_laypersons.pdf and https://health.ec.europa.eu/document/download/8a42b8f5-4ec3-4667-969c-3dd89ea8b270_en?filename=glsp_en.pdf.

275. A Clinical Study Report (CSR) synopsis would be acceptable as summary of results as long as it is compliant with requirements set in the Annex IV of the CTR.

SAFETY REPORTING

7a DEFINITIONS

7.1. Question: How should the definition of an Adverse event be applied in clinical trials, what should be considered?

276. **Answer:** An adverse event (AE) is defined in Article 2 (32) of the CTR[1] as follows: "any untoward medical occurrence in a subject to whom a medicinal product is administered and which does not necessarily have a causal relationship with this treatment".

277. An AE can therefore be any unfavourable and unintended sign (including an abnormal laboratory finding), symptom or disease temporally associated with the use of a medicinal product, whether or not considered related to the medicinal product (see ICH guideline E2A section 2A1)[2].

278. Clinically significant abnormal laboratory findings are considered an AE, if occurring or worsening during (or after, if considered pharmacologically appropriate) the administration of the medicinal product. Unfavourable medical events occurring in subjects before investigational product administration (e.g. during screening) should only be considered and reported to the sponsor if required by the protocol (see ICH guideline E6 (R3) section 2.7.2)[3].

[1] Regulation (EU) No 536/2014 of the European Parliament and of the Council of 16 April 2014 on clinical trials on medicinal products for human use

[2] ICH E2A Clinical Safety Data Management: Definitions and Standards for Expedited Reporting

[3] ICH E6 (R3) Guideline for good clinical practice (GCP)

7.2. Question: What should be taken into consideration in defining Serious adverse events?

279. **Answer:** A serious adverse event (SAE) is defined in Article 2 of the CTR as follows: 'Any untoward medical occurrence that at any dose requires inpatient hospitalisation or prolongation of existing hospitalisation, results in persistent or significant disability or incapacity, results in a congenital anomaly or birth defect, is life-threatening, or results in death.' These characteristics/consequences of a SAE have to be considered at the time of the event. For example, regarding a life-threatening event, this refers to an event in which the subject was at risk of death at the time of the event. It does not refer to an event which hypothetically might have caused death if it were more severe.

280. SAEs include all serious events independent of whether they have a suspected causal relationship to a medicinal product or not, whereas a serious adverse reaction (SAR) is suspected by the investigator or sponsor to have a causal relationship with the use of a medicinal product.

281. Planned surgical procedures or hospitalisations scheduled prior to enrolment or required by the Clinical Investigation Plan (CIP), related to pre-existing conditions without a serious deterioration in health, do not need to be recorded as Adverse Events (AEs) or Serious Adverse Events (SAEs). Such procedures will be documented as medical history / planned procedures in the medical record and CRF. Any unexpected complications or clinically significant deterioration resulting from the procedure must be reported as AE/SAE according to standard reporting procedures.

282. 'Important medical events' (IME) are medical events that may not be immediately life-threatening or result in death or hospitalisation, but may jeopardize the subject's health or may require intervention to prevent one of the other outcomes listed in Question 7.2 point 279, and should be considered serious. One may consider the EMA 'IME list' and/or EMA document 'Inclusion/exclusion criteria for the "Important Medical Events" list'[4] to determine the seriousness of an AE.

283. Medical and scientific judgement should be exercised in deciding whether an event is 'serious' in accordance with these criteria.

284. In accordance with applicable regulatory requirements (Art 41 CTR 536/2014)[1], the protocol may describe SAEs not requiring immediate reporting by investigator to the sponsor; for example, deaths or other events that are defined as endpoints of the clinical trial (See ICH guideline E6 (R3) section 2.7.2).

[4] IME list inclusion-exclusion criteria

7.3. Question: what is the definition of inpatient hospitalisation for the purpose of classifying an adverse event as serious?

285. **Answer:** Inpatient hospitalisation means that the subject has been admitted to the hospital for inpatient care, either to an inpatient ward or an emergency room for observation and/or treatment, that would not have been appropriate in the physician's office or an outpatient setting. As the trend in modern healthcare shifts toward reducing inpatient hospitalisation, it is of utmost importance that medical and scientific judgement is exercised when determining whether an adverse event should be considered serious in other circumstances. This is particularly relevant in cases involving important medical events.

7.4. Question: What is the difference between an Adverse Event and an Adverse Reaction?

286. **Answer:** An adverse event (AE) does not necessarily have a causal relationship with a medicinal product, whereas an adverse reaction (AR) implies at least a reasonable possibility of a causal relationship, i.e. the relationship cannot be ruled out, between the adverse event and the medicinal product, in accordance with ICH-E2A (is any noxious and unintended response to a medicinal product related to any dose administered).

287. In the context of clinical trials, particularly in early phases of development, there may be limited or no available information (facts or evidence) on the investigational medicinal product (IMP) to support or refute a causal association. Therefore, in the clinical trial setting the concept of "reasonable possibility" of a causal association should be applied according to a more conservative approach (i.e. the relationship cannot be ruled out)

7.5. Question: What is a Serious Adverse Reaction?

288. **Answer:** Serious adverse reactions (SARs) are defined as all noxious and unintended responses to a medicinal product related to any dose administered that:

- result in death,
- are life-threatening,
- require inpatient hospitalisation or prolongation of existing hospitalisation,
- result in persistent or significant disability or incapacity or are a congenital anomaly or birth defect (Article 1 of Directive 2001/83/EC)[5].

[5] Directive 2001/83/EC of the European Parliament and of the Council of 6 November 2001 on the Community code relating to medicinal products for human use

Except for the relatedness (causality), the definitions of SAEs apply (see Question 7.2).

7.6. Question: How should the definition of an Unexpected Serious Adverse Reaction be applied in clinical trials?

289. **Answer:** An unexpected serious adverse reaction is defined in Article 2 (34) of the CTR as a Serious Adverse Reaction whose nature, severity or outcome is not consistent with the Reference Safety Information (RSI, see Chapter 7b REFERENCE SAFETY INFORMATION).

7.7. Question: What is the difference between seriousness and severity?

290. **Answer:** The severity of an adverse event refers to the intensity of the event and is usually categorized as mild, moderate, or severe. Severity is assessed based on the degree to which the AE disrupts a patients' normal daily activities. As an example, to standardize this assessment, the Common Terminology Criteria for Adverse Events (CTCAE)[6] can be used, CTCAE defines five grades of severity: Grade 1: Mild, Grade 2: Moderate, Grade 3: Severe, Grade 4: Life-threatening, Grade 5: Death.

291. Seriousness is a regulatory definition and is based on patient or event outcome or action criteria usually associated with events that pose a threat to a patient's life or vital functions. It serves as a guide for defining regulatory reporting obligations.

292. For example, a headache may be severe (prevents everyday activities) but is not considered serious (it does not require inpatient hospitalisation or results in persistent disability/incapacity/congenital anomaly/birth defect and is neither life-threatening nor results in death).

[6] Common Terminology Criteria for Adverse Events (CTCAE) v6.0 (MedDRA 28.0)

7b REFERENCE SAFETY INFORMATION

7.8. Question: What is the purpose of the Reference Safety Information and what should it contain?

293. **Answer**: The Reference Safety Information (RSI) should be used as the basis for expectedness assessment of Serious Adverse Reactions (SARs) by the sponsor for expedited reporting of suspected unexpected serious adverse reactions (SUSARs) and for annual safety reporting.

294. The RSI contains expected SARs which should be restricted to SARs that have been observed more than once during treatment with the Investigational Medicinal Product (IMP; see Question 7.9). Additionally, after a thorough assessment by the sponsor, reasonable evidence of a causal relationship between the event and the IMP treatment should exist. This evidence should be based on, for example, the comparative incidence of other SARs in all previous and ongoing Clinical Trials (CTs) and on a thorough evaluation of the causality of the individual reported case. The evaluation should be performed from the perspective of events previously observed with the IMP treatment, not based on what might be anticipated from the pharmacological properties of the IMP[7].

295. SARs that have occurred only once do not usually qualify for inclusion in the RSI, unless there is a very strong plausibility of a causal relationship with the IMP and a robust justification based on medical judgement is provided. Biological plausibility based on the mechanism of action of the IMP and the presence of risk-mitigation strategies alone are not sufficient.

296. The occurrence of a SAR more than once is not by itself an adequate justification for the addition to the RSI as an expected SAR. A thorough assessment by the sponsor is also required. A justification for the addition to the RSI should be submitted alongside the proposed addition. Explicit justification is also needed for including SARs from post-marketing experience (see Question 7.10). SAR terms from post-marketing exposure can be added to the RSI for the same or relevant indications if justified. If the indications of the authorised (EU/EEA Marketing Authorisation) IMP application differ from the CT indication, the RSI should be based only on the clinical experience in the relevant indication. Thus, separate RSIs might be needed within one IB for different indications or populations, e.g. paediatric patients (see Question 7.9).

297. Generally, sponsors should not expect an IMP to cause fatal SARs. Life-threatening SARs should also not be considered expected for IMPs, unless supported by a positive benefit/risk balance.

[7] Clinical Trials Regulation (EU) No 536/2014: Annex III

298. If there are expected life-threatening or fatal SARs listed in the RSI section of an IB, the RSI should include the number of suspected life-threatening and fatal suspected SARs that have occurred. The numbers of life-threatening/fatal SARs that have previously occurred but that are considered unexpected should not be listed in the RSI and should be listed elsewhere in the IB. Exception: IMPs with a MA in the EU/EEA/ICH country, when it is clearly stated in section 4.8 of the Summary of Product Characteristics (SmPC) that the IMP may cause these fatal or life-threatening SARs, in such cases, fatal and life-threatening SARs might be considered expected. The RSI of an IMP that has not received a Marketing Authorization in the EU/EEA should never include fatal SARs.

299. If a SAR is added to the RSI, an update of the benefit/risk statement should be provided, and adequate Risk Minimization Measures (RMMs) should be proposed in the updated protocol(s) and described in the updated Investigators Brochure.

7.9. Question: Which document should contain the Reference Safety Information?

300. Serious Adverse Reactions (SARs), for an unauthorised Investigational Medicinal Product (IMP), should be a clearly defined section in the Investigators Brochure (IB). This section, titled 'Reference safety information' can be part of section 7 ('Summary of Data and Guidance for the investigator') or a separate, distinct section, e.g. section 8.

301. For an IMP with an EU/EEA Marketing Authorisation (MA), which is used according to the MA, Section 4.8. 'Undesirable Effects' of the appropriate Summary of Product Characteristics (SmPC) can be used as RSI. If the IMP has a MA in several EU Member States (MSs) with differing SmPCs, the sponsor should justify its selection of the most appropriate SmPC as the RSI, with reference to subject safety. An EU SmPC should be submitted, but if it does not fit the use of the IMP in the trial, a relevant SmPC may be submitted. If an SmPC is used as the RSI, the study protocol should be compliant with the Risk Minimization Measures (RMMs) included in the SmPC. The SmPC should be submitted as a separate document (section 4.8 of the SmPC should not be copied into the RSI of the IB). If it is proposed to use an IMP outside an EU/EEA authorised indication, please specify this in the application's cover letter. Otherwise, the RSI should be a specific section within the IB.

302. The Company Core Data Sheet (CCDS) is not accepted as RSI on its own. However, the CCDS may be contained in an appendix to the IB and include the RSI as a separate, clearly identified, section. The RSI section must be compliant with the guidance provided in this document.

303. The location of the RSI should always be clearly indicated in the cover letter of the Clinical Trial application.

7.10. Question: Which format should be chosen for the Reference Safety Information?

304. **Answer:** The Reference Safety Information (RSI) should be presented as a tabular listing of expected Serious Adverse Reactions (SARs) organised by Medical Dictionary for Regulatory Activities (MedDRA) System Organ Class (SOC) and Preferred Term (PT), followed by event frequency (n and %) and frequency category. The latest MedDRA version should always be used. Importantly, the RSI does not represent a comprehensive overview of the safety profile of the Investigational Medicinal Product (IMP)[7].

305. The frequency must be calculated at an aggregated level and should be based on the previously observed SARs. The frequency is preferred to be categorised in line with Adverse Reaction (AR) frequencies included in Section 4.8. of the Summary of Product Characteristics (SmPC)[8]. If an insufficient number of subjects has been exposed to the IMP, or low numbers (e.g. two) of the expected SARs have been observed, the number of each expected SAR, together with the number of exposed patients, should be provided instead of the frequency categories (refer to Table below for example)

306. Including post-marketing ARs in the RSI is acceptable, but only if they were serious, i.e. SARs. If the frequency is not known, the absolute numbers for each SAR or a frequency category calculated as per the 'adverse reactions from spontaneous reporting' guidance for a SmPC should be used.

Example of an RSI:

Serious Adverse Reactions for the IMP considered expected for safety reporting purposes.

SOC	SARs[1]	Number of subjects exposed (n) = 328	
		All SARs n* (%)	Frequency category[2]
Gastro-intestinal disorders	Intestinal perforation	9 (2.7)	Common
	ALT increase	12 (3.6)	Common

[8] Guideline on Summary of Product Characteristics - SmPC: Eudralex Volume 2C – Regulatory guideline

Hepatobiliary disorders	AST increase	9 (2.7)	Common
Cardiovascular disorders	Myocarditis	3 (0.9)	Uncommon
	Bradycardia	0 (0)[3]	Uncommon

n = number of subjects who have experienced the SAR

[1)] If fatal / life-threatening SARs are considered expected for an IMP, this information should always be included as a footnote. If no fatal / life-threatening SARs are expected for the IMP, this must be clearly stated in the RSI section.
2) Frequencies are defined as: very common (≥ 1/10); common (≥ 1/100 to < 1/10); uncommon (≥ 1/1 000 to < 1/100); rare (≥ 1/10 000 to < 1/1 000); very rare (< 1/10 000). Within each frequency grouping, adverse reactions are presented in order of decreasing seriousness.
3) Bradycardia seen in post-marketing settings only, not in CTs. Frequency calculated as per SmPC guidance: event not seen in 328 subjects exposed in CTs. Post-marketing events were serious and occurred more than once.

307. If the IMP is under development for different indications, route of administration or populations (e.g. adults and paediatrics) and the expected SARs differ, separate tables of expected SARs by indication or population should be provided (e.g. for oncology conditions, non-oncology diseases and for paediatric trials).

308. For paediatric populations, it is expected that fewer expected SARs are included in the RSI than for adults as it cannot be assumed that the safety profile for the paediatric population is the same as for the adult population. Only SARs expected for the paediatric population, based on available experience with the IMP in this population, should be included, unless otherwise justified.

7.11. Question: Which terms should be used for expected Serious Adverse Reaction in the Reference Safety Information?

309. **Answer:** The latest version of the Medical Dictionary for Regulatory Activities (MedDRA) Preferred Terms (PTs) should be used for reporting Serious Adverse Reactions (SARs). The use of medical concepts or unspecific terms in the Reference Safety Information (RSI) like 'Infection' or 'Rash' is not acceptable. Only MedDRA PTs for specific diagnosis, e.g. exfoliative dermatitis, urticarial rash or hives, herpes zoster, pneumonia, sepsis or atrial fibrillation are allowed.

310. If a PT is listed, all assigned Low Level Terms (LLTs) are considered expected. For example, if 'Pyrexia' is listed, then 'Fever' is also expected. The investigator should aim to provide the most suitable and specific diagnosis.

311. If the IMP is used in an EU/EEA and ICH countries has approved indication, the use of Section 4.8. of the SmPC, in its approved format, may be acceptable.

7.12. Question: When should expected serious adverse reactions be considered as unexpected?

312. **Answer:** If the specificity or severity of an expected Serious Adverse Reaction (SAR) has changed, it should be considered unexpected and thus be reported as a SUSAR to the EudraVigilance. This applies for all clinical trials but is especially relevant after the early phase of development when sufficient data is available for analysis.

313. A provision of severity grades using Common Terminology Criteria for Adverse Events (CTCAE) grading system in the Reference Safety Information (RSI) is not required.

314. If the sponsor detects an increased frequency of an expected SAR, they should report this as an Unexpected Event (UE) in CTIS (see Question 7.26).

7.13. Question: What is understood by synonymous medical terms and are they allowed in the Reference Safety Information of the Investigator's Brochure?

315. **Answer**: Synonymous medical terms (e.g. Neutropenia and Neutrophil count decreased, see Table below), representing the same medical condition, should be merged, meaning grouped within the same Medical Dictionary for Regulatory Activities (MedDRA) Preferred Terms (PT), in the Reference Safety Information (RSI) and explained in a footnote.

<u>Examples of synonymous medical terms:</u>

PTs for expected SARs	Synonymous medical terms
Pneumonia	Right upper lobe pneumonia
Hypophosphataemia	Blood phosphorus decreased

7.14. Question: What safety information should not be included in the Reference Safety Information, but may be presented elsewhere in the Investigator's Brochure?

316. **Answer:** The following information should not be included in the Reference Safety Information (RSI), but should be presented elsewhere in the Investigator's Brochure (IB) (e.g. in subsections 'Effects in Humans' or 'Summary of Data and Guidance for the Investigator'):

- Serious and non-serious Adverse Events (AEs) that were considered unrelated to the Investigational Medicinal Product (IMP) by both the investigator and the sponsor
- Non-serious Ars
- All SARs that are not considered expected
- SARs that have occurred only once, unless there is a very strong plausibility of a causal relationship with the IMP and a robust justification based on medical judgement is provided
- Deaths or SAEs also considered efficacy endpoints in Clinical Trials with high mortality or morbidity as defined in the protocol approved by the competent authorities are to be treated as disease-related events and should therefore not be subject to systematic unblinding. However, careful assessment should be performed in cases where disease related events appear to be enhanced by the IMP
- SARs that are expected for similar products within the therapeutic class, which did not occur in subjects taking the IMP.

- Information regarding the overall safety profile of the IMP: In accordance with the International Council for Harmonisation (ICH) E6 (R3) guidance, the 'Summary of Data and Guidance for the Investigator' section should provide the investigator with an overview of the potential and identified risks, contraindications, warnings, potential drug-drug interactions, effects on pregnancy and fertility, etc. This section should also discuss measures to mitigate the risks associated with the IMP treatment. These risk mitigation strategies should also be reflected in the protocol, as appropriate, for example, presented in a table listing the identified and potential risks along with the corresponding mitigation measures.

7.15. Question: What should be it included in the section Reference Safety Information in trials if there are no 'expected' serious adverse reactions?

317. **Answer:** If the Investigational Medicinal Product (IMP) is not expected to cause any Serious Adverse Reactions (SARs), or it is not yet known if SARs can be expected, e.g. during early clinical development when subject exposure is low the Reference Safety Information (RSI) section should still be included in the Investigator's brochure (IB). It should contain a brief text stating that no SARs are considered expected for the IMP by the sponsor for the purpose of expedited reporting. Consequently, all SARs should be reported as Suspected Unexpected Serious Adverse Reactions (SUSARs) for the IMP.

Sample Text: *'No serious adverse reactions are considered expected for [IMP] for the purpose of expedited reporting of suspected unexpected serious adverse reactions.'*

7.16. Question: When should the Reference Safety Information be updated?

318. **Answer:** It is recommended to evaluate on a yearly basis if an update of the Reference Safety Information (RSI) section of the Investigator's Brochure (IB) is needed. The annual IB (including the RSI) update should be done in alignment with the preparation of the ASR (see Question 7.487d ANNUAL SAFETY). It is expected that cumulative safety data will be reviewed during the preparation of an ASR and used to support an RSI update.

319. The RSI should be kept up to date for all trials with the relevant IMP, a current version of the IB should be submitted to CTIS for all trials with the IMP. The pdated RSI can only be used for expectedness assessment of Serious Adverse Reactions (SARs) after a positive decision by the first EU/EEA Member state. This applies to all approved CTs with the relevant IMP. Thus, the expectedness assessment should be performed according to the version of the RSI approved for the respective CT at the time of SAR occurrence.

320. The updated RSI has to be approved by at least one MS concerned for each specific CT with the IMP prior to use in that specific CT.

321. In the case of multiple clinical trials conducted by the Sponsor at a global level, it may be necessary to establish a single effective date for the RSI at the project level. In such circumstances, the Sponsor may postpone the effective date of the RSI for the purpose of expedited reporting until all required approvals have been obtained.

322. An RSI update or an update of section 4.8 of a Summary of Product Characteristics (SmPC) when it is used as an RSI (e.g. addition of new expected SAR Preferred Terms (PTs), change of the frequency of expected SARs, Medical Dictionary for Regulatory Activities (MedDRA) updates) is always a Substantial Modification (SM). Changes to the format of the table, that do not affect the expected SARs or minor modifications of exposure rates, not resulting in a change of the frequency category, and without the addition of new expected SARs and/or new PTs classification, are not considered substantial.

323. When submitting a SM that involves an IB or SmPC update, the cover letter must indicate if the RSI is being updated (for clarification on the concept of "update", please refer to point 46). When submitting a SM with an RSI update of the IB or SmPC, but without any modifications to the protocol, the sponsor should specify in the submission cover letter what Risk Minimization Measures (RMMs) are currently in place in the protocol to manage any new safety issues and if these new safety issues are adequately covered in the subject information leaflet (informed consent form) or if it needs to be updated. References to any parallel ASR submissions should also be given in the cover letter. A tracked changes version of the IB or a list of section 4.8 changes in case of use of an SmPC should be provided. In cases where justifications for modifications to the RSI are provided in additional documents, these documents should be submitted simultaneously.

324. It is strongly recommended, when submitting a SM with an RSI update, to submit this to all Clinical Trials (CTs) using this RSI for expectedness assessment of SARs (i.e. using the same IMP) as soon as possible. Information on all ongoing CTs for which this SM and RSI update should apply, should be listed in the cover letter.

325. If simultaneous submission is not possible, for example, due to another ongoing SM in a trial, the EU CT numbers of the previously submitted RSI update should be indicated in the cover letter at the time of submitting the current SM application.

326. If the RSI is included in an IB which is not prepared and updated by the sponsor of the ongoing CT (e.g. for non-commercial sponsors using a company's IB), this sponsor should have a written agreement in place with the stakeholder responsible for updating the IB and RSI. The agreement should include confirmation that the approved updated IB with the RSI will be shared with other relevant sponsors using the same IMP, immediately. The (non-commercial) sponsor should submit the approved updated IB, together with any necessary modifications to the protocol, as a SM for their CT without undue delay. The reporting of new relevant safety issues from the stakeholder preparing the IB, to all relevant sponsors using the same IMP, should always be immediate and not be dependent on approval of the RSI update.

327. If the RSI is section 4.8 of the SmPC and a new public version of the SmPC with an updated section 4.8 becomes available during a trial, it is recommended to submit a SM, depending on the changes (see Annex IV. Classification of changes to ongoing clinical trials). Following approval of the updated SmPC for use as RSI in at least one MS of the specific CT, the updated SmPC should be used for the purpose of expectedness assessment of SARs.

328. An urgent update to the safety data in the IB may be deemed necessary by the sponsor or regulatory authorities at any time during the conduct of a CT. This information can be added to other sections of the IB (preferably to the 'Safety and Efficacy' section under 'Effects in Humans' and / or 'Summary of Data and Guidance for Investigators' section). The RSI section should only be updated with the results of the SUSARs analysis.

7.17. The Reference Safety Information is not a clearly identified section in the Investigator's Brochure accompanying a new clinical trial application. Does the IB have to be amended?

329. **Answer:** Yes, if the Reference Safety Information (RSI) is contained within the Investigator's Brochure (IB) but there is no clearly identified section for 'Reference Safety Information', where all expected Serious Adverse Reactions (SARs) are listed in a table (see Question 7.9), CT applications risk being rejected. If there are no expected SARs for the Investigational Medicinal Product (IMP) at the point of submission, see Question 7.14 for further instructions.

7.18. Question: Who should assess the causality between a Serious Adverse Event and an Investigational Medicinal Product and how should it be done?

330. **Answer:** The causal relationship should primarily be assessed by the investigator. The sponsor can upgrade the assessment (from unrelated to related) but may not downgrade it. If the sponsor disagrees with the causal relationship assessed by the investigator, the opinions and justifications of the investigator and the sponsor should be recorded in the Individual Case Safety Report (ICSR) in line with International Council for Harmonisation (ICH) E2B (R3)9.

331. In accordance with ICH E2A, and ICH E6 (R3) the definition of an Adverse Reaction (AR) implies at least a reasonable possibility of a causal relationship between a medicinal product and an Adverse Event (AE). An AR, in contrast to an AE, is characterised by the fact that a causal relationship between a medicinal product and an occurrence is suspected (see Question 7.4). Several methods for performing causality assessments of ARs are available, and the International Organization for Standardization (ISO) ICSR standard allows for providing several outcomes of causality assessments using one or more methods. In Clinical Trials (CTs) all AEs should be considered at least possibly related to the Investigational Medicinal Product (IMP) unless it was assessed as 'not related'. If an investigator uses the World Health Organization (WHO) classification of causality the terms 'unlikely' and 'not related' may be considered as not related. In case an AE is assessed as 'unknown' or 'not assessed' by the investigator, the sponsor should request follow up information from the reporting investigator and encourage them to provide a causality assessment.

332. If the investigator does not provide a causality assessment, even after the sponsor requested for causality assessment as follow-up information, the sponsor should assume at least possible causality and the event qualifies as a SAR. However, only SARs with a clear causality should be included in the Reference Safety Information (RSI).

[9] ICH E2B (R3) Clinical Safety Data Management: Data Elements for Transmission of Individual Case Safety Reports (ICSRs)

7.19 Question: What should it be used as Reference Safety Information for clinical trials with combinations of Investigational Medicinal Products?

333. **Answer**: In case of Clinical Trials (CTs) investigating a combination of Investigational Medicinal Products (IMPs), the sponsor can either:

- Use one Reference Safety Information (RSI) for each IMP in the combination. Each RSI can be in either the Investigator's Brochure (IB) or Summary of Product Characteristics (SmPC).

or

- Create a distinct RSI table for the combination by evaluating suspected Serious Adverse Reactions (SARs) from previous CTs involving the same combination of IMPs.

334. The sponsor should explain how the RSI has been compiled and especially for new combinations, new indications or new populations, take a risk-based approach for inclusion of expected SARs in the RSI.

335. When using fixed-dose combinations of IMPs, a distinct RSI section for this combination should always be provided.

7.20. Question: How should the Reference Safety Information for the development of biosimilar drug products be written?

336. **Answer:** The Reference Safety Information (RSI) of the originator may be accepted for a biosimilar product, if adequately justified. In addition, the protocol should include measures to mitigate both the known risks associated with the originator and any new risks associated with the biosimilar (for example potential risk of reduced efficacy when compared with the originator).

7.21. Question: Which version of the Reference Safety Information should be used to determine the expectedness of Serious Adverse Reactions for follow up reports?

337. Answer: The Reference Safety Information (RSI) in effect and approved at the time of occurrence of the Serious Adverse Reaction (SAR) should also be used to assess expectedness for follow up reports to cases. Suspected Unexpected Serious Adverse Reactions (SUSARs) should not be downgraded on the basis that the RSI was updated after the occurrence of an event.

7c REPORTING OF ADVERSE EVENTS/REACTIONS

7.22. Question: How should relevant information on Suspected Unexpected Serious Adverse Reactions (SUSARs) be reported to Member States?

338. **Answer**: Rules for SUSAR reporting are established in CTR Article 42 and Annex III[7]. All SUSARs relating to CTs under the CTR must be submitted via the EudraVigilance Clinical Trial Module (EVCTM) on the basis of the Individual Case Safety Reports (ICSR), receiver ID should be the EVCTMPROD[10]. Separate submissions to Member States are not foreseen in the CTR.

339. Sponsors should report all SUSARs occurring in CTs of which they are sponsor of, independent of where they occur, EU/EEA or 3rd countries. SUSARs from clinical trials conducted by another sponsor who is not part of the same parent company as the sponsor of the clinical trial, and who does not develop a medicinal product with the same active substance jointly, on the basis of a formal agreement, with the sponsor of the clinical trial, do not need to be reported (CTR article 42 section 1).

[10] EU Individual Case Safety Report (ICSR)1 Implementation Guide EMA/51938/2013 Rev 2* , (2021); Chapter I.C.3.1.2. EV Message Receiver Identifiers

340. In addition to the minimal data required for SUSAR reporting (CTR Annex III), the sponsor must also report all medical information that is relevant to the case. Medical and scientific judgement should be applied in identifying relevant information. Determining what information is relevant should be based on medical and scientific judgment, ensuring the case is properly evaluated to continuously assess the risk-benefit balance for subjects and the corresponding clinical trials (see Question 7.29).

341. New administrative information that could have an impact on the case management should also be considered relevant. This includes, for example, any information that may help detect potential case duplicates (e.g. identification of new case identifiers used in previous transmissions). General and specific guidance for safety data collection and analysis is available in Good Vigilance Practice (GVP) modules VI and IX[11,12]. The same principles are valid for clinical trial pharmacovigilance. Minor corrections of dates or typographical errors from previous case versions, or updates due to new versions of MedDRA, are not considered relevant information if they have no impact on the medical content.

342. New medical information that has an impact on the medical evaluation or interpretation of the case (e.g. causality, severity, restart of treatment or seriousness) or on the benefit risk analysis of the subject or the CT is considered relevant information which qualifies for a follow up report to EVCTM.

343. Comparators and placebos are IMPs. Therefore, SUSARs associated with comparators and placebos follow the same reporting requirements as for the test IMP (including reporting to EudraVigilance). Events associated with placebos will usually not fulfil the criteria for a SUSAR and for expedited reporting: If a SUSAR is unblinded and reveals the IMP was actually a placebo, the event generally no longer meets the definition of a SUSAR and usually does not require expedited reporting to regulatory authorities as a SUSAR. The unblinding information must be documented in the case narrative, and the case should be updated in the safety database (e.g., removing the active drug and acknowledging placebo).

344. However, where SUSARs are associated with placebos (e.g. reaction due to an excipient or impurity), the sponsor should report such cases and take appropriate Risk Minimization Measures.

[11] Guideline on good pharmacovigilance practices (GVP) - Module VI – Collection, management and submission of reports of suspected adverse reactions to medicinal products (Rev 2)
[12] Guideline on good pharmacovigilance practices (GVP) - Module IX – Signal management (Rev 1) and Addendum

345. In case a causal link between an IMP and an AE cannot be ruled out, and where an Auxiliary Medicinal Product (AxMP) was also administered, the reporting rules for the IMP apply. See also the specific guidance for AxMPs[13] and Question 7.25 and 7.26.

346. When, after the initial report, it is considered that an event no longer fulfils the SUSAR reporting criteria, for example upon receiving follow-up information from the investigator leading to a change of causality (from Related to Not Related), seriousness (from Serious to Non-Serious), or expectedness (from Unexpected to Expected) a follow up report downgrading the SUSAR should be sent (see Question 7.23). A change in expectedness can only lead to a downgrade if the investigator changes the diagnosis, but not because of an update to the RSI (compared to the RSI at time of SUSAR). That change initiated by the investigator should be considered relevant information and the report qualifies for 'downgrade' in EVCTM. SUSAR report downgrading should be done in accordance with the timelines mentioned in section 2.4 of Annex III of the CTR.

347. A sponsor shall never downgrade a report, even when they disagree with the investigator's assessment. The opinions of both the investigator and the sponsor should be provided in the narrative and in the relevant structured ICH E2B data elements.

348. Please note that a case downgrade is not equivalent to a nullification, see Question 7.23.

7.23. Question: Is there a difference between downgrading and nullification of an earlier reported SUSAR in the EudraVigilance database?

349. **Answer:** Yes, downgrading and nullification are not equivalent.

350. **Downgrading** of cases can occur when new information leads to a change of the following SUSAR reporting criteria: seriousness criteria, from serious to non-serious by the investigator; causality, from related to not related by the investigator; expectedness from unexpected to expected (see Question 7.22).

[13] Auxiliary medicinal products in clinical trials – Recommendations on the use of Auxiliary Medicinal Products in Clinical Trials written and endorsed by the Clinical Trials Coordination and Advisory Group (CTAG).

351. In line with ICH E2B, **nullification** of an individual case should be used to indicate that a previously transmitted report is considered completely void / invalid (nullified), Other cases should be downgraded (e.g. seriousness, causality, expectedness, see above). Reasons for nullification are:

- The whole case was found to be erroneous
- Duplicate reports have been identified
- Minimum reporting criteria, like identifiable event, subject, suspected medicinal product and reporter are no longer met

352. In both downgrading and nullification of a cases, new information leads to a significant change in the report, therefore a follow-up with the relevant information has to be submitted to EVCTM, in accordance with the timelines mentioned in section 2.4 of Annex III of the CTR.

353. More information about nullification and downgrading is available in GVP Module VI (VI.C.6.2.2.9 core principle of nullification and VI. Appendix V examples) and technical information is provided in the EU ICSR Implementation Guide.

7.24. Question: Is unblinding required if a reported SAR is unexpected for either the test or a comparator IMP?

354. **Answer:** As for reporting, the sponsor shall unblind the treatment allocation only for the subject the SUSAR is related to (CTR Annex III, 2.5., ICH E6).

355. The sponsor must unblind the treatment for safety evaluation and regulatory reporting purposes if a SAR is unexpected as per the RSI of either of the suspected IMPs.

356. Unblinding is not necessary for SARs assessed as expected for all suspected IMPs.

357. The sponsor should have procedures in place to maintain the blind for persons responsible for the conduct of the ongoing study (such as the management, monitors, investigators) and those responsible for data analysis and interpretation of results. Unblinded information should only be accessible to those who need to be involved in safety evaluation and regulatory reporting.

358. A separate procedure should exist for SARs unblinded for emergency purposes for the clinical management (diagnosis/treatment) of SARs or safety issues by the investigator (ICH E6). As per the CTR**, Annex III, 2.5. "Unblinding treatment allocation**", investigators should only receive blinded information unless unblinded information is **required for safety reasons.**

7.25. Question: Which safety issues should not be reported as SUSARs?

359. **Answer:** SUSARs should be reported in accordance with Article 42 of Regulation (EU) No 536/2014, the following should not be considered SUSARs:

- Adverse reactions considered only related to **authorised** AxMPs or concomitant medication received by the subject and without interaction with the IMP (see Question 7.25 and 7.26). For those Adverse reactions, the rules on pharmacovigilance as set out in Directive 2001/83/EC and Regulation (EC) No 726/2004[14] are applicable (see Question 7.26 and AxMP recommendation).
- Reports of deaths or SAEs also considered efficacy endpoints in trials with high mortality or high morbidity and accepted to be considered as disease related events in the protocol; systematic unblinding at the time of the event is not required for such reports (CTR Annex III, section 2). Careful assessment is required in cases where disease-related events appear to be enhanced by the IMP. In accordance with the CTR, a causality assessment is required for each SAE, and if the investigator considers disease-related events to be related to the IMP as well and the event is serious and unexpected then it has to be reported as a SUSAR.
- SUSARs occurring in a CT performed (partly or exclusively) in the EU which are not conducted by the sponsor. These SUSARs may come to the attention of the sponsor through individual reports, publications (such as academic literature) or regulatory authorities.
- SARs occurring in a third country outside clinical trials.
- A SAE which could be associated with trial procedures, and which could modify the conduct of the trial.
- A significant hazard to the subject population such as lack of efficacy of an IMP used for the treatment of a life-threatening disease.
- A major safety finding from a newly completed animal study (such as potential carcinogenicity).
- Recommendations of the Data Safety Monitoring Board (DSMB), where relevant to the safety of subjects.

[14] Regulation (EC) No 726/2004 of the European Parliament and of the Council of 31 March 2004 laying down Community procedures for the authorisation and supervision of medicinal products for human and veterinary use and establishing a European Medicines Agency

- Relevant safety information regarding the procurement or donors for advanced therapy investigational medicinal products.

360. The information described in the bullet points above should instead be addressed through the reporting of events other than SUSAR. Safety relevant information may qualify for notification e.g. unexpected event via CTIS (see Question 7.27). It should be discussed in the IB (e.g. in safety sections other than the RSI) as well as the ASR or in protocol modifications as applicable, especially if relevant for the risk/benefit evaluation. This also applies to any relevant follow-up measures.

7.26. Question: What should be the terminology, formats and standards for the SUSAR reporting to EVCTM?

361. **Answer:** Adverse reactions suspected to be only related to a medicinal product other than the IMP, including Auxiliary Medicinal Products (AxMP), and which do not result from a possible interaction with the IMP should be reported in accordance with regular Pharmacovigilance practice as described in Chapter 3 of Title IX of Directive 2001/83/EC (refer to Article 46 of the CTR for AxMP). This means that they shall be reported to the EudraVigilance Post Authorisation Module (EVPM), irrespective of whether they are used in accordance with the terms of the marketing authorisations or not (see GVP Module VI and the Recommendations on AxMP). Although it is not specified for the AxMPs, this applies only to authorised AxMPs. Note, reduced reporting in line with CTR Art 41 and ICH E19 can be applied to AxMPs too, if specified in the protocol.

362. While reporting the suspected adverse reactions for an authorized AxMP or concomitant medication only, to EVPM, it is recommended to report with the value 'Report from study' in data element 'Type of Report' (ICH E2B (R3) C.1.3) and with the value 'Other studies' in the data element 'Study Type Where Reaction(s) / Event(s) Were Observed' (ICH E2B (R3) C.5.4). The EU clinical trial number, and protocol code should be given in the relevant E2B (R3) data elements. C5.1.r.1 'Study registration number' and in the narrative section, and there to state that the medicinal product was not used as an IMP but as an AxMP or concomitant medication. (in case not possible all information on the CT and role of MP should be at least given in the narrative.).
Note: As of GVP module VI multiple routes of reporting are possible (by whom, to whom, via). Therefore, please clarify reporting responsibilities (and routes) in the protocol.

363. Conversely, expedited safety reporting for non-authorised AxMPs (that should be used only exceptionally in clinical trials – in line with Article 59 of CTR) or modified authorized AxMPs shall be done in accordance with Article 42 and Annex III of the CTR. This means that they shall follow the same requirements as those provided for the IMP. Accordingly, all SUSARs related to non-authorised or modified authorized AxMPs shall be reported to the EVCTM.

7.27 Question: How to report safety issues not falling within the definition of SUSARs?

364. **Answer:** Events may occur during a clinical trial (CT) which do not fall within the definition of a SUSAR and, thus, are not subject to the reporting requirements for SUSARs (examples see Question 7.25), even though they may be relevant in terms of subjects' safety. These events might still require (immediate) risk-mitigation actions followed by regulatory reporting obligations such as:

- Notification of unexpected events with the potential to change the benefit-risk of the trial (CTR Art 53),
- Notification to the MSs via an Urgent safety measure (CTR Art 54)
- Substantial modifications of the clinical trial (CTR Chapter III)[15]
- Early termination or temporary halt of a trial due to safety (CTR Chapter VI)[16] (see Chapter 10 in this document)

365. All notifications of such safety issues should be done via CTIS.

366. When submitting a notification like an urgent safety measure (USM), unexpected event (UE) changing benefit risk, temporary halt or premature end of a CT, sponsors are asked to inform MSs if other CTs are also affected by the issue and if so, list all EU CT numbers of affected trials. The sponsor should indicate if the same or a similar notification will be/has been submitted regarding these listed CT(s). The information should be provided in a table (example given in the table below). Note: If the structured fields not fit, please attach a document for the required information.

Example table for providing EU CT numbers of notification-relevant trials

EU CT number	Type of notification	Risk Minimization Measure	RMS
123456-78-00	Urgent safety measure	Yes	e.g. AT

367. In addition, while submitting a notification all relevant available documents and information related to the issue and the proposed Risk Minimization Measures at time of notification submission should be provided, if possible.

[15] Clinical Trials Regulation (EU) 536/2014, Chapter III
[16] Clinical Trials Regulation (EU) 536/2014, Chapter IV

368. In case of an urgent safety measure which requires an update of protocol, it is expected that the notification of the USM is submitted immediately to inform of the applied changes and/or planned changes, as of CTR with undue delay but not later than 7 days from the date the measures have been taken. That USM information must be followed by a submission of the documents as SM (substantial amendment for approval".

7.28 Question: What should be the terminology, format and standards for SUSAR reporting to EVCTM?

369. Use of terminology: For the classification, retrieval, presentation, evaluation and assessment, electronic exchange and communication of SUSAR information to EVCTM, sponsors should apply the terminology as of Article 25 of the Implementing Regulation (IR) 520/2012 (amended July 2025)[17].

370. Use of internationally agreed formats and standards: For the description, retrieval, presentation, evaluation and assessment, electronic exchange and communication of SUSARs, sponsors should apply the following formats and standards as of Article 26 of the IR 520/2012 (amended July 2025).

371. The current version of the EU Individual Case Safety Report (ICSR) Implementation Guide also see ICH E2B (R3).

372. Reference is also made to EMA's Guidelines on Good Pharmacovigilance Practices (GVP), particularly module VI on 'Collection, management and submission of reports of suspected adverse reactions to medicinal products.

373. Sponsors shall report SUSARs electronically to the Eudravigilance database via EVWEB or using the E2B (R3) electronic ICSR forms. To help sponsors with SUSAR reporting, a web-based form was developed in accordance with Article 40.2 of the CTR in the Clinical Trials Module of the Eudravigilance database ("EVWEB report form"). This form was developed in compliance with Annex III 2.3 of the CTR and based on international guidance documents (including ICH E2B (R3)). The use of this form supports regulatory compliance with EU law and harmonisation of safety data, and it incorporates the relevant standards and terminology.

[17] Commission Implementing Regulation (EU) No 520/2012

7.29 Question: What is the minimum information to be provided in the SUSAR reports?

374. **Answer:** The minimum information to be provided for an initial report of a SUSAR is defined in Annex III section 2.3. in the CTR.

375. There are specific fields in individual case safety reports (ICSRs) that must be completed for a valid SUSAR submission. These fields exist to collect the necessary data for appropriate safety reporting to ensure trial subject's safety. Sponsors shall ensure that SUSAR ICSRs are as complete as possible and shall communicate updates of reports to EVCTM timely, accurately and reliably.

376. Note data confidentiality is given by masking rules provided in the Addendum 2 to GVP Module IV[18]. Please chose the correct report type, study type, including EU CT number (or protocol code if CT only conducted outside EU/EEA), and date. The dates refer to the day on which the report was first received from the source and the date of receipt of the most recent information.

377. A case narrative, should be included presenting the information in a logical time sequence, in the chronology of the patient's experience including clinical course, therapeutic measures, outcome and follow-up information obtained, as well as information of unblinding (date and which treatment the individual subject has received; see Question 7.22. A clear statement has to be made on whether the SAR is unexpected or not (i.e. included in the RSI or not).

378. If relevant, reasons for downgrading or nullifying or amending an ICSR should be included in the case narrative

379. The narrative should include the assessment of expectedness of each SUSAR by the sponsor including the grounds for expectedness that is if the event is not listed in RSI or if the severity/seriousness has increased. Any relevant autopsy or post-mortem findings should also be summarised.

7.30 Question: How should SUSARs of combination IMPs be reported?

380. **Answer**: When the treatment of a CT subject includes a combination of IMPs, the investigator should assess for every SAR if any of the IMPs could have caused it on the basis of medical judgement and without discarding causality for one IMP by only the fact that the suspected AR has been previously described for other IMP in the combination treatment.

[18] GVP Module VI Addendum II – Masking of personal data in ICSRs for EudraVigilance

381. Where the causality indicated by the investigator is suspected for several IMPs, the sponsor should assess the expectedness of the SAR considering the RSIs of all suspected IMPs when separate RSIs for each IMP are used (see Question 7.18). If the AR is unexpected for at least one of the IMPs (according to the separate RSIs), the SAR should be considered unexpected and reported as a SUSAR.

382. Where RSIs of the combination IMP in the IB or SmPC is used (see Question 7.18), if a suspected SAR is not present in the RSI, it should be reported as a SUSAR. These SUSARs should be reported as related to the combination, unless it is – in rare cases – known to which IMP the SAR is related to.

7.31 Question: is the risk proportionate safety recording and reporting possible in clinical trials ? What adverse event reporting should be performed in low intervention trials?

383. **Answer:** Safety recording and reporting in all clinical trials including low intervention trials can be risk proportionately adapted to the specifics of the clinical trial design and purpose. This must be specified and thoroughly justified in the respective protocol (CTR Art 41). It is expected that such justification is supported by the risk assessment of the trial. For further guidance on the specific requirements, risk assessment and how to implement risk-adapted adverse event management in a clinical trial please see the respective recommendation paper on risk proportionate approaches in clinical trials[19] as well as ICH E6 R3[20]. The Sponsor should be aware that risk-based adaptation of adverse event management in a clinical trial protocol should not conflict with ICH E19 on a selective approach to safety data collection in specific late-stage pre-approval or post-approval clinical trials[21] .

7.32 Question: Should SUSARs or ARSs be submitted also to ethics committees?

384. **Answer:** Article 42 (SUSARs) and article 43 (ASRs) of the CTR[1] describe the submission to the MSs (= NCA and Ethics Committee) through the electronic database for Safety reporting (EudraVigilance for SUSARs, and CTIS for ASRs). Additional direct submissions from sponsors to ethics committees are not foreseen.

[19] Risk proportionate approaches in clinical trials
[20] ICH E6(R3)
[21] ICH guideline E19 Step 5 on optimisation of safety data collection

385. Ethics Committees can be involved in the assessment of safety information by the MSs, if that is the national decision of the individual MS.

7.33 Question: Should sponsors also send SUSARs or safety findings to investigators of a CT?

386. **Answer:** The sponsor should promptly notify all concerned investigators/institutions of findings that could adversely affect the safety of the subjects. New data that may affect the subject's safety, the benefit-risk balance of the trial or the willingness of subjects to continue in the clinical trial should be communicated to subjects and investigators in a timely manner (ICH E6 (R3), section 3.13.1).
The reporting of SUSARs to investigators/institutions should be in a manner that reflects the urgency of action required and should take into consideration the evolving knowledge of the safety profile of the product (ICH E6 (R3) section 3.13.2). Investigators should get informed of safety findings other than SUSARs that could affect the safety of the subjects accordingly.

387. SUSAR reporting to investigators is not mentioned in the CTR, however, it is of importance to inform investigators of changes to the safety profile, but not individual SUSAR reports. The safety information for investigators should be concise and practical. When possible, information on SUSARs should contain a list of relevant blinded SUSARs, together with a summary analysis of the updated safety profile, Risk Minimisation Measures, if applicable, and an updated benefit/risk evaluation for the ongoing CTs. The frequency of such periodic reports to the investigators should be based on the nature of the research or clinical development project, the volume of SUSARs generated, on the emerged issue(s) and especially if the safety profile changes.

388. SUSAR reports contain unblinded data that usually should not be sent to investigators. The submission of individual safety reports to investigators may be justified if unblinded data is relevant for the management/treatment of a serious adverse reaction (SAR).

389. In case of urgency and/or an impact on the benefit- risk the Dear Investigator Letter might be part of a risk mitigation measure and therefore qualify as notification of an urgent safety measure or an unexpected event impacting benefit-risk balance of the subjects. These notifications should be followed by a substantial modification when applicable, e.g. change of protocol. In case of information which is considered to have no/low impact on the safety of subjects, the submission can be done with the (next) substantial modification.

7.34 Question: When do requirements to record and report safety issues start and end for the investigator and the sponsor?

390. AEs, including SAEs, should be recorded by the investigator from the signature of informed consent to the end of the trial unless otherwise provided for in the protocol, e.g. if AE/SAE occur in subjects before investigational product administration they should only be considered and reported to the sponsor if required by the protocol.

391. SARs or follow-up information for a SAR that the investigator becomes aware of after the end of the trial should be reported to the sponsor (CTR Art 41 and Annex III).

392. The sponsor shall report all SUSARs from the beginning (see Question 10.2) to the end of the trial (see Question 10.12) and after the trial (CTR Art 42c), within timelines defined in Article 42 and Annex III of CTR.

393. While SAEs prior treatment often do not qualify as a SUSAR, these unfavourable medical events (ICH E6 (R3) may need to be notified to MSs e.g. as Unexpected Event (see Question 7.25 and 7.27).

394. Reporting after end of CT depends on the specific characteristics / profile, of the IMPs, like pharmacokinetics (PK), pharmacodynamics (PD), toxicity and population of the CT and should be specified in the protocol. Standard operating procedures should be followed to ensure compliance with the necessary quality standards at every stage of case documentation, data collection, validation, evaluation, archiving, reporting and follow-up.

7.35 Question: How should pregnancy during the trial, medication errors, use outside the protocol, including misuse or abuse of IMPs be reported?

395. **Answer:** All reports of exposure during pregnancy (of subject or the partner), medication errors, use outside the protocol, including misuse or abuse in relation to the IMP should be recorded by the investigator and notified to the sponsor. If the medication error, pregnancy, misuse or abuse resulted in a SAE which was deemed related and unexpected in accordance with the RSI, it should be reported as a SUSAR. The general rules of the CTR as well as the guidance given in this Q&A document apply with regard to the expedited reporting of SUSARs (including reporting only unexpected SARs) in case of adverse events/reactions, the submission of ASR and the implementation of risk minimisation measures. If the (collected) medication error, pregnancy, misuse or abuse did not result in an AE, it should still be reported to the sponsor by the investigator (CTR Annex III section 2.1 number 2). The sponsor should reflect on and put corrective measures in place to prevent these in the future.

7d ANNUAL SAFETY REPORTS

7.36 Question: What should be the content and format of ASRs?

396. **Answer**: The main objective of ASRs is to present a comprehensive, thoughtful annual review and evaluation of pertinent safety information collected during the reporting period related to an IMP, whether or not it is marketed, by: (1) examining whether the information obtained by the sponsor during the reporting period is in accordance with previous knowledge of the IMP's safety; (2) describing new safety issues that could have an impact on the protection of clinical trial subjects; (3) summarising the current understanding and management of identified and potential risks; and (4) providing an update on the status of the clinical investigation/development programme and study results[22].

[22] ICH guideline E2F on development safety update report, Section 1.2.

397. ASRs should be concise and provide information to assure regulators that sponsors are adequately monitoring and evaluating the evolving safety profile of the IMP. This includes also a description of the risk minimisation measures[23]. All safety issues discovered during the reporting period should be discussed in the text of the ASR; however, it should not be used to provide the initial notification of significant new safety information or provide the means by which new safety issues are detected[22].

398. The content and format of an ASR should comply with ICH guideline E2F on Development Safety Update Report[24] and CTR annex III section 3. Additional details are provided in dedicated questions:

- Question 7.37 regarding data protection,
- Question 7.39 regarding multiple active substances,
- Question 7.43 regarding reduced content of ASRs,
- Questions 7.47 and 7.48 regarding EU/EEA-specific information.

399. ASRs should be written in English, irrespective of the MSCs.

7.37 Question: How to comply with CTR article 43 section 3?

400. **Answer:** To comply with CTR article 43 section 3, stipulating that ASRs shall only contain aggregate and anonymised data, line listings should contain the information described in ICH guideline E2F section 3.7.2., except for the subject identification code, as this information is only pseudonymised but not anonymised.

401. Instead, individual cases should be identified by the Worldwide Unique Case Identification Number.

402. If no Worldwide Unique Case Identification Number has been assigned, this unavailability should be indicated, but the subject identification code should still not be provided.

403. If a sponsor is requested to provide further information on a certain case, the sponsor should be able to provide this information without revealing the subject identification code and thus rendering the provided data as anonymous to the member states.

[23] ICH guideline E2F on development safety update report, Appendix C
[24] ICH guideline E2F on development safety update report

7.38 Question: When and for how long should sponsors submit ASRs?

404. **Answer:** ASRs should be submitted to CTIS from the first authorisation of a clinical trial in any EU/EEA member state until the end of the last clinical trial conducted by the sponsor with the IMP in any EU/EEA member state. In cases where all clinical trials with the IMP have ended in all EU/EEA member states prior to the submission deadline of the ASR (i.e. 60 calendar days after the DLP), the submission of ASRs is not required[1]. Similarly, clinical trials that have ended (for the definition of the end of trial, reference should be made to Article 2(2)(26) of CTR) in all EU/EEA member states should not be linked in the submission of ASRs for ongoing clinical trials. The sponsor remains responsible for the ongoing safety evaluation of the investigational product(s) and for the managing of risks even after the end of the obligation to submit an ASR.

405. Submission of ASRs is not required in case the sponsor is conducting only a single trial with an IMP with a duration of less than one year.

406. Sponsors also need to submit ASRs for IMPs investigated in phase IV trials, low intervention trials and long-term follow-up trials.

407. ASRs should be submitted yearly within 60 days after the DLP[24].

408. When subjects from different clinical trials with various IMPs are being rolled over to long-term follow-up, for example in gene therapy trials, and the clinical development for the IMP is ongoing in the EU/EEA, the safety data of this IMP in all clinical trials, including safety data related to this IMP in the long-term follow-up study, should be covered within the ASRs of this IMP. If no clinical trials are ongoing with the respective IMP, the ASRs can be trial-specific for this one long-term follow-up clinical trial, covering safety data of all IMPs used or followed up, where no other clinical trial is ongoing.

7.39 Question: How should an ASR for combination including multidrug therapies be submitted?

409. **Answer:** In general, separate ASRs should be prepared for each active substance of a multi-drug therapy.

410. Notwithstanding the general rule, a single ASR should be prepared for fixed combination products, i.e. for products consisting of at least two active substances in a fixed dose that are administered in a single dosage form (see CTR and Art. 43 and ICH E2F section 2.5). This approach can also be followed for multi-drug combinations where it is intended to develop a fixed combination product. If the sponsor is also conducting clinical trials with individual component(s) of the fixed combination product, separate ASR should be submitted for each component, and relevant findings from each ASR should be summarised in section 8.5 of the other ASR(s).

411. Notwithstanding the general rule, a single ASR may be prepared for trials involving multi-drug therapy, i.e. combinations of drugs that are not fixed and not intended to be developed as a fixed combination product (ICH E2F section 2.5)[24]. It is assumed that this approach is generally only applicable to trial-specific ASRs prepared by non-commercial sponsors.

7.40 Question: What is the Development International Birth Date, how is it defined, and what is it used for?

412. **Answer:** The DIBD is used to determine the start of the annual period for the ASR. This date is the date of the sponsor's first authorisation to conduct the first clinical trial with the IMP in any country worldwide (ICH E2F section 2.2).

413. The start of the annual period for the ASR is the month and date of the DIBD (e.g. when the DIBD is December 6th, each ASR period should cover the period from December 6th to December 5th of the following year). When the sponsor's first clinical trial is conducted in a country without a formal authorisation process, the sponsor should designate an appropriate date linked to the commencement of the first clinical trial (ICH E2F section 2.2).

414. To aid harmonisation, it is strongly recommended that the DIBD is indicated by the sponsor within the ASR (ICH E2F section 3.1).

415. The data lock point (DLP) of an ASR should be the last day of the one-year reporting period. If desired by the sponsor, the DLP can be designated as the last day of the month prior to the month of the DIBD (e.g. when the DIBD is December 6th, the DLP could be set to November 30th of the following year). ASRs should be submitted within 60 days after their DLP (ICH E2F section 2.2).

7.41 Question: Can the DLP of an ASR be aligned with the DLP of the PSUR/PBRER?

416. **Answer:** When clinical development of a medicinal product continues following a marketing approval in any country worldwide, Periodic Safety Update Reports (PSURs), Periodic Benefit-Risk Evaluation Reports (PBRERs) and ASRs should be submitted as specified by national or regional laws or regulations (ICH E2F section 2.2).

417. If desired by the sponsor, an ASR can be prepared based on the PSUR International Birth Date (IBD) so that the ASR and the PSUR can be synchronised (ICH E2F section 2.2).

418. In synchronising the data lock points for the ASR and PSUR, the period covered by the next ASR should be no longer than one year (ICH E2F section 2.2).

419. The data lock points for PSURs are available on the EMA website (Excel file "List of European Union reference dates (EURD) and frequency of submission of periodic safety update reports (PSURs)", column E "DLP")[25].

7.42 Question: What date should be the start date and the end date of the ASR reporting period for investigator-initiated/academic clinical trials (not by the MAH (marketing authorisation holder))?

420. **Answer:** Where an ASR is submitted for one clinical trial, the start date of the ASR reporting period should be the DIBD, which is the date of the first authorisation of this specific clinical trial. Alternatively, the sponsor may follow the approach described in question 7.41.

421. The end date of the ASR reporting period should be the DLP which is the day before the start of the reporting period i.e. when the start date of the reporting period is December 6th, the DLP is December 5th of the next year.

422. None of the ASR reporting periods should be longer than one year.

[25] Available at List of European Union reference dates (EURD) and frequency of submission of periodic safety update reports (PSURs).

7.43 Question: Is a full ICH guideline E2F-compliant ASR always required? When could a simplified ASR be submitted?

423. **Answer:** The content of an ASR may be simplified in the following cases:

424. The ASR is prepared by a non-commercial sponsor for a single clinical trial, where the IMPs are authorised (for the definition of authorised investigational medicinal products, please refer to CTR article 2 paragraph 2 number 9) and where section 4.8 of the SmPC is used as RSI (for further information, please refer to CTR annex I section E number 28). Detailed instructions on what information is expected and what may be omitted in the above cases are available on the CTCG website[26].

425. The ASR is prepared by a non-commercial sponsor for one or more clinical trials, and none of the trials has started yet (for the definition of the start of a clinical trial, please refer to CTR article 2 paragraph 2 number 25), or all clinical trials have been halted or suspended for more than one year without administration of the relevant investigational medicinal product(s) (for the definition of temporary halt and suspension of a clinical trial, please refer to CTR article 2 paragraph 2 numbers 27 and 28). In this case, a document stating that the trials have not started yet or have been halted or suspended for more than one year without administration of the investigational medicinal product(s), providing the results of the literature review (ICH guideline E2F section 3.13)[24], providing the results of the IB/SmPC review (CTR article 55 paragraph 2, ICH guideline E2F section 3.4) and providing a statement on the benefit-risk ratio of the trial and the necessity to update the protocol or the subject information sheet/ informed consent form (ICH guideline E2F section 3.20 is sufficient.

426. In all other cases, including cases where the above criteria are only partially met, a full ICH guideline E2F-compliant ASR should be submitted.

[26] CTCG Key documents list: Guidance, clinical trial safety, simplified template of Annual Safety Report.

7.44 Question: When a non-commercial sponsor runs several clinical trials with the same IMP or if different non-commercial sponsors run independent clinical trials with the same non-authorised IMP, is one consolidated ASR needed?

427. **Answer:** For IMPs without a MA and where the clinical trial is supported by the developer, it is recommended that the developing company writes a single ASR per IMP covering all the safety information from the non-commercial clinical trials. See also ICH E2F section 2.4.2. This implies that SARs line listings, cumulative summary tabulation of SAEs, and the region-specific information which includes the cumulative summary tabulation of SARs, and list of patients who died or dropped out during the clinical trial should include also the data from the non-commercial sponsors. The ASR should clearly specify which data is included and from which clinical trials it originates, including the EU CT number. Submission of one single ASR on the IMP level is highly recommended if the same IMP (IMP combination) is used in several CTs. However, the member states concerned accept a trial-specific ASR if justified.

428. The submission of an ASR on the clinical trial level is encouraged in case the non-commercial sponsor uses several IMPs within the clinical trial where not all of them have support from the MAH, as thus safety information from those without support would not be incorporated into any ASR.

7.45 Question: Are ASRs required for placebos, active comparators and AxMPs?

429. **Answer:** ASRs are required for all IMPs other than placebo (CTR article 43 paragraph 1), whereas IMPs are all medicinal products being tested or used as reference/ comparator (CTR article 2 paragraph 2 number 5).

430. Separate ASRs are not required for comparators, but relevant safety information should be included in the appropriate sections of the ASRs for the test investigational medicinal product (ICH guideline E2F section 1.3, among others).

431. ASRs are not required for AxMPs, but relevant safety information and any other safety issues should be included in the appropriate sections of the ASRs for the test IMP.

7.46 Question: What information is required in the Cumulative Summary Tabulations of SAEs?

432. **Answer:** To enhance the utility of section 7.3 of the ASR, titled 'Cumulative Summary Tabulations of Serious Adverse Events' and in addition to the requirements specified in ICH guideline E2F, this section should also include the absolute numbers of patients treated as per the column headings, of the Cumulative Tabulation of SAEs. This information can be provided either in the text of the ASR or, preferably, directly within the table itself (see table below, adapted from table 6 of the ICH E2F guideline).

Cumulative summary tabulation of SAEs (example)

Cumulative Summary Tabulation of Serious Adverse Events (SAEs)

System Organ Class	**Total up to 31-Dec-09**			
Preferred Term	[Study drug]	Blinded	Active comparator	Placebo
	n=100	n=1	n=98	n=15
Investigations	**18**	**4**	7	**2**
Alanine aminotransferase increased	9	2	4	1
Aspartate aminotransferase increased	9	2	3	1
Nervous System Disorder	**2**	**2**	**4**	7
Syncope	2	2	4	7

433. If feasible/possible the sponsor should also calculate patient-years of treatment. This information may be especially useful in the interpretation of data when there are substantial differences in time of exposure between subjects randomised to the tested product(s) and comparator(s).

434. A single Cumulative Summary Tabulation of SAEs should be presented for all clinical trials covered in the ASR. A sponsor may also include additional Cumulative Summary Tabulations of SAEs presented for separate populations or indications; however, these must be in addition to the single table covering all trials.

435. In a combined study, information related to SAEs occurring with the IVD or the IMD can be reported. For further details on IVD or MD please see "COMBINE Safety Reporting Sponsor Guide" and Q7.52 .

7.47 Question: What 'Region-Specific Information' is required in the ASR in the EU/EEA?

436. **Answer:** The following EU/EEA-specific information is required in accordance with ICH guideline E2F section 3.16:

- Cumulative summary tabulation of SARs: The format should be the same as for the cumulative summary tabulation of SAEs (ICH E2F section 3.16 and table 6) and Question 7.46.
- List of subjects who died during the reporting period: The listing should include information on case number, assigned treatment (could still be blinded), causality to the study drug, and cause of death.

437. List of subjects who dropped out of clinical trials in association with an adverse event: The listing should include information on assigned treatment (could still be blinded), causality to the study drug, and cause of drop-out, i.e. the PT of the AE triggering the drop-out.

- Safety signals: The outcome of the safety signal review process during the ASR reporting period should be outlined. Potential new safety signals that were identified should be listed including a brief description of the signal, the date when the sponsor became aware of the signal, the status of the signal at the end of the reporting interval (closed or ongoing), the date when the signal was closed, if applicable, the source of the signal, a brief summary of the key data, plans for further evaluation and actions taken (i.e. proposed risk mitigation strategies). The outcome of the safety review should be provided in a tabular format. An example of such a table is presented below (Table 5 and Appendix C of ICH E2C (R2)) [27].

Table format example for the outcome of the safety review in the ASR.

Signal term	Date detected	Status (ongoing or closed)	Date closed (for closed signals)	Source of signal	Reason for evaluation & summary of key data	Method of signal evaluation	Action(s) taken or planned
Anaemia	04 March 2015	Ongoing	NA	Single serious case	The signal consisted of a single report of....	Individual case analysis; Review of relevant scientific literature. Reassessment of preclinical and clinical development safety data.	Review at the next Safety Review Team meeting

438. It is acknowledged that signal evaluation for clinical trials may not always be possible or appropriate, in which case a justification for not including this information should be provided instead.

[27] ICH E2C (R2) Periodic benefit-risk evaluation report - Scientific guideline.

439. The term 'region-specific' indicates that the above information is required in the EU/EEA, but not that only data collected in the EU/EEA should be considered.

7.48 Question: What RSI should be used for the ASR?

440. **Answer:** The RSI in effect at the start of the reporting period shall serve as RSI during the reporting period.

441. For the purposes of ASRs, the RSI prepared on the basis of the data available at the DLP of the previous ASR may be considered the RSI in effect at the start of the reporting period even if it was formally approved after the start of the reporting period.

7.49 Question: What should a sponsor be aware of during and after ASR submission to CTIS?

442. Sponsors are asked to check that the active substance(s) are completed in the structured fields when submitting an ASR in CTIS. Note that only the active substances for which the ASR is submitted should be ticked.

443. An ASR document submitted cannot be changed or withdrawn. In case there is a critical mistake or correction to be done, please check with the national contact point of the assigned Safet Assessing Member State (saMS) of this ASR (after the saMS selection has been completed). During an RFI process, an updated version of the ASR document can be uploaded.

444. Sponsors should be aware that the RFI on the ASR does not trigger any separate notice or alert in CTIS. One needs to actively monitor the ASR module.

445. Sponsors should be aware that the RFI on the so-called ad-hoc workflow does not trigger any separate notice or alert in CTIS. One needs to actively monitor the separate RFI tab for incoming RFIs on surveillance issues. These RFIs can be sent anytime and are not linked to any other application.

446. Sponsors should adhere to the deadlines given in the RFI raised within an ASR or ad-hoc workflow. In case a sponsor cannot meet the deadline, the national contact point of the MS raising the RFI should be contacted to discuss submitting the response after the deadline.

447. In cases where a single ASR is prepared for a high number of clinical trials and/or a high number of investigational medical products must be selected for submission, or for clinical trials with different sponsors, co-sponsors or CROs, the submitter may technically not be able to select all clinical trials covered by the ASR. In such case, multiple ASR submissions of the same ASR should be avoided, and the submitter is recommended to submit only one ASR to CTIS and select as many CTs covered by the ASR as possible: minimum should be the CT with the lowest EU CT number and the one with the highest EU CT number. The cover letter should list all CTs covered by the ASR and all member states concerned with the ASR. In addition, the co-sponsor (with equal responsibilities to safety surveillance and submission of the ASR) or the sponsor with different Org ID of 'the same sponsor', or the different sponsor (like non-commercial, where safety of the CT is covered within the developer's ASR, see Question 7.43) should submit a document as 'ASR' in which they refer to the ASR submission and the ASR submission ID of the co-sponsor or other organization ID.

Note: This recommendation is valid till the technical limitation in CTIS has been solved.

448. For information, if applicable, an invoice for ASR assessment may be sent by MSCs and/or by the saMS, the latter may not be a MSC (nor the member state named in CTIS as saMS). The reason for this is that the responsibility of a saMS is per active substance, regardless of whether a saMS is an MSC in a clinical trial (Commission Implementing Regulation (EU) 2022/20 article 2 section 2 letter c).

7.50 Question: Are there additional reporting requirements related to safety reporting under Chapter VII of the CTR to the member states, such as the submission of monthly, quarterly or half-yearly safety reports or SUSAR line listings?

449. **Answer**: Documents covering shorter than annual reporting periods, such as monthly, quarterly or half-yearly safety reports or SUSAR line listings, should only be submitted upon exceptional request by the RMS or the saMS for a clinical trial or IMP, respectively.

450. If they are requested, the sponsor will receive an RFI in the ad-hoc workflow to be able to submit the requested documents.

7.51 Question: How long does the ASR assessment process last?

451. The assessment shall be completed within 42 days or in the case of an RFI within 84 days after the submission of the ASR[28]. This includes the time that sponsors have to respond to an RFI, which is usually 14 days. In practice, procedures may last longer than 84 days due to several reasons, including multiple RFIs.

7.52 Question: What information is required in the ASR in case of combined studies?

452. The Annual Safety Report should summarize the relevance of information concerning SAEs related to investigational medical devices (IMDs) or investigational in vitro diagnostic medical devices (IIVDs) and device deficiencies (DDs) that theoretically could lead to SAEs observed during the reporting period in the "evaluation of risks" section or where appropriate in the section "benefit-risk considerations". For more guidance also view "COMBINE Safety Reporting Sponsor Guide" [*Reference will be added when available*].

453. If actions for safety reasons have been taken during the ASR reporting period due to SAEs related to investigational medical devices (IMDs) or investigational in vitro diagnostic medical devices (IIVDs), this should also be mentioned in the appropriate section of the ASR.

7.53 Question: The Clinical Trials Regulation (EU) No 536/2014 mentions only some aspects specific to radiopharmaceuticals, it does not specify any documentation regarding exposure to ionising radiation in clinical trials. Does that mean that sponsors are no longer expected to present such information in the protocol and/or application?

454. **Answer:** No, the sponsor is expected to include information on exposure to ionising radiation in the protocol in line with CTR Annex I, section D to allow

[28] Commission Implementing Regulation (EU) 2022/20; article 7 paragraph 1 letter c.

assessment of the benefits and risks of the clinical trial, see CTR article 6 paragraph 1 (b)(i) and (ii).

455. The specifics of the information to be included will depend on the situation of exposure (see below).

456. Exposure to ionising radiation in clinical trials can be divided into two main situations – radiodiagnostic procedures and radiotherapeutic procedures.

- **Radiodiagnostic procedures** include both radiological procedures and nuclear medicine procedures. In the diagnostic situation, the ionising radiation exposure is a consequence of the procedure, and the risk to the trial participants therefore needs to be justified in the clinical trial protocol. The ALARA principle prevails, i.e., the exposure should be maintained as low as reasonably achievable without compromising the diagnostic imaging quality. In line with ICRP[29] criteria and CTR article 6, paragraph 1(b)(ii)) the risks and inconveniences for the trial subjects regarding interventions involving radiation exposure should be justified in comparison to the exposure involved in the procedures used in normal clinical practice. When discussing the benefits and risks in the protocol, sponsors should describe the following in order to provide maximum clarity minimising then number of Request For Information (RFI) considerations based on lack of information, possibly in a protocol appendix if Member State-differences in national Standard of Care are envisaged: risk category of trial participants according to criteria, radiodiagnostic trial procedures, maximum effective dose per procedure (mSv), number of procedures/trial participant/year, and estimated number of additional radiodiagnostic procedures/trial participant/year compared to normal clinical practice for the same indication.

- **Radiotherapeutic procedures** can be further divided into external beam radiotherapy, brachy therapy and systemic radiation therapies with radiopharmaceuticals. Only the latter is subject to regulation by the CTR. With therapeutic radiopharmaceuticals, the radiation dose absorbed by tissues and organs is the mechanism of action through which efficacy of the therapy is achieved, but it may also cause toxicity. The risks of both ineffective treatment due to insufficient absorbed dose to the target lesions and risks of severe/irreversible long-term toxicity due to excessive absorbed dose to risk organs, need to be monitored and mitigated during the trial to optimise the benefits and risks for the individual trial participant, in line with article 6.1(b)(i) and (ii) of the CTR. The AHASA principle prevails – the absorbed radiation dose to the target tissue(s) should be as high as safely attainable, i.e., preventing severe and/or irreversible long- term toxicity while at the same time maintaining a high likelihood of efficacy. In addition to the benefit/risk section, sponsors should describe dosimetric procedures in the protocol, as well as target absorbed doses (in Gy) to tumour lesions and dose limits to risk organs based on the best available evidence as well

[29] ICRP, 1992. Radiological Protection in Biomedical Research. ICRP Publication 62. Ann. ICRP 22 (3)

as any necessary adaptations of the treatment plan e.g. due to combination therapy that may affect the biological effect of the radiation therapy.

457. In order to assess the benefits and risks, any deviations from the principles above should be justified in the protocol.

AUTHORISATION OF MANUFACTURING AND IMPORTATION OF IMPS

8.1. Question: A clinical trial with an investigational medicinal product (IMP) which is an *officinal* or *magistral* formula falls within the scope of the Clinical Trials Regulation. What does this mean for the requirements as regards manufacturing authorisation?

458. **Answer**: Chapter IX of the Clinical Trials Regulation applies to the manufacturing and import of the investigational medicinal product (IMP), which is subject to the holding of an authorisation. However, article 61 (5) of the Regulation provides for exceptions where an authorisation is not required under certain conditions. The conditions to benefit for these exemptions are that the IMPs having undergone one of the processes referred to in Article 61(5)(a), (b) and (c)[1] shall be used for the same clinical trial for which the process was done, the IMPs should be used in the same Member State where the process was done, or in an other Member State in which the same trial is being conducted and if this Member State allows it, but the IMPs are not necessarily used in the same hospitals, health centres or clinics where the preparation of the IMP was done.

459. The preparation of investigational medicinal products with an *officinal* or *magistral* formula does not require a manufacturing authorisation where this process is carried out in hospitals, health centres or clinics for exclusive use in these same places taking part in the same clinical trial in the same Member State.

460. In such cases Member States shall set up appropriate and proportionate requirements, including regular inspections, to ensure subject safety and reliability and robustness of the data generated in the clinical trial.

[1] re-labelling or repackaging, preparation of radiopharmaceuticals used as diagnostic investigational medicinal products and preparation of medicinal products referred to in points (1) and (2) of Article 3 of Directive 2001/83/EC for use as investigational medicinal products)

8.2. Question: What are the regulatory requirements for the preparation and labelling of radiopharmaceuticals used as diagnostic investigational medicinal products?

461. **Answer**: the preparation of radiopharmaceuticals used as diagnostic investigational medicinal products do not require a manufacturing authorisation where this process is carried out in hospitals, health centres or clinics for exclusive use in these same places taking part in the same clinical trial in the same Member State[2].

462. In line with article 68 of the CTR, radiopharmaceuticals used as IMP or AxMP do not need to adhere to the labelling requirements in article 66 and 67. Nevertheless, the product must be labelled appropriately in order to ensure subject safety and reliability and robustness of the data generated.

8.3. Question: What documentation is required in the application for the authorisation of a clinical trial relating to compliance with good manufacturing practice (GMP) for an investigational medicinal product?

463. **Answer:** The documentation required to show compliance of the IMP and AxMP with GMP is outlined in Chapter IX and Annex 1 section F of the Clinical Trials Regulation:

a) For IMPs authorised in the EU (even if not manufactured in the EU) no documentation is required.
b) For IMPSs that are not authorised in the EU and do not have a marketing authorisation from a third country that is party to ICH, and are not manufactured in the EU, an authorisation referred to in article 61(1) and a QP declaration of GMP equivalence is required. In the latter case, if a Mutual recognition Agreement (MRA) covering also clinical trials is in place with the particular country, the latter declaration is not required if the MRA provides for GMP equivalence already.

[2] Article 61 (5) of Regulation (EU) No 536/2014

c) In all other cases, an authorisation according to article 61 of the Clinical trial Regulation) is required.

Information regarding the GMP compliance of the active pharmaceutical ingredients is not required by the CTR (and can therefore not be required by the Member States Concerned.)

8.4. Question: Are there specific requirements for the labelling of cryopreserved medicinal products?

464. **Answer**: CTR Annex VI labelling requirements should be fulfilled. However, considering the cryopreserved nature of certain IMPs (such as deep cryopreserved products) and the possible risks due to re-labelling, it may be proportionate to omit CT reference code and subject identification number from the immediate packaging label, if adequately justified in the IMPDQ and label section or in the protocol. In these cases, information should be provided on how subject safety, traceability as well as reliability and robustness of the data generated in the clinical trial are ensured by other measures, e.g. by providing the respective information on secondary packaging or accompanying letter.

465. For ATMP, traceability should be verified in accordance with article 15 of Regulation (EC) No 1394/2007.

466. Digital stand-alone solutions (such as a QR-code only system) for primary packaging labels are not endorsed.

“INFORMED CONSENT” AND OTHER SUBSTANTIAL REQUIREMENTS FOR CONDUCTING CLINICAL TRIALS

9.1. Question: What is meant by ‘compensation for participation’ in a trial involving incapacitated subjects, minors and pregnant and breast feeding women?

467. **Answer:** according to **a**rticle 31(1)(d), article 32(1)(d) and article 33(d) of the Clinical Trials Regulation no incentives or financial inducements, other than compensation for the participation in the clinical trial, are to be given to incapacitated subjects, legal representatives, minors and pregnant and breast feeding women. This compensation should not cover more than expenses and loss of earning, directly related to the participation in the clinical trials. Examples of expenses directly related to the participation in the clinical trials are travel costs for the participating subject and the legally designated representative (if applicable) or (if applicable) the person accompanying the subject, costs for accommodation, or additional costs due to participation in the clinical trial collected by the subjects’ health insurance (compulsory patient contributions/own risk). The information on compensation shall be submitted in the application dossier (CTR Annex I, P(70)) and as such is subject to assessment by Member States. A small token of appreciation is not considered an incentive, but needs to be explicitly allowed by the ethics committee.

9.2. Question: When can the obligation to ensure the compensation of a damage of article 76 stop?

468. **Answer:** According to article 76 of the Clinical Trials Regulation, a clinical trial may be undertaken only if provision has been made for ensuring that a subject is compensated for any damage suffered which resulted from participation in a clinical trial. The sponsor shall

make use of any appropriate arrangements existing in the Member State concerned (be it an insurance or guarantee or a similar arrangement).

469. There are no specific Union provisions on when the obligation of providing compensation for damage suffered in a clinical trial should stop.

470. However, the purpose of article 76 of the Clinical Trials Regulation is to ensure that a clinical trial subject will obtain compensation for damages caused by participating in the clinical trial independently of the financial capacity of the investigator/sponsor. Article 76 stresses also that any damage should be compensated. In view of this purpose of the provision the sponsor should ensure that the arrangements ensuring the compensation of damage are in place for the period in which such damages can arise and lawfully be claimed by the clinical trials subject.

471. The obligation to ensure the compensation of a damage proposed by the sponsor should be subject to assessment by each Member State according with national law.

9.3. Question: What is meant by "the informed consent shall be documented" (article 29(1) of the Clinical Trials Regulation)?

472. **Answer:** Informed consent should be written, dated and signed by the person performing the interview and by the subject or the legally designated representative in cases when the subject is unable to give informed consent. Appropriate alternative means can be used to give and record informed consent in cases when the subject is unable to write. This should be done in the presence of at least one impartial witness. Details of the process shall be recorded and the informed consent form shall be kept as evidence.

9.4. Question: What is meant by “his or her express informed consent shall be obtained before the subject can continue to participate in the Clinical Trial” (article 32(3) of the Clinical Trials Regulation)?

473. As soon as a minor participating in a clinical trial reaches the age of legal competence (as defined in national law, which varies between 12 and 18 years), he/she needs to express his/her informed consent to continuing participation in the study by signing the informed consent form, after having been properly informed, in line with the requirements of the Clinical Trials Regulation.

For this purpose, the investigator should provide the trial subject with an information sheet and informed consent form appropriate for the age of legal competence. The information sheet and informed consent form may be provided prior to the trial subject's attainment of legal competence (e.g. during the last visit before reaching the age of legal competence), which would be an opportunity to clarify any questions. The interview that is required under Article 29(2)(c) of the Clinical Trials Regulation should take place once the trial subject has reached the age of legal competence. The new informed consent shall be in writing, dated and signed by the person performing the interview and by the trial subject in accordance with Article 29(1) of the Clinical Trials Regulation (unless an exception as provided for in Article 29(1) applies or when the requirements listed under Article 30 are met). The information process and signature of the informed consent should take place before any further clinical trial protocol-related actions are taken, e.g., at the next study visit or, depending on the delay between the age of legal competence and the next visit, at a visit organised for this purpose, when practically feasible, without putting an undue burden on the trial subject. This process should not result in a treatment or care interruption, which would be detrimental to the trial subject.

The procedure of obtaining the new written informed consent is independent of the medium chosen (paper-based or electronic) and it should be described in the trial protocol, the informed consent form and the information sheet (including maximum timelines accepted for the additional visit, if applicable) and shall be authorised by the Member States concerned (MSC).

9.5. Could the provisions in article 35 on "deferred informed consent" be used when there is not sufficient time to obtain informed consent, even though the objective of the trial is not to study a medical emergency situation?

474. **Answer**: No. The medical condition studied must be directly related to the emergency situation. There must always be a medically justified need to perform the first intervention within a short time frame in relation to the emergency situation,

9.6. Is a trial with a mixed subject population, where some subjects consent prior to inclusion and others after the first trial-specific intervention, still an emergency trial?

475. **Answer**: Yes. Article 35 is applicable if the protocol provides for inclusion of any subjects without prior informed consent in a medical emergency. If prior informed consent is possible to obtain from some subjects (or their legal representatives) but not from others, the protocol should clearly explain the reasons and justification for this mixed population. It should not be possible to perform the clinical trial as an emergency trial if informed consent can be obtained from ALL subjects or from his/her legally designated representative.

9.7. Is the use of placebo allowed in a clinical trial in an emergency situation?

476. **Answer**: Only trials in emergency situations where scientific grounds exist to expect a potential for a direct clinically relevant benefit for participating subjects as defined in Article 35.1 (b) can be carried out. Where comparison is performed between one or several treatment arms receiving active treatment vs. a control arm, no subject should receive inferior treatment compared to normal clinical practice in a Member State, why placebo-treatment should be restricted to situations where it is added to such standard of care.

9.8. What is meant by a subject's prior objection to participate in an emergency situation trial?

477. **Answer**: If objections to trial participation can be identified among personal belongings of the subject or are known to the investigator responsible for subject inclusion, e.g. found to be clearly stated in the medical record, national registries (if available), such concerns should be respected.

9.9. When should informed consent be sought in an emergency situation trial and what happens to the data obtained if a subject dies before informed consent has been given or if the subject or his/her legally designated representative does not agree to provide informed consent?

478. **Answer**: Informed consent must be sought without undue delay and these efforts must be duly documented in the medical record of the subject, e.g. clearly mentioned in the source documents of the recovery of the conscience and, where relevant, describing efforts to reach a legal representative.

479. In situations when the subject dies before any informed consent has been provided, and the data already gathered has been collected in agreement with Article 35, the data should remain in the trial. In situations when the subject or his/her legally designated representative do not consent but instead disagree with continued trial participation, no further research data can be collected. The data already provided will be kept in the trial if all prerequisites in Article 35 have been fulfilled, including that efforts seeking informed consent have not been unduly delayed.

9.10. Are secondary objectives and corresponding endpoints acceptable even if they are without any expected direct clinically relevant benefit for the subject but instead could provide a group benefit for patients?

480. **Answer**: The main objective of an emergency situation trial should always meet the legal requirement for a scientific basis for the potential to have a direct clinically relevant benefit for the subject.

481. This would make it possible to gather pharmacokinetic and biomarker data, as long as such secondary or exploratory endpoints do not pose more than minimal risk and burden for subjects.

START, END, TEMPORARY HALT, AND EARLY TERMINATION OF A CLINICAL TRIAL (Articles 36-38 of the CTR)

10.1. Overview of notification requirements on status of clinical trials - CTR articles 36 to 38 and 52 to 54

(Abbreviations please see Annex VI)

Event	CTIS notification	Legal basis in the CTR	Further guidance
Start			
Start of a CT in a MS	SoT	Art. 36 (1)	See questions 10.2 and 10.4.
Recruitment			
First visit of the first subject in a MS	SoR	Art. 36 (2)	See questions 10.3 and 10.4.
End of recruitment in a MS	EoR	Art. 36 (3)	See question 10.4.
Temporary halt of recruitment in a MS	EoR	Art. 36 (3)	See questions 10.4 and 10.5.
Partial temporary halt of recruitment in a MS	-	-	E.g. temporary halt of recruitment into one or more, but not all, treatment arms, treatment groups or subprotocols. For CCTs, see further guidance on the CTCG website ([1]).
Restart of recruitment in a MS	RoR	Art. 36 (3) sentence 3	See questions 10.4 and 10.5.
Temporary halt			
Temporary halt of a CT in a MS for reasons not affecting the benefit-risk balance	TH	Art. 37 (5)	See questions 10.5, 10.6 and 10.9. CTR art. 37 (5) refers to a TH in all MSs instead of a TH in one MS, but TH notifications must

[1] https://www.hma.eu/about-hma/working-groups/clinical-trials-coordination-group.html.

			be made in CTIS for each MS individually.
Partial temporary halt of a CT in a MS for reasons not affecting the benefit-risk balance	-	-	E.g. temporary halt of one or more, but not all, treatment arms, treatment groups or subprotocols. For CCTs, see further guidance on the CTCG website ([2]).
Restart of a CT halted in a MS for reasons not affecting the benefit-risk balance	RoT	Art. 37 (6)	See questions 10.2 and 10.5. A RoT is generally accompanied by an RoR.
Restart of a part of a CT halted in a MS for reasons not affecting the benefit-risk balance	-	-	E.g. following a temporary halt of one or more, but not all, treatment arms, treatment groups or subprotocols. For CCTs, see further guidance on the CTCG website ([3]).
Temporary halt of a CT for reasons of a change of the benefit-risk balance	TH	Art. 38 (1)	See questions 10.5, 10.6 and 10.9.
Partial temporary halt of a CT for reasons of a change of the benefit-risk balance	USM	Art. 54 (2)	E.g. temporary halt of one or more, but not all, treatment arms, treatment groups or subprotocols. See questions 10.5., 10.6 and 10.9. For CCTs, see further guidance on the CTCG website ([4]).
Restart of a CT or parts of it halted for reasons of a change of the benefit-risk balance	SM	Art. 38 (2)	See question 10.5. The SM is generally followed by a RoT and RoR after its authorisation (except in the case of partial temporary halts).
Early termination			

[2] https://www.hma.eu/about-hma/working-groups/clinical-trials-coordination-group.html.
[3] https://www.hma.eu/about-hma/working-groups/clinical-trials-coordination-group.html.
[4] https://www.hma.eu/about-hma/working-groups/clinical-trials-coordination-group.html.

Early termination of a CT in a MS for reasons not affecting the benefit-risk balance	ET	Art. 37 (7)	See question 10.12. Examples for reasons generally not affecting the benefit-risk balance are also provided there.
Partial early termination of a CT in a MS for reasons not affecting the benefit-risk balance	-	-	E.g. early termination of one or more, but not all, treatment arms, treatment groups or subprotocols. For CCTs, see further guidance on the CTCG website ([5]).
Decision to early terminate a CT or parts of it for reasons not affecting the benefit-risk balance	-	-	See questions 10.6. and 10.12.
Early termination of a CT for reasons of a change of the benefit-risk balance	ET	Art. 38 (1)	See question 10.12. Examples for reasons generally affecting the benefit-risk balance are also provided there.
Partial early termination of a CT in a MS for reasons of a change of the benefit-risk balance	-	-	E.g. early termination of one or more, but not all, treatment arms, treatment groups or subprotocols. For CCTs, see further guidance on the CTCG website ([6]).
Decision to early terminate a CT or parts of it for reasons of a change of the benefit-risk balance	USM	Art. 54 (2)	See questions 10.6 and 10.12. Examples for reasons generally affecting the benefit-risk balance are also provided there. For CCTs, see further guidance on the CTCG website ([7]).
End			

[5] https://www.hma.eu/about-hma/working-groups/clinical-trials-coordination-group.html.
[6] https://www.hma.eu/about-hma/working-groups/clinical-trials-coordination-group.html.
[7] https://www.hma.eu/about-hma/working-groups/clinical-trials-coordination-group.html.

End of a CT in a MS	EoT	Art. 37 (1)	See question 10.14.
End of a CT in all MSs	N/A – automatically with last EoT notification	Art. 37 (2)	See question 10.14.
End of a CT in all MS and third countries	GEOT	Art. 37 (3)	See question 10.14.
Other			
Unexpected events which affect the benefit-risk balance (except SUSARs)	UE	Art. 53 (1)	See question 7.25. Potential sources of UEs: non-clinical data, product quality issues, trial-procedure related events affecting the benefit-risk balance etc. SUSARs are unexpected events, but are not subject to UE reporting requirements, as specific reporting requirements apply to SUSARs (see CTR art. 42).
Unexpected events where the sponsor or the investigator has taken urgent safety measures	USM	Art. 54 (2)	See questions 10.5, 10.9 and 10.11. Examples for reasons generally affecting the benefit-risk balance are also provided there.
Serious breach	SB	Art. 52 (1)	See the Guideline for the notification of serious breaches of Regulation (EU) No 536/2014 or the clinical trial protocol (EMA/698382/2021).
Inspection reports of third country authorities	TI	Art. 53 (2)	See question 10.14.
Summary of results of an intermediate analysis	Sum	Art. 37 (8)	See question 6.6.
Summary of results	Sum	Art. 37 (4)	See question 10.13.

10.2. Question: How is the "start of a clinical trial" defined?

482. **Answer:** Article 2 (25) of the Clinical Trials Regulation defines the "start of the clinical trial", as "*the first act of recruitment of a potential subject for a specific clinical trial, unless defined differently in the protocol*". Therefore, unless differently defined in the protocol, the date of start of the clinical trial is the date when recruitment for the clinical trial is opened in a Member State concerned. The first act of recruitment shall be identified by the sponsor in the recruitment strategy, as required per CTR Annex I (point K.59). It could be, for example, the date of initiation of the clinical trial in the first site or the date when the first study specific advertisement is published. In some cases, the sponsor may define in the protocol the start of the trial differently than first act of recruitment. This may be justified e.g. for phase I clinical trials. However, in any case the clinical trial cannot neither start earlier than the authorisation date nor later than the first visit of the first subject.

483. The start of the Clinical Trial should be filled in for each MSC through the “start trial” button in the notifications tab of the trial.

10.3. Question: What should be considered as the date of the first visit of the first subject?

484. **Answer:** The date of the first visit of the first subject should be the date the first subject or his/her legally designated representative signs his/her first informed consent to participate in activities that are protocol directed interventions.

485. The date of the first visit of the first subject, as required by article 36(2), should be filled in for each MSC through the “start recruitment” button in the notifications tab of the trial.

10.4. Question: Which dates does the sponsor need to notify to the Member State concerned?

486. **Answer:** The sponsor should notify each MSC of the start of a clinical trial in relation to that Member State through the EU portal, within 15 days from the start of the clinical trial in relation to that Member State.

487. Additionally, the sponsor shall notify each MSC of the first visit of the first subject in relation to that MSC through the EU portal, within 15 days from the first visit of the first subject in relation to that MSC as laid out in article 36 (1-2) of the Clinical Trials Regulation.

488. Moreover, according to article 36(3) of the Clinical Trials Regulation, the sponsor shall notify each MSC of the end of the recruitment of subjects for a clinical trial in that MSC through the EU portal, within 15 days from the end of the recruitment of subjects. In cases when recruitment is re-started sponsors should notify MSC through the portal within 15 days of the re-start in each MSC (see also Q10.5).

10.5. Question: How is "temporary halt of a clinical trial" defined?

489. **Answer:** Article 2 (28) of the Clinical Trials Regulation defines the "temporary halt of a clinical trial" as an "interruption not provided in the protocol of the conduct of a clinical trial by the sponsor with the intention of sponsor to resume it." This could also be part of an urgent safety measure (article 54 of the Clinical Trials Regulation). (see question 10.9).

490. If a sponsor wishes to refrain from notifying the member states of a temporary halt of a clinical trial or one or more parts of it and from submitting a substantial modification to restart the trial or parts of it, the criteria for stopping and restarting the clinical trial must be pre-specified in the protocol.

491. A temporary halt implies that the sponsor makes unforeseen stops of any clinical trial (CT) activity described in the protocol (i.e. recruitment only or recruitment and treatment), due to unexpected circumstances that could affect the benefit/risk ratio or not. Therefore, a temporary halt notification should be accompanied by an end of recruitment notification. In case of safety issues subjects need to be monitored/followed up. During the temporary halt, the issues of concern are assessed together with the need for possible changes in the CT. After this analysis is completed, and reassurance that any potential problem may be solved or mitigated, the sponsor could either restart or end the CT.

492. In case the reasons for the temporary halt have the potential to affect the benefit/risk balance (i.e. concern related to safety, lack of efficacy or IMP quality defect), the sponsor should request a restart of the CT through a substantial modification subject to authorisation, providing the justification for the restart, including conclusions of the analysis, the mitigation measures if applicable and an updated benefit/risk

assessment.

493. When the reasons for a temporary halt have had no potential effect on the benefit/risk balance (e.g. lack of supply of IMP/shortages), the sponsor should notify when the CT is resumed within 15 days of the restart of the CT.

494. If a temporarily halted CT is not resumed within two years, the expiry date of this period or the date of the decision of the sponsor not to resume the clinical trial, whichever is earlier, shall be deemed to be the date of the end of the CT. In the case of early termination of the CT, the date of the early termination shall be deemed to be the date of the end of the CT.

10.6. Question: What information should be provided in notifications of temporary halts and decisions to early terminate a clinical trial or parts of it?

495. Notifications of temporary halts and decisions to early terminate a clinical trial or parts of it should include at least the following information:

- The sponsor should provide the underlying reasons.
- The sponsor should provide, for each concerned member state, the number of subjects included in the study, the number of subjects under treatment and the number of subjects under follow-up.
- The sponsor should specify the follow-up procedure and provide information concerning treatment options for trial subjects after TH or ET (especially in the event of early closure of the research).
- The sponsor should confirm that investigators have been informed of the sponsor's decision of interrupting or early terminating the study and the content of recommendations for management/monitoring of subjects interrupting/ending treatment.
- The sponsor should confirm that subjects have been adequately informed and that this is documented in the patient record.

10.7. Question: If a clinical trial temporarily halted according to articles 37 and 38 is not resumed within two years, can the re-start date of the clinical trial occur after the two-year period?

496. Answer: A sponsor can submit within the two-year period following a temporary halt a SM requesting a restart date **after** the 2-year period. This SM can only be submitted **before** the expiry of the 2-year period and applies to temporary halts for reasons of subject safety or not.

10.8. Question: If a clinical trial temporarily halted according to article 38 is not resumed within two years, will article 37(7) also apply?

497. **Answer:** In case of clinical trials that are temporary halted for reasons of subject safety (article 38: change of benefit-risk balance) sponsors are encouraged to notify the Member States concerned any follow up that has been taken or that is needed, **before** the 2-year expiry.

10.9. Question: How should urgent safety measures (article 54) involving temporary halts (articles 38) be notified?

498. **Answer:** Urgent safety measures may involve a temporary halt of the clinical trial due to safety reasons. In such cases, it is not necessary to submit both a US and a TH notification, but a single notification is sufficient.

499. If the entire clinical trial is halted (recruitment and/or treatment), a

temporary halt notification should be submitted. However, if only parts of the trial are halted (e.g. certain treatment arms, treatment groups or subprotocols), this should be reported as urgent safety measure to ensure that the overall trial status as ongoing is correctly reflected in CTIS.

500. In any case, the timelines for reporting THs and USMs should be adhered to, i.e. a TH notification that is accompanied by additional USMs, should be submitted without undue delay but not later than seven days from the date the measures have been taken (CTR art. 54).

10.10. Question: How should temporary halt or other safety relevant notifications be submitted if the CT has not been initiated or has been on halt in some, but not all MSCs?

501. **Answer**: In a multi-national trial, which has not been started in all MSCs (or has been temporarily halted in some MSCs) notifications on temporary halt (**or other safety related notifications)** can only be submitted to those MSCs where the trial has been initiated and/or not halted. If the same measure (e.g. temporary halt for safety reasons, unexpected event, urgent safety measure …) also applies to the other MSCs the sponsor shall inform the RMS (or any other MSCs where the notification can be done) on the overall 'MS' status (UE or temporary halt applicable for all MSs) via notification supporting document. This should also include a commitment to apply the same measure (e.g. temporary halt for safety reasons) in case the trial is (re-)started before resolution of the reason for the temporary halt in any of the MSCs where the notification is not possible due to the trial status (not started at the time of notification or halted). This method can also be used across trials.

10.11. Question: How is "suspension of a clinical trial" defined?

502. **Answer:** Article 2(29) of the Clinical Trials Regulation defines

suspension of a clinical trial as "interruption of the conduct of a clinical trial by a Member State"**.** This can be decided by the Member State concerned when taking a corrective measure, as defined in article 77, on the grounds that the clinical trial does not meet the requirements set out in the Clinical Trials Regulation.

10.12. Question: How is "early termination" defined?

503. **Answer:** Article 2(27) of Clinical Trials Regulation defines early termination as "the premature end of a clinical trial due to any reason before the conditions specified in the protocol are complied with “”. (for the definition of the end of a clinical trial, see Q10.14). However, when the protocol specifies circumstances that would determine an early termination of the clinical trial, in case such circumstances occur, the sponsor needs to notify also an early termination of the CT according to Articles 37 or 38 of the clinical trials Regulation, clarifying the reasons to the Member States.

504. In the case of early termination of a clinical trial (CT) for reasons not affecting the benefit-risk balance, such as low recruitment, shortage of drug supply, end of development, provided that treatment options for subjects still participating in the clinical trial would not be compromised, or when no subject has been included, the sponsor shall notify each Member State concerned through the EU portal of the reasons for such action and, when appropriate, follow-up measures for the subjects, within 15 days of the early termination, according to article 37 of the clinical trial Regulation.

505. An earlier end of a CT which is based on faster recruitment than anticipated, should not be considered as "early termination".

506. There may be cases where a CT is ended earlier for reasons of lack of efficacy or for reasons related with lack of/insufficient quality of the IMP. Both cases would impact the benefit-risk balance and are to be understood as a safety issue. In such cases, the decision to early terminate the CT should be communicated to the MSs via an USM notification (CTR art. 38 of the Clinical Trials Regulation).

10.13. Question: If no subject has been included in a clinical trial in a Member State concerned, how should a sponsor proceed?

507. **Answer:** The necessary measures depend on the situation.

508. If no subject has been included in a clinical trial (CT) in a Member State concerned (MSC) this means that the first visit of the first subject did not take place and therefore the subject did not sign an informed consent to participate in activities that are protocol directed interventions (see also Q10.3).

509. The first act of recruitment, as defined in the protocol (e.g. publication of an advertisement for recruitment), may have occurred and therefore the CT may have started (see Q10.1). However if no subject was subsequently included due to, for example, unsuccessful recruitment, the authorisation for this MSC will expire within 2 years from the date of authorisation (article 8(9) of the Clinical Trials Regulation). This expiration will be tacit and therefore it is important that sponsors do report the first visit of the first subject before the expiration date.

510. In a situation where no subject was included a sponsor may:

- notify early termination of the CT in the MSC (article 2(27) and article 37 of the Clinical Trials Regulation) (see Q10.10);
- submit a substantial modification according to Chapter III of the Clinical Trials Regulation within two years from the decision on the CT to include further sites;
- submit a substantial modification according to Chapter III of the Clinical Trials Regulation to ask for an extension of the authorisation, including a justification clarifying the feasibility of the CT. If an extension was not submitted and approved within two years from the decision on the clinical trial, the authorisation shall expire in that MSC. The sponsor will then have to submit a new application as per article 14 of the Clinical Trials Regulation.

511. If no subject is included in a CT in *only one of several sites* in a MSC the CT can, in principle, continue. However, scientifically, the sponsor should assess the potential impact on the overall recruitment. Additionally, a substantial modification may be required (e.g. to add another site or extend the recruitment period for other sites).

10.14. Question: How is “end of a clinical trial” defined? What are the sponsor's obligations after the clinical trial ends?

512. **Answer:** Article 2(26) of the clinical trial Regulation defines "end of a clinical trial" as "the last visit of the last subject, or at a later point in time as defined in the protocol".

513. The sponsor shall notify each Member State concerned (MSC) in the EU/EEA of the end of a CT **in relation to that MSC** through the EU portal, within 15 days from the end of the CT in relation to that MSC.

514. Additionally the sponsor shall notify each MSC of the end of a CT **in all MSC in the EU/EEA as well as in all third countries** through the EU portal, within 15 days from the end of the CT in the last of the MSC as well as in the last of the MSC and third countries in which the CT has been conducted.

515. Irrespective of the outcome of a CT, within one year from the end of the CT in all MSC in the EU/EEA (and from not the global end of the CT. See article 37(4), recital 39 and point 184 below), the sponsor shall submit to the EU database:

- a summary of the results of the CT, in line with Annex IV of the Clinical Trials Regulation.
- a summary for laypersons, in line with Annex V of the Clinical Trials Regulation.

516. In cases where the CT was intended to be used for obtaining a marketing authorisation for the investigational medicinal product a clinical study report should be submitted to the EU database by the applicant for marketing authorisation within 30 days after the day the marketing authorisation has been granted, the procedure for granting the marketing authorisation has been completed, or the applicant for marketing authorisation has withdrawn the application.

517. Where, for scientific reasons detailed in the protocol, it is not possible to submit a summary of the results within one year, for example when the clinical trial is still ongoing in third countries and data from that part of the trial are not available, which makes a statistical analysis not relevant, the summary of results shall be submitted as soon as it is available. In this case, the protocol shall specify when the results are going to be submitted, together with a justification (see article 37(4) and Recital 39 of the Clinical Trials Regulation).

MISCELLANEOUS

11.1. Question: Can the reporting Member State be changed?

518. **Answer:** The Clinical Trials Regulation does not provide for a procedure to change the reporting Member State. The Regulation actually specifies in articles 14(2) and 17(1) that the reporting Member State for an initial authorisation procedure will be the reporting Member State for the authorisation of an additional Member State or for a substantial modification.

519. Therefore in case a clinical trial is not on-going in a reporting Member State (due to e.g. a withdrawn or lapsed application) it is not possible to change the reporting Member State.

520. However, it may be possible for a reporting Member State to delegate/contract out the work to another Member State concerned but the responsibility will still lie with the original reporting Member State, who assessed the original application, and should continue to assess any follow ups or substantial modifications under the same criteria.

11.2. Question: Can a corrective measure be taken by a Member State after the end of a clinical trial?

521. **Answer:** Corrective measures referred to in article 77 of the Clinical Trials Regulation are expected to be taken in the majority of cases by Member States while a clinical trial is on-going. However when follow up of patients for safety reasons is deemed necessary Member States may decide to take a corrective measure after a clinical trial has ended and apply article 77(1).

11.3. Submission of notification of Serious Breaches in CTIS

522. If a serious breach identified at a clinical trial site outside the EU/EEA site is likely to affect the safety and rights of subjects in the EU/EEA and/or the reliability and robustness of the clinical trial data, then this breach requires notification to the EU/EEA.

523. Article 52.1 of the CTR requires sponsors to report serious breaches

within seven days of becoming aware of them. The 7-day timeframe starts when the sponsor becomes aware of a potential breach. However, only confirmed serious breaches should be reported. Sponsors should complete their first analysis, confirm the seriousness, and submit the initial report of the serious breach in CTIS within this period.

524. For more guidance on the submission of Serious breaches please consult the Guideline for the notification of serious breaches of Regulation (EU) No 536/2014 or the clinical trial protocol.

11.4. Submission of third-country Inspection Reports in CTIS

525. GCP inspection reports conducted by third-country authorities must be submitted in CTIS even if they concern sites outside the EU. Article 53 of Regulation (EU) No 536/2014 requires the submission of GCP inspection reports for clinical trials conducted under the CTR, regardless of where the inspection takes place or whether it directly impacts EU sites or activities. There is no provision for discretion based on relevance or location.

526. The obligation applies to all GCP inspection reports of all sites involved in the clinical trial—sponsor sites, CROs, and clinical investigator sites—regardless of whether the inspection was conducted in the EU/EEA or in a third country. If the trial inspected is/was conducted in the EEA and falls under the CTR, the third-country GCP inspection report must be submitted in CTIS.

527. The full inspection report must be submitted in CTIS in its original language. Member States may request a translation of either the full report or a summary of key findings, depending on their needs.

Annex I: Decision tree to establish a whether a study is a “clinical trial”

This algorithm and its endnotes will help you answer the question on whether a given investigation on humans is a clinical trial governed by the Regulation EU No 536/2014. Please start in column A and follow the instructions. Additional information is provided in the notes at the end of the table. If you have doubts about the answer to any of the questions, contact the national contact point(s) of the Member State(s) Concerned.

A	B	C	D	E
Is a medicinal product being investigated ? *(1)*	**What effects of the medicinal product are you looking for?**	**Why are you looking for those effects?**	**How are you looking for those effects?**	**Is your clinical trial a low-intervention clinical trial?**
If you answer no to l the question in column A below, the investigation does not fall within the scope of Regulation EU No 536/2014 If you answer yes to f the question below go to column B.	If you answer no to all the questions in column B below, the investigation does not fall within the scope of Regulation EU No 536/2014 If you answer yes to any of the questions below go to column C	If you answer no to all the questions in column C below, the investigation does not fall within the scope of Regulation EU No 536/2014 If you answer yes to any of the questions below go to column D - the investigation is a clinical study as described in article 2(2)(1) of Regulation EU No 536/2014.	If you answer NO to all the questions in column D below, the clinical study is a non-interventional study that does not fall within the scope of Regulation EU No 536/2014 If you answer yes to any of the questions below go to column E – the study is a clinical trial according to Regulation EU No 536/2014	If your answer NO to any of the questions below in column E, the trial is a clinical trial within the scope of Regulation EU No 536/2014 but is NOT a low-intervention clinical trial as defined in Regulation EU No 536/2014. If you answer YES to ALL of the questions below, the trial is a low- intervention clinical trial. A specific set of risk- adaptations can be applied.
A. Is the investigated substance or product either presented as a medicinal product or does it function as such, in accordance with point 2 of article 1 of Directive 2001/83/EC ? *(2)*	B. Is the aim of the investigation on the medicinal product : B.1. To discover or verify/compare its clinical	C. Is the objective of the investigation on a medicinal product : *C.1.* To ascertain or verify/compare the	*D.1.* Is the assignment of any patient involved in the study to a particular therapeutic strategy decided in advance by a clinical trial protocol *(5)*, and does the assignment not fall within normal clinical	E.1. Is this a study of one or more medicinal products, which all have a marketing authorization in the Member State(s) concerned?

	effects? B.2. To discover or verify/compare its pharmacological effects, e.g. pharmacodynamics? B.3. To identify or verify/compare its adverse reactions? B.4. To study or verify/compare its pharmacokinetics, e.g., absorption, distribution, metabolism or excretion?	efficacy of the medicine? *(3)(4)* *C.2.* To ascertain or verify/compare the safety of the medicine?	practice in the Member State(s) Concerned ? *(6)* D.2. Is the decision to prescribe a particular medicinal product clearly taken together with the decision to include the patient in the study? D.3. Are diagnostic or monitoring procedures applied to the patients included in the study, other than those which are applied in normal clinical practice in any of the Member State(s) concerned? *(6)*	E.2. Does the protocol of the clinical trial specify that (i) the investigational medicinal products are used in accordance with the terms of the marketing authorisation; or (ii) the use of the investigational medicinal products is evidence-based and supported by published scientific evidence on the safety and efficacy of those investigational medicinal products in any of the Member States concerned; E.3. Do the additional diagnostic or monitoring procedures not pose more than minimal additional risk or burden to the safety of the subjects compared to normal clinical practice (6) in any Member State concerned? ("Yes" to this answer means that the additional procedures do not pose more than minimal risk or burden; "No" means that the additional procedures do pose more than minimal risk or burden)

(1) Please refer to Q&A "Is the definition of 'medicinal product' relevant for the scope of the Clinical Trials Regulation?" and Q&A "Can a study be considered as clinical trial within the scope of Regulation (EU) No 536/2014 if it starts after administration/exposure of the investigational medicinal product has finished?

(2) The following substances are not considered to be medicines

- Human whole blood, blood cells, or plasma (this does not include derivatives of human whole blood, human blood cells and human plasma that involve a manufacturing process)
- Food products, including dietary supplements
- Cosmetic products (Regulation on cosmetic products EU no 1223/2009, article 2.1.a.)
- Medical device (Medical Device Regulation EU no 2017/745, article 1.2 and 2.1)

The qualification of borderline products is a national competence. When there is an uncertainty on the status of a given product, this needs to be clarified with the national competent authorities.

(3) Efficacy is the concept of demonstrating scientifically whether and to what extent a medicine is capable of diagnosing, preventing or treating a disease and derives from EU pharmaceutical legislation.

(4) This includes studies on "drug utilisation" of medicinal products used in normal clinical practice and trials on "palatability" intended to assess the suitability of a formulation for a particular population.

(5) Assignment of patients to a treatment group by randomisation planned by a clinical trial protocol cannot be considered as current practice

(6) Please refer to Q&A "What is not considered as "normal clinical practice?" and the guidance for Risk proportionate approaches in clinical trials: 2017_04_25_risk_proportionate_approaches_in_ct_0.pdf (europa.eu)

(7) In case of doubt whether an intervention poses only minimal burden or risk to subjects, please contact the concerned national competent authorities.

Annex II: Language for part I documents in case of multinational trials and translated Patient Facing Documents submitted in Part II

The table below provides answers to the following question "My document is in English – do I have to translate it into National language/s?". As a general rule to read the table below:

- For those Member States that do not require other languages than EN, the table reports "-".
- When the document must also be submitted in at least one national language, the table reports "Yes".
- MS accept just the EN version of the Investigator's Brochure, GMP compliance, IMPD, AxMPD and Scientific advice &PIP.

Member States clarify specific circumstances as per below:

- In case of mononational clinical trials, one of the national language(-s) is acceptable.
    - When a trial becomes multinational, the sponsor should provide the translation in EN of the documents as outlined in the table.
- In case a translation in the national language is required, it is always acceptable to have an original version in EN (e.g. protocol synopsis in EN and national language).
- All Member States accept in Part I Patient Facing Documents (PFD) in English (or in the national language in case the trial is mononational). Translated versions of Patient Facing Documents are to be submitted in Part II as outlined in the table.

(1) CY, EL: outside public health emergencies, a case-by-case approach is applied for trials where the IMP is administered by the investigator and not handled to the patient.
(2) CY, CZ, EL, IT, LU accept EN version of the labelling of IMP if administered by a physician (or qualified health personnel) only in case of emergency (E.g COVID-19).
(3) HU and CZ require scientific summary of the study protocol in national language.

Member states clarify that investigational and auxiliary medicinal products for clinical trials may be labelled in EN if they are administered by a physician (or dentist in case of dental trials) or qualified healthcare personnel, and in accordance with the protocol of the clinical trial.

- (4) BG, DE, EL require the physician to be a member of the investigating team and in accordance with the protocol of the clinical trial.
- (5) AT labelling only in EN is accepted where the IMP is administered exclusively by healthcare professionals with sufficient command of English. This must be stated in the protocol. Also, only the scientific and public title in "CTIS fields" need to be in German.

- (6) BE, BG, DK labelling only in EN is accepted if they are administered by a physician (or dentist in case of dental trials) or qualified healthcare personnel, and in accordance with the protocol of the clinical trial, and the IMP to be administered at the clinical trial site.

(7) FI: labelling should always be in FI and also in SV if Swedish-speaking patients are to be recruited.
(8) BE: To be submitted at least in the official national language(s) of the region(s) where the trial is conducted.
(9) FR: National language for the text fields to be made public.
(10) in all national languages (FR/NL/DE).
(11) LU: in at least two national languages.

EU Member State	Cover letter	Protocol	Protocol synopsis	Translated PFD in Part II	Investigator s brochure	GMP compliance	IMPD	AMPD	Scientific advice and PIP	Labelling	Labelling for an IMP not handed to the subject	CTIS fields
Austria	_	_	Yes	Yes	_	_	_	_	_	Yes	_(5)	Yes(5)
Belgium	_	_	Yes(10)	Yes(8)	_	_	_	_	_	Yes(10)	_(6)	_
Bulgaria	Yes	_	Yes	Yes	_	_	_	_	_	Yes	_(4)(6)	Yes
Croatia	Yes	_	Yes	Yes	_	_	_	_	_	Yes	_	Yes
Cyprus	_	_	Yes	Yes	_	_	_	_	_	Yes	_ (1)(2)	_
Czechia	_	_	Yes(3)	Yes	_	_	_	_	_	Yes	_(2)	_
Denmark	_	_	_	_	_	_	_	_	_	Yes	_(6)	_
Estonia	_	_	_	Yes	_	_	_	_	_	Yes	_	_
Finland	_	_	_	_	_	_	_	_	_	Yes(7)	_	_
France	_	_	Yes	Yes	_	_	_	_	_	Yes	_	Yes(9)
Germany	_	_	_	Yes	_	_	_	_	_	Yes	_(4)	_
Greece	Yes	Yes	Yes	Yes	_	_	_	_	_	Yes	_ (1)(2)	Yes
Hungary	Yes	_	Yes(3)	Yes	_	_	_	_	_	Yes	_	Yes
Iceland	_	_	Yes	Yes	_	_	_	_	_	Yes	_	_

EU Member State	Cover letter	Protocol	Protocol synopsis	Translated PFD in Part II	Investigator s brochure	GMP compliance	IMPD	AMPD	Scientific advice and PIP	Labelling	Labelling for an IMP not handed to the subject	CTIS fields
Ireland	_	_	_	_	_	_	_	_	_	_	_	_
Italy	_	_	Yes	Yes	_	_	_	_	_	Yes	_ (2)	_
Latvia	_	_	_	Yes	_	_	_	_	_	Yes	_	_
Lichtenstein												
Lithuania	_	_	Yes	Yes	_	_	_	_	_	Yes	_	_
Luxembourg	_	_	_	Yes	_	_	_	_	_	Yes(11)	_(2)	_
The Netherlands	_	_	Yes	_	_	_	_	_	_	Yes	_	Yes
Norway	_	_	Yes	_	_	_	_	_	_	Yes	_	_
Poland	_	_	Yes	Yes	_	_	_	_	_	Yes	Yes	Yes
Portugal	_	_	Yes	Yes	_	_	_	_	_	Yes	_(1)	Yes
Romania	_	_	Yes	Yes	_	_	_	_	_	Yes	_	_
Slovakia	Yes	_	Yes	Yes	_	_	_	_	_	Yes	Yes	Yes
Slovenia	Yes	_	Yes	Yes	_	_	_	_	_	Yes	_	_
Spain	_	_	Yes	Yes	_	_	_	_	_	Yes	_	Yes
Sweden	_	_	Yes	Yes	_	_	_	_	_	Yes	_	_

DISCLAIMER: National contact points are responsible for the content and for promptly informing the Commission of any change that impacts the information provided in the table. Sponsors are invited to consult the national websites indicated in Annex III.

Annex III: Part II documentation - where sponsors can find national requirements Member State

	Websites where sponsors can find important information to submit high quality Part II documents as part of their clinical trial applications.	**Email address for enquiries related to Part I clinical trial applications**	**Email address for enquiries related to Part II clinical trial applications**
Austria	• www.basg.gv.at	clinicaltrials@basg.gv.at	clinicaltrials@basg.gv.at
Belgium	• CTR page on the FAMHP website : https://www.famhp.be/en/eu_regulation_536/2014	ct.rd@fagg-afmps.be	ct.rd@fagg-afmps.be
Bulgaria	• https://www.bda.bg/bg/62-business-info/clinical-examinations-biz	clintrialsquestions@bda.bg	clintrialsquestions@bda.bg
Croatia	• https://zdravlje.gov.hr/o-ministarstvu/djelokrug-1297/lijekovi-i-medicinski-proizvodi/1349 • https://www.halmed.hr/O-HALMED-u/Sredisnje-eticko-povjerenstvo-SEP/Naputci-podnositeljima-zahtjeva/ • https://www.halmed.hr/O-HALMED-u/Usluge-i-cjenik/Cjenik-usluga-HALMED-a/	klinicka.ispitivanja@miz.hr	klinicka.ispitivanja@miz.hr
Cyprus	• Cyprus National Bioethics Committee website: http://www.bioethics.gov.cy/moh/cnbc/cnbc.nsf/index_en/index_en?OpenDocument • Pharmaceutical Services website:	clinicaltrials@phs.moh.gov.cy	cnbc@bioethics.gov.cy

	https://www.moh.gov.cy/moh/phs/phs.nsf/home_en/home_en?openform		
Czechia	• https://www.sukl.cz/leciva/klh-ctis-01 (Czech) • https://www.sukl.eu/medicines/klh-ctis-01 (English)	ctis-dpo@sukl.gov.cz	eticka.komise@sukl.gov.cz
Denmark	• https://videnskabsetik.dk/ansoegning-til-etisk-komite/kliniske-forsoeg-med-laegemidler- under-ctr • Fees: https://laegemiddelstyrelsen.dk/en/licensing/clinical-trials/trials-in-humans/fees/ • https://researchethics.dk/information-for-researchers/clinical-trials-with-medicinal-products- under-the-ctr	kf@dkma.dk	kontakt@dvmk.dk
Estonia	• https://ravimiamet.ee/ravimid-ja-ohutus/ravimiuuringud/ravimiuuringute-eetikakomitee (Estonian) • https://ravimiamet.ee/en/node/1007 (English)	trials@ravimiamet.ee	ethics@ravimiamet.ee
Finland	• https://tukija.fi/en/applications-under-regulation (English) • https://tukija.fi/laaketutkimusasetuksen-mukaiset-hakemukset (Finnish) • https://tukija.fi/sv/forskningar-enligt-eu-forordiningen (Swedish)	clinicaltrials@fimea.fi	info@tukija.fi

France	• https://solidarites-sante.gouv.fr/IMG/pdf/documents_attendus_en_france_concernant_la_partie_ii_de_cta_2022 • _02_02.pdf	ecda2@ansm.sante.fr	DGS-RBM@sante.gouv.fr
Germany	• https://www.akek.de • information of German drug act as well as CTR https://www.akek.de/en/aktuelle-hinweise/eu-verordnung-536-2014/	ctr@bfarm.de	ctr@bfarm.de
Greece	• https://www.eof.gr/web/guest/eed		eed@eof.gr
Hungary	• https://ogyei.gov.hu/uj_klinikai_vizsgalat_engedelyezese_az_europai_tanacs_536_2014eu_ren_delete_szerint • https://ogyei.gov.hu/new_european_clinical_trial_legislation • https://ett.aeek.hu/kfeb/ under heading "Tájékoztatás a Gyógyszerrendelet alapján beadandó vizsgálati kérelmek benyújtóinak"	ctrcontacthu@nngyk.gov.hu	kfebtitkarsag@bm.gov.hu
Iceland	• https://www.ima.is/development/clinical_trials/	kliniskar.rannsoknir@lyfj astofnun.is	kliniskar.rannsoknir@lyfjas tofnun.is
Ireland	• https://www.nrecoffice.ie/submit-under-the-clinical-trial-regulation	ctreg@hpra.ie	clinicaltrials@nrec.ie
Italy	• https://www.aifa.gov.it/web/guest/centro-coordinamento-comitati-etici • https://www.aifa.gov.it/web/guest/regolamento-europeo-sperimentazioni-cliniche	reg.eu.sperimentazioni@a ifa.gov.it	reg.eu.sperimentazioni@aif a.gov.it
Latvia	• www.zva.gov.lv • https://www.zva.gov.lv/en/industry/sponsors-clinical-trials/clinical-trials-	ct@zva.gov.lv	etikas-komiteja@stradini.lv

	regulation		
Lichtenstein	•		
Lithuania	• https://bioetika.sam.lt/ • https://www.vvkt.lt/	vvkt@vvkt.lt	vvkt@vvkt.lt ; lbek@bioetika.sam.lt
Luxembourg	• www.CNER.lu	Clinicaltrials@ms.etat.l u	contact@cner.lu
Malta			
Netherlands	• https://english.ccmo.nl/investigators/clinical-trials-with-medicinal-products-ctr/preparation- ctr/research-dossier-part-ii	ctr@ccmo.nl	ctr@ccmo.nl
Norway	• https://rekportalen.no/#omrek/REK_KULMU		rek-kulmu@medisin.uio.no
Poland	• General information about CTR/CTIS: https://urpl.gov.pl/pl/komunikat-prezesa- urz%C4%99du-z-dnia-29-grudnia-2021-r-w-sprawie-stosowania-przepis%C3%B3w-rozporz%C4%85dzenia-0	urpl@urpl.gov.pl	urpl@urpl.gov.pl
Portugal	• https://www.ceic.pt/regulamento-ec	ensaios.clinicos@infarme d.pt	ctis@ceic.pt
Romania	• https://www.anm.ro/medicamente-de-uz-uman/studii-clinice/	clinicaltrials@anm.ro	secretariat@bioetica- medicala.ro
Slovakia	• https://www.health.gov.sk/?Eticka-komisia-pre-klinicke-skusanie	trial-sukl@sukl.sk	eticka.komisia@health.gov. sk
Slovenia	• https://www.gov.si/zbirke/delovna-telesa/komisija-rs-za-medicinsko-etiko/	ct@jazmp.si	kme.mz@gov.si
Spain		aecaem@aemps.es	General questions on part II: aecaem@aemps.es

	• https://www.aemps.gob.es/medicamentos-de-uso-humano/ensayosclinicos/		Questions about RFI of a specific CT application: e- mail of the corresponding Ethic Committee (CEIm) that can be found in https://www.aemps.gob.es/medicamentos-de-uso-humano/investigacionclinica_ceim/directorio-de-los-
Sweden	• Regarding fees: https://www.lakemedelsverket.se/en/permission-approval-and-control/clinical-trials/medicinal-products-for-human-use/fees General information regarding CTR including Part II templates: https://www.lakemedelsverket.se/en/permission-approval-and-control/clinical- trials/medicinal-products-for-human-use/clinical-trials-regulation-eu-536-2014/apply-for- clinical-trial-permit-according-to-regulation-536-2014	kp.central@lakemedelsverket.se	kp.central@lakemedelsverket.se

DISCLAIMERS

The European Commission is not responsible for the quality and completeness of the information reported in the Annex III nor for the functioning of the websites. For questions and remarks on the links and on the information reported in the websites listed in the table in Annex III, please contact the national contact point(s).

Please note that the national competent authorities or the contact points indicated in Annex III may not reply to enquiries for which a reply is already available either in the Clinical Trials Regulation (EU) 536/2014 or in the Questions and Answers document available on Eudralex volume 10 or in national Questions and Answers documents.

Annex IV. Classification of changes to ongoing clinical trials

	SM	81.9NSM	NSM	Part I/II
Changes to initial documents				
Sponsor	Change of sponsor entity that involves additional changes: e.g. insurance, legal representative, addition of a new sponsor/co-sponsor.	Change of the existing sponsor's name, keeping the same legal entity. Change of existing sponsor[1] or co-sponsor legal entity if it does not involve additional changes in the trial documentation apart from administrative changes. Changes regarding which co-sponsor is responsible for the tasks referred to in article 72(2) of the Clinical Trial Regulation. Change in the sponsor/co-sponsor contact details (address, email and phone number)[2]. Change of contact point to the Union[3], scientific and public contact point (name and contact	Minor changes in the contact details e.g. change of mailingaddress (like PO Box, not physical change) or email address of a site without impact for thesupervision of the trial.	

[1]Co sponsor is used in this document in the meaning of Art 72 of the CTR
[2]when timely provision of this information is necessary for adequate supervision of the trial
[3]This term is used in CTIS for the entity who is responsible for being a contact point for receiving all questions from subjects, investigators or any Member States concerned regarding the clinical trial and providing answers to them (Art 72.2b).

		details)		
Sponsor's Legal Representative within the EEA	Change of legal representative	Change of contact details of legal representative provided that there are no other changes that would be substantial	Minor changes in the contact details e.g. change of mailing address (like PO Box, not physical change) or email address of a site without impact for the supervision of the trial.	

Persons/third parties to whom the sponsor has delegated tasks (e.g. CRO)		• Addition of a new third party. • Change of contact details of third party/other persons to whom the sponsor delegated sponsor tasks • Change in the third party (incl. CRO) contact details (address, email and phone number)[2] • Change of delegated tasks	Minor changes in the contact details e.g. change of mailing address (like PO Box, not physical change) or email address of a site without impact for the supervision of the trial.	
Upload data/document to meet a condition	Always when the provision or update of data/document if in the decision the condition requested as a SM or exceptionally when the route is not defined by RMS/MSCs but the change has a substantial impact on safety and right or data robustness in the opinion of the sponsor and was not authorised previously (i.e. in the case of trials with adaptive design)	Any other cases, as defined by the RMS (part I) or MSC (part II) ([75]) (can trigger a SM as part of a corrective action)	--	Part I and/or II

Full title (English or common language for the assessments)	Changes that modify the 'meaning (normally it is expected to be submitted with other SM with changes the study design)		Administrative changes (typos)	Part I
Addition/deletion of non-EEA countries into trial		Inclusion or exclusion of Non-EEA 3rd countries into the application dossier with no additional substantial changes to the trial (e.g. no significant impact on the absolute number of subjects in the trial or in a MSC)		
Amendments to the trial protocol				
	1. Change of primary or secondary endpoint; 2. New mode of measurement for the primary endpoint; 3. New toxicological or pharmacological data or new interpretation of toxicological or pharmacological data which is likely to impact on the risk/benefit	1. Significant increase in duration of the overall time of the trial, provided that the following conditions are met (77): • the exposure to treatment with the IMP is not extended; • the definition of the end of the trial is unchanged;	1. Minor clarifications to the protocol. 2. The addition/deletion of exploratory and/or tertiary endpoints as recorded in the TMF with no significant effect on	Part I

	assessment; 4. A change in the definition of the end of the trial; 5. Change in the trial design (e.g. removal of a trial arm, addition of a new trial period (e.g. open label extension) not foreseen in the currently authorized protocol); 6. Addition of a new sub-protocol trial arm[4] 7. Change of inclusion or exclusion criteria if these changes are likely to have a significant impact on the right and safety of trial subjects or the scientific value of the clinical trial (e.g. resulting in changes the overall subjects population); 8. Reduction in the number of scheduled subject study visits (including replacement of physical visits with "remote" visits); 9. Introduction or change of a diagnostic or medical monitoring procedure which is likely to have a significant impact on the safety,	and • scheduled subject study visits arrangements are unchanged; If there is a change in one or more of these conditions, it would be considered to be a substantial modification. 1. In case of low interventional trials, additional diagnostic or medical monitoring procedure which is not requested by a MSC if it does not pose more than minimal additional risk or burden to the subjects[5]	the conduct of the trial. 3. A minor increase in the duration (<10%) of the trial 4. A change in the number of trial subjects per Member State if the absolute number of subjects in the trial is identical or the decrease/increase is insignificant.	

[4] if not predefined in the latest version of the authorised protocol

[5]https://ec.europa.eu/health/sites/health/files/files/eudralex/vol-10/2017_04_25_risk_proportionate_approaches_in_ct.pdf

	burden on subjects, or scientific value of the clinical trial (including increased number or volume of biological samples taken for the purpose of the trial); 10. Addition/Removal/withdrawal of an independent safety/data monitoring board; 11. Change of IMP/AxMP and/or treatment modalities (mode of administration/duration/frequency /dosing) of IMPs, including the criteria to define treatment modality and stopping rules); 12. A change of study design and conduct which is likely to have a significant impact on primary or major secondary statistical analysis or the risk/benefit assessment. 13. Any change (increasing/decreasing) in the absolute number of subjects to be included in the trial unless it is specified in the currently authorised protocol. The change may be due to e.g. an adaptation of			

	the sample size calculation or to maintain a previously defined sample size calculation due to more withdrawals/drop outs than expected. 14. Addition/Deletion of an interim/intermediate analysis unless it is pre-specified in the currently authorised protocol.			
GMP related documents				
Change of source country of IMP/AxMP	See annex V			
Changes to IMPD- Quality (reference to QWP[6] and BWP[7] guidance, if there are additional items, they will be explained here)				
Changes to the IB/IMPD safety and efficacy (non-quality IMPD[8])				

[6] https://www.ema.europa.eu/en/documents/scientific-guideline/guideline-requirements-chemical-pharmaceutical-quality-documentation-concerning-investigational_en.pdf -- chapter 9

[7] https://www.ema.europa.eu/en/documents/scientific-guideline/guideline-requirements-quality-documentation-concerning-biological-investigational-medicinal_en-0.pdf -- chapter 6

[8] CTIS refers to this dossier as "IMPD safety and efficacy". It needs to be noted that in addition to safety and efficacy information it includes also for example the risk-benefit assessment
and additional non-clinical and clinical data

	1. New toxicological or pharmacological data or new interpretation of toxicological or pharmacological data of relevance for the investigator or with an impact on risk/benefit; 2. new clinical data with impact on the risk/benefit ratio 3. change in the overall risk and benefit assessment and analysis	Annual IB update without safety, efficacy or benefit/risk update		Part I
RSI (If the IB is not an SmPC, it shall contain a clearly identifiable RSI section[9])	If the RSI is located in the IB, an update to the RSI which has an effect on subjects' safety and/or safety reporting and expectedness assessment: i. addition of new expected SAR PTs, ii. change of the frequency of expected SARs, iii. MedDRA updates having an impact on subjects' safety and/or		1. changes to the format of the table that do not affect the expected SARs 2. slight modification of exposure rates that do not result in a change in the category of frequency without the addition of new expected SARs and/or new preferred terms (PTs)	

[9] CTR Annex I. E30

	on safety reporting and expectedness assessment (e.g. new preferred term (PT)s listed in the RSI) If the RSI is contained in the SmPC, any update of section 4.8. of the SmPC with an impact on safety and/or safety reporting and expectedness assessment (e.g. addition of a new term) Change of the location of the RSI information (e.g. change from IB to SmPC) if an impact on safety reporting			
Part II document changes				
Addition of a site	• Addition of a new site • Change of the site address with possible impact on suitability		• Closure of a site • Change of site address with no	

			impact on suitability[10]	
Principal investigator (incl. change of an investigator in case that he or she is the only investigator at a trial site, Art 2.2.16)	Change of PI (Art 15) or its contact details if it is at another new trial site, when this change could impact suitability	Change of contact details of principal investigator (email address, phone number) when timely provision of this information is necessary for adequate supervision of the trial		
Insurance policy	• New insurance policy • Change in the content of the insurance policy, eg. a new entity for the insurance provider, changes in insurance coverage, conditions and/or insured amounts;		Extension of validity of an already approved insurance certificate	

[10] This change should be introduced as a NSM into CTIS due to technical reasons

Other part II documents	• Significant changes in the content of any documents addressed to subjects and/or prospective subjects (including advertisement material)[11] • Change in access, disclosure, dissemination, alteration of information and personal data processed related to either subjects' or trial team member data (e.g. new, non-EU storage place, compromised); 3. • Change in collection and future use of biological samples from clinical trial subjects(e.g. new location, outside of		• Technical and administrative changes (including language corrections) • Description of any other agreement than the ones as classified as substantial modification between the sponsor and the site during the study duration • A validated translation of the local approved ICF in another language in order to be used for a potential subject who is not fluent in the local (country) language	

[11]including any significant consequential changes originating from other Part I / Part II documentation

	EU); 4. • Change in financial arrangements with subjects and/or site/investigator; • Change in the compensation paid to subjects and/or investigator/site for participating in the trial; 5. Change in recruitment arrangements including procedures for inclusion of subjects.			
Other				
Extension	Extension of start of recruitment beyond 2 years to avoid expiration of authorization (Art 8.9) Extension of temporary halt (art 37, i.e. not for reasons of subject safety) beyond 2 years to avoid end of trial (See also Q&A 10.5 in Q&A			Part I+II or Part II only

	on CTR)			
Implementation of documentary changes related to urgent safety measures	After the immediate implementation of an USM, (i.e. assessment by EC of urgently implemented changes to the ICF)			Part I/II

Explanatory notes to the table: Horizontal changes to IMPD affecting several trials using the same drug as IMP: proposal is under development by this group Terminology in this guidance is aligned with that used in CTIS Correction of typos and other administrative changes with no impact on the content and meaning are always expected to be updated as non-substantial modifications In clinical trials with adaptive design (e.g. complex clinical trials), those changes, which are described and specified in the currently authorised protocol can be implemented if their authorisation through a SM was not requested as a condition in the decision[12]. Art 81.9 NSM can be submitted only if the change does not trigger additional changes which are expected to be submitted as an SM application. The combination of different art 81.9 changes can cumulate into a change that needs to be submitted as an SM.

[12] http://www.hma.eu/fileadmin/dateien/Human_Medicines/01-About_HMA/Working_Groups/CTFG/2019_02_CTFG_Recommendation_paper_on_Complex_Clinical_Trials.pdf

Annex IV. Classification of changes to ongoing clinical trials

	SM	81.9NSM	NSM	Part I/II
Changes to initial documents				
Sponsor	Change of sponsor entity that involves additional changes: e.g. insurance, legal representative, addition of a new sponsor/co-sponsor.	Change of the existing sponsor's name, keeping the same legal entity. Change of existing sponsor[1] or co-sponsor legal entity if it does not involve additional changes in the trial documentation apart from administrative changes. Changes regarding which co-sponsor is responsible for the tasks referred to in article 72(2) of the Clinical Trial Regulation. Change in the sponsor/co-sponsor contact details (address, email and phone number)[2]. Change of contact point to the Union[3], scientific and public	Minor changes in the contact details e.g. change of mailingaddress (like PO Box, not physical change) or email address of a site without impact for thesupervision of the trial.	

[1]Co sponsor is used in this document in the meaning of Art 72 of the CTR
[2]when timely provision of this information is necessary for adequate supervision of the trial
[3]This term is used in CTIS for the entity who is responsible for being a contact point for receiving all questions from subjects, investigators or any Member States concerned regarding the clinical trial and providing answers to them (Art 72.2b).

		contact point (name and contact details)		
Sponsor's Legal Representative within the EEA	Change of legal representative	Change of contact details of legal representative provided that there are no other changes that would be substantial	Minor changes in the contact details e.g. change of mailing address (like PO Box, not physical change) or email address of a site without impact for the supervision of the trial.	

Persons/third parties to whom the sponsor has delegated tasks (e.g. CRO)		• Addition of a new third party. • Change of contact details of third party/other persons to whom the sponsor delegated sponsor tasks • Change in the third party (incl. CRO) contact details (address, email and phone number)[2] • Change of delegated tasks	Minor changes in the contact details e.g. change of mailing address (like PO Box, not physical change) or email address of a site without impact for the supervision of the trial.	
Upload data/document to meet a condition	Always when the provision or update of data/document if in the decision the condition requested as a SM or exceptionally when the route is not defined by RMS/MSCs but the change has a substantial impact on safety and right or data robustness in the opinion of the sponsor and was not authorised previously (i.e. in the case of trials with adaptive design)	Any other cases, as defined by the RMS (part I) or MSC (part II) ([75]) (can trigger a SM as part of a corrective action)	--	Part I and/or II

Full title (English or common language for the assessments)	Changes that modify the 'meaning (normally it is expected to be submitted with other SM with changes the study design)		Administrative changes (typos)	Part I
Addition/deletion of non-EEA countries into trial		Inclusion or exclusion of Non-EEA 3rd countries into the application dossier with no additional substantial changes to the trial (e.g. no significant impact on the absolute number of subjects in the trial or in a MSC)		
Amendments to the trial protocol				
	1. Change of primary or secondary endpoint; 2. New mode of measurement for the primary endpoint; 3. New toxicological or pharmacological data or new interpretation of toxicological or pharmacological data which is likely to impact on the risk/benefit	1. Significant increase in duration of the overall time of the trial, provided that the following conditions are met (77): • the exposure to treatment with the IMP is not extended; • the definition of the end of the trial is unchanged;	1. Minor clarifications to the protocol. 2. The addition/deletion of exploratory and/or tertiary endpoints as recorded in the TMF with no significant effect on	Part I

	assessment; 4. A change in the definition of the end of the trial; 5. Change in the trial design (e.g. removal of a trial arm, addition of a new trial period (e.g. open label extension) not foreseen in the currently authorized protocol); 6. Addition of a new sub-protocol trial arm[4] 7. Change of inclusion or exclusion criteria if these changes are likely to have a significant impact on the right and safety of trial subjects or the scientific value of the clinical trial (e.g. resulting in changes the overall subjects population); 8. Reduction in the number of scheduled subject study visits (including replacement of physical visits with "remote" visits); 9. Introduction or change of a diagnostic or medical monitoring procedure which is likely to have a significant impact on the safety,	and • scheduled subject study visits arrangements are unchanged; If there is a change in one or more of these conditions, it would be considered to be a substantial modification. 1. In case of low interventional trials, additional diagnostic or medical monitoring procedure which is not requested by a MSC if it does not pose more than minimal additional risk or burden to the subjects[5]	the conduct of the trial. 3. A minor increase in the duration (<10%) of the trial 4. A change in the number of trial subjects per Member State if the absolute number of subjects in the trial is identical or the decrease/increase is insignificant.	

[4] if not predefined in the latest version of the authorised protocol

[5]https://ec.europa.eu/health/sites/health/files/files/eudralex/vol-10/2017_04_25_risk_proportionate_approaches_in_ct.pdf

	burden on subjects, or scientific value of the clinical trial (including increased number or volume of biological samples taken for the purpose of the trial); 10. Addition/Removal/withdrawal of an independent safety/data monitoring board; 11. Change of IMP/AxMP and/or treatment modalities (mode of administration/duration/frequency /dosing) of IMPs, including the criteria to define treatment modality and stopping rules); 12. A change of study design and conduct which is likely to have a significant impact on primary or major secondary statistical analysis or the risk/benefit assessment. 13. Any change (increasing/decreasing) in the absolute number of subjects to be included in the trial unless it is specified in the currently authorised protocol. The change may be due to e.g. an adaptation of			

	the sample size calculation or to maintain a previously defined sample size calculation due to more withdrawals/drop outs than expected. 14. Addition/Deletion of an interim/intermediate analysis unless it is pre-specified in the currently authorised protocol.			
GMP related documents				
Change of source country of IMP/AxMP	See annex V			
Changes to IMPD- Quality (reference to QWP[6] and BWP[7] guidance, if there are additional items, they will be explained here)				
Changes to the IB/IMPD safety and efficacy (non-quality IMPD[8])				

[6] https://www.ema.europa.eu/en/documents/scientific-guideline/guideline-requirements-chemical-pharmaceutical-quality-documentation-concerning-investigational_en.pdf -- chapter 9

[7] https://www.ema.europa.eu/en/documents/scientific-guideline/guideline-requirements-quality-documentation-concerning-biological-investigational-medicinal_en-0.pdf -- chapter 6

[8] CTIS refers to this dossier as "IMPD safety and efficacy". It needs to be noted that in addition to safety and efficacy information it includes also for example the risk-benefit assessment
and additional non-clinical and clinical data

	1. New toxicological or pharmacological data or new interpretation of toxicological or pharmacological data of relevance for the investigator or with an impact on risk/benefit; 2. new clinical data with impact on the risk/benefit ratio 3. change in the overall risk and benefit assessment and analysis	Annual IB update without safety, efficacy or benefit/risk update		Part I
RSI (If the IB is not an SmPC, it shall contain a clearly identifiable RSI section[9])	If the RSI is located in the IB, an update to the RSI which has an effect on subjects' safety and/or safety reporting and expectedness assessment: i. addition of new expected SAR PTs, ii. change of the frequency of expected SARs, iii. MedDRA updates having an impact on subjects' safety and/or		1. changes to the format of the table that do not affect the expected SARs 2. slight modification of exposure rates that do not result in a change in the category of frequency without the addition of new expected SARs and/or new preferred terms (PTs)	

[9] CTR Annex I. E30

	on safety reporting and expectedness assessment (e.g. new preferred term (PT)s listed in the RSI) If the RSI is contained in the SmPC, any update of section 4.8. of the SmPC with an impact on safety and/or safety reporting and expectedness assessment (e.g. addition of a new term) Change of the location of the RSI information (e.g. change from IB to SmPC) if an impact on safety reporting			
Part II document changes				
Addition of a site	• Addition of a new site • Change of the site address with possible impact on suitability		• Closure of a site • Change of site address with no	

			impact on suitability[10]	
Principal investigator (incl. change of an investigator in case that he or she is the only investigator at a trial site, Art 2.2.16)	Change of PI (Art 15) or its contact details if it is at another new trial site, when this change could impact suitability	Change of contact details of principal investigator (email address, phone number) when timely provision of this information is necessary for adequate supervision of the trial		
Insurance policy	• New insurance policy • Change in the content of the insurance policy, eg. a new entity for the insurance provider, changes in insurance coverage, conditions and/or insured amounts;		Extension of validity of an already approved insurance certificate	

[10] This change should be introduced as a NSM into CTIS due to technical reasons

Other part II documents	• Significant changes in the content of any documents addressed to subjects and/or prospective subjects (including advertisement material)[11] • Change in access, disclosure, dissemination, alteration of information and personal data processed related to either subjects' or trial team member data (e.g. new, non-EU storage place, compromised); 3. • Change in collection and future use of biological samples from clinical trial subjects(e.g. new location, outside of		• Technical and administrative changes (including language corrections) • Description of any other agreement than the ones as classified as substantial modification between the sponsor and the site during the study duration • A validated translation of the local approved ICF in another language in order to be used for a potential subject who is not fluent in the local (country) language	

[11]including any significant consequential changes originating from other Part I / Part II documentation

	EU); 4. • Change in financial arrangements with subjects and/or site/investigator; • Change in the compensation paid to subjects and/or investigator/site for participating in the trial; 5. Change in recruitment arrangements including procedures for inclusion of subjects.			
Other				
Extension	Extension of start of recruitment beyond 2 years to avoid expiration of authorization (Art 8.9) Extension of temporary halt (art 37, i.e. not for reasons of subject safety) beyond 2 years to avoid end of trial (See also Q&A 10.5 in Q&A			Part I+II or Part II only

	on CTR)			
Implementation of documentary changes related to urgent safety measures	After the immediate implementation of an USM, (i.e. assessment by EC of urgently implemented changes to the ICF)			Part I/II

Explanatory notes to the table: Horizontal changes to IMPD affecting several trials using the same drug as IMP: proposal is under development by this group Terminology in this guidance is aligned with that used in CTIS Correction of typos and other administrative changes with no impact on the content and meaning are always expected to be updated as non-substantial modifications In clinical trials with adaptive design (e.g. complex clinical trials), those changes, which are described and specified in the currently authorised protocol can be implemented if their authorisation through a SM was not requested as a condition in the decision[12]. Art 81.9 NSM can be submitted only if the change does not trigger additional changes which are expected to be submitted as an SM application. The combination of different art 81.9 changes can cumulate into a change that needs to be submitted as an SM.

[12] http://www.hma.eu/fileadmin/dateien/Human_Medicines/01-About_HMA/Working_Groups/CTFG/2019_02_CTFG_Recommendation_paper_on_Complex_Clinical_Trials.pdf

ANNEX V: CHANGES TO SOURCE COUNTRY

1. **Impact of the change of source country on the regulatory requirements of IMPs and AxMPs without a Marketing Authorisation in EU/EEA (unauthorised IMP/AxMP)**

Regulatory requirements with regards to labelling:

Article 66: Labelling required: on the outer and immediate packaging: identification of the CT, the contact person, the medicinal product, information related to the use of the medicinal product in accordance with Annex VI A and B to ensure subject safety and reliability and robustness of the data generated in the clinical trial.

Regulatory requirements with regards to GMP

Article 61: The manufacturing and import of investigational medicinal products in the Union shall be subject to the holding of an authorisation. An I. F: a copy of the manufacturing and import authorisation as referred to in Article 61 and a certification by the qualified person in the Union that the manufacturing complies with GMP at least equivalent to the GMP in the Union[1].

Source country change within EU/EEA, or from Non-EEA to EEA, or from EU/EEA to non-EU/EEA (implies change of manufacturer, manufacturing site and/or manufacturing process)

The change of the source country is a change to be submitted for authorisation as a Substantial Modification[2].

AxMP: The SM submission must cover the change in the **Cover Letter**, **Protocol**, **IB**, SmPC[3] / (simplified) IMPD, GMP/Labelling section and **product section** (whichever is relevant)

[1] unless there are specific arrangements provided for in mutual recognition agreements between the Union and third countries.
[2] In exceptional cases, when an application with two manufacturing sites from the same company in different countries is authorised in a clinical trial with the same or equivalent manufacturing process, but identical specifications and with GMP-related documents provided for both sites in the latest approved version of the trial documentation, the change of the source country is no change.
[3]Document equivalent to the European SmPC

IMP: The SM submission must cover the change in the **Cover Letter**, **Protocol**, **IB**, SmPC[87] / (simplified) IMPD, **GMP/Labelling section** and **product section** (whichever is relevant)**.**

2. **Impact of the change of source country on the regulatory requirements of IMPs and AxMPs with a Marketing Authorisation in EU/EEA (authorised IMP/AxMP)**

Regulatory requirements with regards to labelling:

AxMP with a Marketing Authorisation in EU/EEA

- **Article 67** : No labelling required additionally to what is described in Article 66(1) or in 2001/83/EC
- **Annex VI**: no labelling requirements described for authorised AxMPs

Optionally and at the sponsor's discretion relabelling might be appropriate e.g. for ensuring the authorised AxMP is dedicated to a specific clinical trial.

IMP with a Marketing Authorisation in EU/EEA

- **Article 67:** Labelling required additionally to what is described in Art 66(1) or 2001/83/EC: identification of the CT and of the contact person on outer and immediate packaging, where the specific circumstances, provided for in the protocol so require to ensure the safety of the subject or the reliability and robustness of data generated in the CT
- **Annex VI, chapter C** (Additional labelling for authorised IMPs)

7. In accordance with Article 67(2), the following particulars shall appear on the immediate and the outer packaging: in accordance with Art. 67 (2) CTR ***(a) name of the main contact; (b) CT reference code allowing identification of the CT site, investigator, sponsor and subject; (c) 'For clinical trial use only' or similar wording****.*

8. The particulars listed in sections A, B and ***C****, ...,* ***may be omitted from the label of a product*** *and made available by other means, for example by use of a*

centralised electronic randomisation system, use of a centralised information system, provided that the safety of the subject and the reliability and robustness of data are not compromised. This shall be justified in the protocol.

1.Sourcing by each Investigator/clinical trial site from EU/EEA market – 'local sourcing'

Sourcing case 1a): Authorised IMP/AxMP identified by active substance (AS) name or ATC code (level 3 to 5)

Definition in application dossier: active substance name or ATC code (including if limited to certain pharmaceutical form(s) and strength(s), where applicable per protocol)

- **Annex I B 7. (h):** in the Cover Letter it will be stated that each investigator/clinical trial site will individually source Authorised AxMP/IMP indicated by the active substance name or ATC code and including if limited to certain pharmaceutical form(s) and strength(s)

- **Annex I D 17. (b):** In the Protocol the active substance name (INN name) or ATC code of the Authorised AxMP/IMP will be stated (according to Annex I D 18), including if limited to certain pharmaceutical form(s) and strength(s).

Supporting rationale: Annex I D 18.: If a clinical trial is conducted with an active substance available in the Union under different trade names in a number of authorised medicinal products, the protocol may define the treatment in terms of the active substance or Anatomical Therapeutic Chemical (ATC) code (level 3-5) only and not specify the trade name of each product.

Source country change of Authorised AxMPs/Authorised IMPs within EU/EEA

It is understood that the above way of submission allows for the sourcing of Authorised AxMPs/Authorised IMPs from different MAHs and/or different states of the EU/EEA, provided any language related re-labelling is covered by exemption of art. 61 5. (a) CTR, and provided that there is no change to the pharmaceutical form(s) or strength(s) as covered by the currently authorised protocol version.

The change of the source country is not considered to be a change and as such not to be submitted as NSM, Art. 81.9 NSM or SM.

A change to pharmaceutical form(s) or strength(s) not covered by the currently authorised protocol version is a change to be submitted for authorisation as SM.

Sourcing case 1b): Authorised AxMP/Authorised IMP identified by potential trade name(s)

Definition in application dossier: Trade name(s)

- **Annex I B 7. (h)**: In the Cover Letter it will be stated that each investigator/clinical trial site will source Authorised AxMP/IMP as indicated by trade name(s)

- **Annex I D 17. (b):** In the Protocol the trade name(s) of Authorised AxMPs/Authorised IMPs will be stated.

Source country change of Authorised AxMP/IMP within EU/EEA **without** change of trade name(s) as currently authorised

The change of the source country is not considered to be a change and as such not to be submitted as NSM, Art. 81.9 NSM or SM.

Supporting rationale: Annex VI (C 7.) and any language related re-labelling is covered by exemption of art. 61 5. (a) CTR. A risk for the patient safety or reliability/robustness of the clinical trial by such a change is not seen.

Source country change of Authorised AxMP/IMP within EU/EEA **with** change to a trade name not currently authorised.

The change of the source country is a change to be submitted for authorisation as SM.

Authorised AxMP: The SM submission must cover the change in the Cover Letter, Protocol and update the product section as required;

Authorised IMP: The SM submission must cover the change in the Cover Letter, Protocol, SmPC, GMP/Labelling section and update the product section as required.

The change between sourcing with identification by active substance name or ATC code(Case 1a)) and by potential trade names (Case 1b)) or vice-versa is a **change to be submitted as SM.**

2) Sourcing by/on behalf of Sponsor from EU/EEA market – 'central sourcing'

Sourcing case 2a): Authorised AxMP/Authorised IMP identified by active substance name or ATC code (level 3 to 5)

Definition in application dossier: active substance name or ATC code only (including if limited to certain pharmaceutical form(s) and strength(s), where applicable per protocol)

- **Annex I B 7. (h):** In the Cover Letter the active substance name or ATC code will be listed, including if limited to certain pharmaceutical form(s) and strength(s), where applicable per protocol

- **Annex I D 17. (b):** In the Protocol the active substance name or ATC code will be listed, including if limited to certain pharmaceutical form(s) and strength(s)

Supporting rationale: Annex I D 18.: If a clinical trial is conducted with an active substance available in the Union under different trade names in a number of authorised medicinal products, the protocol may define the treatment in terms of the active substance or Anatomical Therapeutic Chemical (ATC) code (level 3-5) only and not specify the trade name of each product.

Source country change of Authorised AxMPs/Authorised IMPs within EU/EEA

It is understood that the above way of submission allows for the sourcing of Authorised AxMPs/Authorised IMPs from different MAHs and/or different states of the EEA; provided any language related re-labelling2 is covered by the currently authorised site for re-packaging/re-labelling of sourced Authorised AxMPs/IMPs if this site will also do any re-labelling after change of source country.

The change of the source country is not considered to be a change and as such not to be submitted as NSM, Art. 81.9 NSM or SM.

A change to pharmaceutical form(s) or strength(s) not covered by the currently authorised protocol version is a change to be submitted for authorisation as SM.

Sourcing case 2b): Authorised AxMP/Authorised IMP identified by potential trade name(s)

Definition in application dossier: Trade name(s)

- **Annex I B 7. (h)**: In the Cover Letter the Authorised AxMPs/Authorised IMPs will be listed with its trade name(s).

- **Annex I D 17. (b):** In the Protocol the Authorised AxMPs/Authorised IMPs will be listed with its trade name(s).

Source country change of Authorised AxMPs /IMPs within EU/EEA without change of trade name(s) as submitted

The change of the source country is not considered to be a change and as such not to be reported as NSM, art. 81.9 NSM or SM.

Rationale:

- *The currently authorised site for re-packaging/re-labelling of sourced Authorised AxMPs/IMPs isn't changed*

- *A risk for the patient safety or reliability/robustness of the clinical trial by such a change is not seen*

Source country change of Authorised AxMPs/IMPs within EU/EEA with a change to a trade name not initially submitted

The change of the source country is a change to be submitted as SM.

Authorised AxMP: The SM submission must cover the change in the Cover Letter, Protocol and product section;

Authorised IMP: The SM submission must cover the change in the Cover Letter, Protocol, SmPC, GMP/Labelling section and product section.

The change between sourcing with identification by active substance name or ATC code(Case 1a)) and by potential trade names (Case 1b)) or vice-versa is a change to be submitted for authorisation as SM. Likewise, the change between sourcing by Investigator/Clinical trial site and sourcing by/on behalf of Sponsor or vice-versa is a change to be submitted for authorisation as SM.

Annex VI: ABBREVIATIONS (Valid for Chapter 7 on Safety reporting)

AE	Adverse event
AR	Adverse reaction
ASR	Annual safety report
CCDS	Company core data sheet
CCT	Complex clinical trial
CT	Clinical trial
CTCAE	Common terminology criteria for adverse events
DIBD	Development international birth date
DLP	Data lock point
DSMB	Data safety management board
DSUR	Developmental safety update report
EoR	End of recruitment
ET	Early termination
EudraCT	European Union drug regulating authorities clinical trials
EV	Eudravigilance
EVCTM	EudraVigilance clinical trials module
GEOT	Global end of trial
IB	Investigator's brochure
IBD	International birth date
ICSR	Individual case safety report
IMP	Investigational medicinal product
LLT	Lowest level term
MA	Marketing authorisation
MedRA	Medical dictionary for regulatory activities
MS	Member state
NCA	National competent authority
PBRER	Periodic benefit-risk evaluation report
PSUR	Periodic safety update report
PT	Preferred term
PV	Pharmaco-vigilance
RoR	Restart of recruitment
RoT	Restart of trial
RSI	Reference safety information

SAE	Serious adverse event
SAR	Serious adverse reaction
SB	Serious breach
SM	Substantial modification
SmPC	Summary of product characteristics
SOC	System Organ Class
SoR	Start of recruitment
SoT	Start of trial
Sum	Summary of results
SUSAR	Suspected unexpected serious adverse reaction
TH	Temporary halt
TI	Third country inspection
UE	Unexpected event
USM	Urgent safety measure